

काय चिकित्सा 1 (A)

काय, चिकित्सा & कायचिकित्सा	आवरण धातु प्रदोषज विकार	मनस & मानसिक रोग
चिकित्सा चतुष्पाद	सामान्यज & नानात्मज विकार	Nutritional deficiency disorders
रोगी रोग परीक्षा		
क्रियाकाल	Importance of दोष-	वार्धक्यजन्य विकार
वृद्धि & क्षय of दोष – धातु – मल	दूष्यादि in चिकित्सा	इन्द्रियप्रदोषज विकार
	द्विविधोपक्रम	Alzheimer's disease
ओज	षड्विधोपक्रम	Sleep disorders
साम – निराम	शोधन & शमन	General debility
रोगानुत्पत्तिकर चिकित्सा	निदान परिवर्जन	Genetic factors
रोग प्रशमन चिकित्सा	औषध मात्रा	Environmental factors
दोषोपक्रम	औषध सेवन काल	Iatrogenic factors
स्थानान्तर दोष & लीन दोष	अनुपान	Drug allergy
	पथ्य-अपथ्य	Food allergy

काय, चिकित्सा & कायचिकित्सा

काय

निरुक्ति (Etymology of Kaya) →

- चीयतेन्नादिभिः इति कायः ।

The term 'Kaya' means nourishment and building up of the body with ingested food.

- चीयते प्रशस्तदोषधातुमलैः इति कायः ।

Kaya or the body forms dosha, dhatu and mala after assimilation of food.

- जाठरः प्राणिनामाग्निः काय इत्यभिधीयते ।
- कायस्य अन्तरग्रेः।

The digestive fire of the human body is also known as 'Kaya'. The term 'Kaya' denotes the meaning of whole process of metabolism.

पर्याय (Synonyms of Kaya) →

अथ कलेवरं गात्रं वपुः संहननं शरीरं बर्ष्मं विग्रहः ।
कायो देहः क्लीव पुंसो स्त्रियां मूर्तिस्तनुस्तनू ॥ (अमरकोश)

- कलेवर → Effect of combination of shukra and shonita.
- गात्र → That which moves.
- वपु → That which carries.
- संहनन → Formed by the proper combination of pancha-mahabhoota.
- शरीर → That which undergoes destruction continuously.
- बर्ष्म → That which has the capacity to reproduce.
- विग्रहः → That which receives sukha and dukha.
- काय → That which gets nourishment with ingested food.
- देह → That which grows continuously.
- क्लीव → That which has capacity to copulate.

11. पुरुष → Male.

12. स्त्री → Female.

13. मूर्ति → That which has shape.

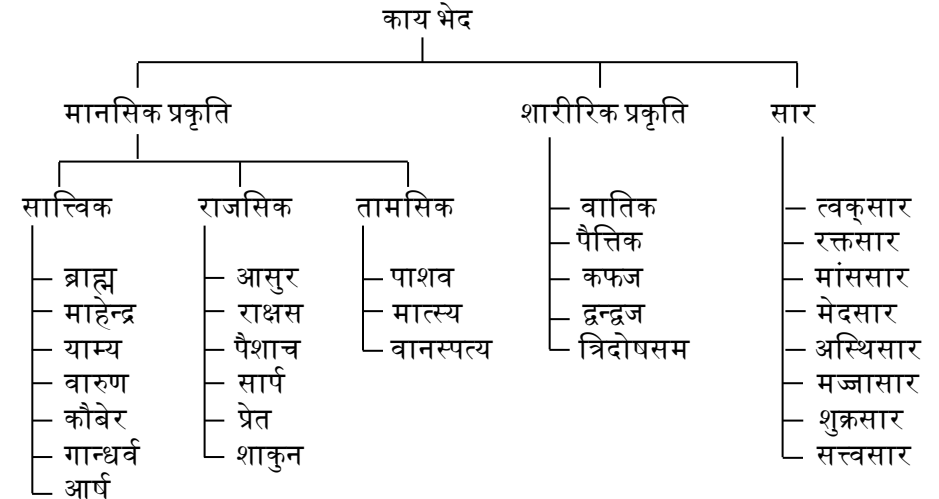
14. तनु → That which expands.

15. तनू → That which grows and expands with proper nourishment.

भेद (Types of Kaya) →

Kaya can be classified on the basis of following factors →

- मानसिक प्रकृति,
- शारीरिक or दोषज प्रकृति,
- सार



काय-भेद based on मानसिक प्रकृति → 16

(A) सात्त्विक काय → 7

Individuals of a sattvik kaya possess a good intellect and memory and have an inherent instinct for cleanliness. Although they usually have good amount of knowledge, they always make efforts to get more. They possess a good will and allow others to prosper well. They are polite and have faith in the divine and devotion to the good.

1. **ब्राह्म काय** → Respectable, authoritative, free from passion and anger. Possessing the knowledge and the power of discrimination.
2. **माहेन्द्र काय** → Devotion to sacred books, studies rituals and oblations. Has good farsightedness, and strong courage. Knowledgeable and has authoritative behavior and speech.
3. **याम्य काय** → Free from mean and conflicting desires and acts. Having initiative and excellent memory as well as leadership. Free from emotional binds, hatred, ignorance and envy. Has good capacity for timely action.
4. **वारुण काय** → Good looking, lovable, free from mean acts, exhibits emotions in its proper place. Observes all religious rites.
5. **कौबेर काय** → Enjoys pleasure in recreation but has liking for virtuous acts and purity. Has good courage, patience and hatred for impure thoughts.
6. **गान्धर्व काय** → Possession of wealth, attendants and luxuries. Likes music, dance and all kinds of arts. Also fond of poetry, stories and epics. Takes pleasure in various perfumes, good clothes, garlands and flowers. Full of passion.
7. **आर्ष काय** → Religious, following all the rituals in correct manner, having excellent memory, purity, love and self control. Free from pride, ignorance, greed or anger. Possessing the power of understanding and retention.

(B) राजसिक काय → 6

Those of a rajasika kaya have a nature that tries to overpower others. They are not satisfied with the positions and possessions they achieve, and therefore always strive for more. Hence they are ambitious and industrious in nature. Usually these people are short-tempered and egoistic. They have a brave but jealous and cruel character.

8. **आसुर काय** → Has great pleasure in self-praise. He is very brave but cruel and full of envy as well as ruthless. He may have appearance that can create terror in others. Likes physical and verbal disguise.
9. **राक्षस काय** → He has constant behavior, which is full of cruel activity, anger and intolerance. Likes to eat, drink and sleep too much.
10. **पैशाच काय** → Has unclean habits. Also likes to eat and drink, but has abnormal diet and daily regimen. Likes opposite sex.
11. **सार्प काय** → He may have brave or cowardly attitude. Has sharp reactions for others comments. Many times has fearful disposition.
12. **प्रेत काय** → Has envious character, is greedy and actions which are without discrimination. Also has excessive desire for food.
13. **शाकुन काय** → He is full of passion. The character is unsteady but ruthless and has excessive appetite.

(C) तामसिक काय → 3

Those of a tamasika kaya are lazy and arrogant – both physically and mentally. Consequently they are not curious about anything. Usually they have lesser intelligence. They prefer not to work and are interested mainly in eating, drinking, and sleeping. They avoid cleanliness and are not health conscious. They are afraid of many things; hence they do not initiate any work on their own.

14. **पाशव काय** → His behavior is like animal. He lacks normal intelligence. Has excessive indulgence in sex and sleep.
15. **मात्स्य काय** → He has unsteady character, with constant passion and is coward in nature. Has excessive desire for intake of water and other drinks.
16. **वानस्पत्य काय** → He also lacks natural intelligence and has excessive indulgence in food and drinks. He has no interest in arts and science as well as scientific achievements.

काय-भेद based on शारीरिक / दोषज प्रकृति → 7

1. वातिक प्रकृति →

शारीरिक लक्षण → Dry, lean, small body with prominent veins and tendons, voice is longdrawn, dry, low, broken, hallow and hoarse. Restless movement of joints, eyes, eyebrows, jaws, lips, hands, feet etc. Rough hair, nails, teeth, cracked limbs, joints produces sound while walking. Vata Prakriti Purushas are having less strength, short life span, less wealth and progeny.

मानसिक लक्षण → Always wakeful, inconsistant behavior, variation of moods, quick in grasping and forgetting, ungrateful, jealous, thievish, unsteady in nfriendship, quickly affected by fear, like music, gambling and hunting. Short tempered, grinds his teeth during sleep.

2. पैत्तिक प्रकृति →

शारीरिक लक्षण → Soft, delicated body with many moles, spots and pimples, flabby and soft joints and limbs, colour of the body is gouravavarna (fair), nails, eyes, tongue, palate, lips, palm and sole arw coppery colour. Eyes are small and unsteady with less lashes, desire of cold things, profusely sweating with strong offensive smell from the body. Frequently passing urine and faeces. Early wrinkles, grey hair and baldness, less hairs with brownish colour and soft.

मानसिक लक्षण → Incapable to bear sufferings, irritable temper, intelligent and scholar, quick grasping mind, makes lengthy speech, brave, never overpowered by fear, dislikes sunlight and hot things. These persons are having moderate knowledge, experience, wealth, strength, and lifespan, small quantity of semen. Limited sex, appetite and offspring.

3. कफज प्रकृति →

शारीरिक लक्षण → Pleasant look, dark complexion, firm and well interlocking joints, stable and strong body, attractive appearance, smooth, plump and rounded limbs, stable movements, deep thundering voice, talks less, strong, curly, black, plenty of hairs, sleeps more, eyes are red with a slight red tinge in the corners, he has profuse semen, more desire for sexual act, more children, very little perspiration. Kapha prakriti persons possess good strength, wealth, knowledge, vitality, gentleness and long life span.

मानसिक लक्षण → He is self controlled forbearing, unselfish and strong mentality. He has no hurry in taking decisions and giving opinions. He is fast in his enmity, but unflinching and unchanging in his friendship. He is grateful, humble and not having excessive desire towards wordly things. He bears hunger, thirst, suffering and sunlight. He has a dignified nature, respectful towards his superiors, obedient to his teachres, makes large gifts after long delibration.

4. 5. & 6. द्वन्द्वज प्रकृति → (वात-पित्त, वात-कफ, पित्त-कफ)

Mixed lakshanas of two doshas.

7. त्रिदोषज सम प्रकृति →

Mixed lakshanas of three doshas.

काय-भेद based on सार → 8

1. त्वक्सार पुरुष → Tvaksara or rasasara purusha has got soft, smooth, thin, shining skin & hairs. They are happy, healthy & wealthy, intelligent, charming & soft nature, and log life.

2. **रक्तसार पुरुष** → Nakha (nails), nayana (eyes), taalu (palate), jihva (tongue), otha (lips), paani (palms), padatara (soles) are so soft and tamra (coppery) or red coloured. Rakta sara purushas are always happy, enthusiastic, and intelligent. Soft nature, less strength. They are unable to withstand stress and strain.
3. **मांससार पुरुष** → No depressions are seen in the body that means body is round and highly muscular. Bones are completely surrounded by muscles and not visible, body is heavy. These persons are courageous, forgiving in nature. Healthy, firm, solid, happy and have good life span.
4. **मेदोसार पुरुष** → These persons sweat more and voice is soft. Huge body but they are unable to withstand stress and strain. Healthy, wealthy, happy, helping and serving in nature.
5. **अस्थिसार पुरुष** → These persons' head and shoulder are big in size and their teeth, nails, mandibular joint and bone are so strong. These persons look very enthusiastic, active and will have good endurance and strong body.
6. **मज्जासार पुरुष** → Majjasara purushas are not lean, having good strength, smooth & deep voice, large eyes, and wealthy. Having pleasant colour with bulky, long and round joints. They possess long life span, good physical stamina and scientific and literary knowledge. They have good progeny and social dignity.
7. **शुक्रसार पुरुष** → Shukrasara persons are soft with solid strong bones, teeth and nails are white, has excessive desire for coitus. They possess healthy progeny many in number, pleasant looks, charming face, and the eyes look as if floating in pure milk. Overwhelming enthusiastic, attractive voice, pleasant colour and broad buttocks.

8. **सत्त्वसार पुरुष** → These persons are having excellent working, thinking, analyzing ability, and has high aims and ambitions which are properly planned. They are strong and happy. They can withstand all types of stress and strain. They are confident in doing all their work; they take Sukha-dukha, Maan-apamaan, Jay-parajay equally. Lifespan is long, and they have got respectable place in the society.

चिकित्सा

निरुक्ति (Etymology of Chikitsa) →

- कित् रोगापनयने ।

The word chikitsa is formed by the root word 'kit roganayanane' that means removal of disease.

- चिकित्सा रूक् प्रतिक्रिया ।

Chikitsa is that which prevents and cures the painful conditions.

- या क्रिया व्याधिहरणी सा चिकित्सा निगद्यते ।

Chikitsa is the procedure that which destroys or removes disease.

पर्याय (Synonyms of Chikitsa) →

चिकित्सितं व्याधिहरं पथ्यं साधनमौषधम् ।

प्रायश्चित्तं प्रशमनं प्रकृतिस्थापनं हितम् ॥ (च.चि.1/3)

1. चिकित्सितं → method of treatment
2. व्याधिहरं → which cures disease
3. पथ्यं → wholesome, and which is beneficial for the srotas
4. साधनं → an instrument for treatment
5. औषधं → which cures disease
6. प्रायश्चित्तं → daivavyapashraya viz. pooja, bali, upahara etc.
7. प्रशमनं → subsides or pacifies the disease
8. प्रकृतिस्थापनं → brings the vitiated dosha to normal state
9. हितं → beneficial to both body and mind

Definition of भेषज →

“रोग भयं जयति” That which defeat or overpower the fear of disease. The term ‘भेषज’ is used as synonym of चिकित्सा (treatment) and औषध (medicine) as well.

भेद (Types of Chikitsa) →

According to Acharya Charaka, there are innumerable (असंख्य) diseases and hence treatments (chikitsa) are also innumerable. There are various classifications of chikitsa →

एकविध चिकित्सा (One type) →

निदान परिवर्जन – Avoiding the causative factors, or the treatment of underlying cause is an important type of chikitsa. Acharya Sushruta has told “संक्षेपतः क्रिया योगो निदानपरिवर्जनम्”.

द्विविध चिकित्सा (Two types) →

- (I) 1. शीत चिकित्सा – Hot therapy for diseases caused by coldness
2. उष्ण चिकित्सा – Cold therapy for diseases caused by hotness
- (II) 1. संतर्पण चिकित्सा (बृंहण) – Tonifying or nourishing the body
2. अपतर्पण चिकित्सा (लघन) – Reducing or making the body light
- (III) 1. शोधन चिकित्सा – Purificatory treatments
2. शमन चिकित्सा – Palliative or pacifying treatments
- (IV) 1. ऊर्जस्कर चिकित्सा – Promoting the strength (preventive rx)
2. रोगघ्न चिकित्सा – Pacifying the disease (curative rx)
- (V) Types of ऊर्जस्कर चिकित्सा
1. रसायन – Rejuvenatives (bringing youth and longevity)
2. वाजीकरण – Aphrodisiacs (helping in progeny)

(VI) Types of रोगघ्न चिकित्सा

1. रोग प्रशमन चिकित्सा – Treatment for pacification of disease
2. अपुनर्भव चिकित्सा – Treatment for un-occurrence of disease

(VII) 1. द्रव्यभूत चिकित्सा – Drug therapy

2. अद्रव्यभूत चिकित्सा – Non drug therapy

त्रिविध चिकित्सा (Three types) →

- (I) 1. दैवव्यपाश्रय चिकित्सा – Devine or spiritual therapy
2. युक्तिव्यपाश्रय चिकित्सा – Rational therapy (objectively planned)
3. सत्त्वाजय चिकित्सा – Psychological therapy
- (II) 1. अन्तः परिमार्जन चिकित्सा – Internal administration of medicines
2. बहिः परिमार्जन चिकित्सा – External application of medicines
3. शस्त्रप्रणिधान – Shastrakarma or surgery
- (III) 1. हेतु विपरीत चिकित्सा – Opposite to etiology (causative factors)
2. व्याधि विपरीत चिकित्सा – Opposite to disease
3. उभयार्थकारि चिकित्सा – Mixed approach
- (IV) 1. दोष प्रत्यनीक चिकित्सा – Treatment of aggravated dosha
2. व्याधि प्रत्यनीक चिकित्सा – Treatment of yvadhi (disease)
3. उभय प्रत्यनीक चिकित्सा – Mixed approach
- (V) According to Bhaishajya Ratnavali →
1. आसुरी चिकित्सा – Surgery is called aasuri chikitsa because of the violent procedures (हिंसा) such as incision, excision etc.
2. मानुषी चिकित्सा – Herbal and herbomineral approach
3. दैवी चिकित्सा – Spiritual approach including parada-preparations

(VI) Types of द्रव्य (drugs used in chikitsa) →

1. भौम द्रव्य (minerals) – पारद, गंधक, अभ्रक, स्वर्ण, रजत, ताम्र, लौह etc.
2. औद्धिद् द्रव्य (herbs) – वनस्पति, वानस्पत्य, वीरुध, औषध.
3. जांगम द्रव्य (animal products) – दुग्ध, घृत, मधु, गोरोचन, प्रवाल, शुक्ति etc.

(VII) Types of अपतर्पण चिकित्सा →

1. लंघन – Fasting (उपवास), or use of light foods (लघु आहार or द्रव्य) such as पेया, विलेपी, मुद्गयूष etc. for those patients who have अल्पदोष or अल्पबल.
2. पाचन – Along with लंघन or उपवास, use of digestants (पाचन द्रव्य) such as चित्रक, मुस्ता, त्रिकटु etc. for those patients who have मध्यमदोष or मध्यमबल.
3. दोषावसेचन – Purificatory procedures (संशोधन) for elimination of the aggravated dosha in those patients who are बलवान or having बहुदोष.

(VIII) Types of कृमि चिकित्सा →

1. अपकर्षण – Expulsion of unwanted things that means to take out or remove by force. This has been again subdivided into 2 types →
 - (i) बाह्य अपकर्षण – e.g. Surgical removal of gulma, arbuda, krimi etc.
 - (ii) आभ्यन्तर अपकर्षण – e.g. Removal of aggravated dosha by shodhana.
2. प्रकृति विघात – It means to break the pathogenesis of disease by means of either external application (बहिर्परिमार्जन) or internal medicine (अन्तःपरिमार्जन).
3. निदान परिवर्जन – Avoiding the etiology factors (cause) of the disease.

चतुर्विध चिकित्सा (Four types) →

Acharya Sushruta has mentioned four types of treatments →

1. संशोधन – Purificatory therapy
2. संशमन – Palliative therapy
3. आहार – Dietic regimen (pathya)
4. आचार – Good conduct of body, mind & speech

पंचविध चिकित्सा (Five types) →

• संशोधन चिकित्सा (पंचकर्म) – According to Charaka →

1. वमन – Emesis
2. विरेचन – Purgation
3. अनुवासन बस्ति – Unctuous enema
4. निरूह बस्ति – Decoction enema
5. नस्य – Errhines / Nasal insufflation

• According to Sushruta →

1. वमन – Emesis
2. विरेचन – Purgation
3. निरूह बस्ति – Enema
4. शिरोविरेचन (नस्य) – Errhines
5. रक्तमोक्षण – Blood letting

षष्ठविध चिकित्सा (Six types) →

षडोपक्रप or षडविध उपक्रम (Acharya Charaka has explained six types of treatment measures) →

1. लंघन – यत्किंचित लाघवकरं देहे तल्लंघनं स्मृतम् ।
Reducing therapy – that which brings lightness in the body.
2. बृंहण – बृहत्वं यच्छरीरस्य जनयेत् तच्च बृंहणम् ।
Nourishing therapy – increases the musculature or bulk of body.
3. रूक्षण – रौक्ष्यं खरत्वं वैशद्य यत्कुर्याद तद्विरूक्षणम् ।
Drying therapy – brings dryness, roughness and non-sliminess.
4. स्नेहन – स्नेहनं स्नेह विष्यन्द मार्दवं क्लेद कारकम् ।
Oleation therapy – brings unctuousness, sliminess, softness & moisture.
5. स्वेदन – स्तम्भगौरव शीतघ्नं स्वेदनं स्वेद कारकम् ।
Sudation – removes stiffness heaviness coldness, and induces sweating.
6. स्तम्भन – स्तम्भनं स्तम्भयति यद् गतिमन्तं चलं ध्रुवम् ।
Astringent therapy – that which stops the flow of doshas.

सप्तविध चिकित्सा (Seven types) →

There are 7 types of शमन चिकित्सा →

1. पाचन (आमदोष पाचन) – Using digestants e.g. चित्रक, मुस्ता
2. दीपन (अग्नि दीपन) – Using appetizers e.g. त्रिकटु (शुण्ठी, मरिच, पिप्पली)
3. क्षुधा (उपवास) – Fasting / or intake of less food
4. तृष्णा – Intake of less or no water
5. आतप सेवन – Exposure to sun rays
6. मारुत सेवन – Exposure to fresh air
7. व्यायाम – Exercise

अष्टविध चिकित्सा (Eight types) →

अष्टविध शस्त्रकर्म – Acharya Sushruta has explained eight types of surgical procedures.

1. छेदन – Excision
2. भेदन – Incision
3. लेखन – Scrapping
4. वेधन – Puncturing
5. एषण – Probing
6. आहरण – Extraction
7. विस्रावण – Drainage or Blood letting
8. सीवन – Suturing or Stitching

दशविध चिकित्सा (Ten types) →

There are 10 types of लघन चिकित्सा in which 4 types of संशोधन and 6 types of अद्रव्यभूत चिकित्सा are included.

1. वमन – Emesis
2. विरेचन – Purgation
3. निरूह बस्ति – Enema
4. नस्य – Errhines

5. पाचन (आमदोष पाचन) – Using digestants e.g. चित्रक, मुस्ता
6. उपवास – Fasting / or intake of less food
7. पिपासा (तृष्णा) – Intake of less or no water
8. आतप सेवन – Exposure to sun rays
9. मारुत सेवन – Exposure to fresh air
10. व्यायाम – Exercise

अष्टादश भेद (Eighteen types) →

There are 18 types of उपशय (therapeutic tests) explained by Acharya Charaka →

- | | |
|-------------------------------|--------------------------------|
| 1. हेतु विपरीत | } × 3 (औषध-अन्न-विहार)
= 18 |
| 2. व्याधि विपरीत | |
| 3. हेतु-व्याधि विपरीत | |
| 4. हेतु विपरीतार्थकारी | |
| 5. व्याधि विपरीतार्थकारी | |
| 6. हेतु-व्याधि विपरीतार्थकारी | |

कायचिकित्सा**परिभाषा (Definition of Kayachikitsa) →**

- कायचिकित्सा: नाम सर्वाङ्गसंश्रितानां व्याधीनां ज्वररक्तपित्तशोषोन्मादापस्मारकुष्ठ-मेहातिसारादीनामुपशमनार्थम् । (सु.सू. 1/11)

Kayachikitsa deals with treatment of all the diseases of body like jwara, raktapitta, shosha, unmada, apasmara, kushtha, prameha, atisara etc.

- जाठरः प्राणिनामग्निः काय इत्यभिधीयते ।
सस्तं चिकित्सेत् सीदन्तं स वै कायचिकित्सकः ॥ (चक्रपाणि)

Kaya is agni of the body, and the physician, who can treat the agni properly, definitely become successful.

- कायचिकित्सा इति कायस्य अन्तरग्रेश्चिकित्सा । (गंगाधर)
Kayachikitsa is the treatment of agni of the body.

चिकित्सा चतुष्पाद

The prognosis of any disease – whether the disease is easily curable, difficult to cure, or incurable – depends upon its nature and the healthy combination of the four factors required for the successful treatment → the physician, the medicine, the attendant, and the patient himself.

भिषग्द्रव्याण्युपस्थाता रोगी पादचतुष्टयम् ।
चिकित्सितस्य निर्दिष्टं, प्रत्येक तद्वतुरुणम् ॥ (अ.ह.सू.1/27)

There are four limbs of the treatment → भिषक or वैद्य (physician), द्रव्य (medicine), उपस्थाता or परिचारक (attendant), and रोगी (patient). Each of them is having four qualities those are important for successful treatment.

भिषक (physician) →

दक्षस्तीर्थात्तशास्त्रार्थो दृष्टकर्मा शुचिर्भुषक । (अ.ह.सू.1/27)

1. Skilled in the practical application of medical knowledge
2. Acquired theoretical knowledge
3. Acquired experience in practical work
4. Purity in mind, body (action) and speech

द्रव्य (medicine) →

बहुकल्पं बहुगुणं सम्पन्नं योग्यमौषधम् । (अ.ह.सू.1/27)

1. Available in many formulations
2. Having many qualities
3. Potent (having good potency)
4. Capable to cure respective disease, and suitable to the patient

परिचारक (attendant) →

अनुरक्तः शुचिर्दक्षो बुद्धिमान् परिचारकः । (अ.ह.सू.1/27)

1. Sympathy for the patient
2. Purity in mind, body (action) and speech
3. Skilled in nursing techniques
4. Intelligent

रोगी (patient) →

आढ्यो रोगी भिषग्वश्यो ज्ञापकः सत्त्वावानपि । (अ.ह.सू.1/27)

1. Should be able to afford the treatment
2. Should be obedient, or follow the advice of physician
3. Should be able to describe his ailments
4. Should have will power (courageous with balanced mind, and willing to overcome disease)

चिकित्स्य पुरुष

The person who is treated is called 'Chikitsya purusha'. Treatment of Ayurveda is aimed at living person; and the living person has been described as चिकित्स्य पुरुष that is having various synonyms such as चेतन पुरुष, कर्म पुरुष, संयोग पुरुष, स्थूल पुरुष, राषि पुरुष, & चतुर्विंशतिक पुरुष, and various classifications based on its constitution such as एकधातुज पुरुष, षड्धातुज पुरुष, & चतुर्विंशति तत्वात्मक पुरुष.

1. एकधातुज पुरुष → composed of आत्मा (चेतना धातु).
2. षड्धातुज पुरुष → composed of पंचमहाभूत + चेतना धातु.
3. चतुर्विंशति तत्वात्मक पुरुष → composed of 24 elements → अष्टप्रकृति (अव्यक्त – महान – अहंकार – पंचमहाभूत) & षोडश विकार (पंचतन्मात्रा – पंचज्ञानेन्द्रिय – पंचकर्मेन्द्रिय – मन)

अचिकित्स्य पुरुष

The patients unsuitable for treatment are called 'Achikitsya purusha'. The physician should reject those patients who are अचिकित्स्य →

तद्यथा – अनपवाद प्रतीकारस्याधनस्यापरिचारकस्य वैद्यमानिनश्चण्डस्यासूयकस्य - तीव्रधर्मारुचिरतिक्षीणबलमांसशोणित तस्यासाध्यरोगोपहतस्य मुमुर्षुलिंगान्वितस्य चेति ॥ (च.वि.3/45)

- अनपवाद प्रतीकार → Those who don't follow the beneficial advice.
- अधन → Those who don't have money to afford the treatment.
- अपरिचारक → Those who don't have nurse or caretaker.
- वैद्यमानिन → Those who pose themselves as physician.
- चण्ड → Those who are having violent behavior.
- असूयक → Those who are envious or cynic (निंदक).
- तीव्र धर्म अरुचि → Those who don't have faith in dharma (rightful acts), or who enjoys the vicious acts (adharma).
- अतिक्षीण बल मांस शोणित → Excessive depletion of strength, muscles or flesh, and blood.
- असाध्य रोगोपहत → Those who are suffering from incurable disease.
- मुमुर्षु लिंगान्वित → Those who are having symptoms of imminent death (अरिष्ट लक्षण).

If the physician takes such patients under his treatment, then he is defamed because of unsuccessful treatments, and such treatment is considered as sinful act for the physician.

रोगी-रोग परीक्षा

Examination of the patient and disease is essential for the proper diagnosis so that the disease can be treated successfully,

रोगी परीक्षा → The patient should be examined first and then the disease. Examination of the patient can be done by following methods →

(A) त्रिविध परीक्षा →

1. दर्शन – Inspection – Direct observation of the patient, वर्ण (change in colour), रूक्षता (dryness), ग्लानि (lethargy), प्रमाण (measurements), आकृति (structure), वैरूप्यता deformities etc.
2. स्पर्शन – Palpation – शीतता-उष्णता (temperature), गुरुता-लघुता, सुप्तता-असुप्तता, खरता-क्षुण्णता, सस्पन्दता-अस्पन्दता (palpitations), organomegaly, tenderness etc.
3. प्रश्न – Interrogation (History taking) – Name, age, occupation, address, complaints, H/o present illness, H/o past illness, family history etc. and आहार, विहार, कोष्ठ, अग्नि, मल-मूत्र प्रवृत्ति, निद्रा, व्यसन etc.

(B) पंचविध परीक्षा (पंचज्ञानेन्द्रिय) →

1. चक्षुरिन्द्रिय परीक्षा – प्राकृत & विकृत वर्ण, छाया, आकृति, प्रमाण etc.
2. त्वगिन्द्रिय (स्पर्शनेन्द्रिय) परीक्षा – प्राकृत & विकृत स्पर्श, body temperature, organomegaly, tenderness etc.
3. श्रोत्रेन्द्रिय परीक्षा – Gurgling sound (आन्त्रकूजन), cracking & crepitations in joints, auscultatory sounds like wheezing, ronchi, murmurs etc. abnormal sounds, voice of patient like normal voice or hoarseness of voice etc.
4. रसनेन्द्रिय परीक्षा – प्राकृत & विकृत रस (Examined by अनुमान)
5. घ्राणेन्द्रिय परीक्षा – प्राकृत & विकृत गन्ध

(C) षड्विध परीक्षा →

1. to 5. पंचज्ञानेन्द्रिय परीक्षा
6. प्रश्न परीक्षा – Interrogation (History taking)

(D) अष्टविध (अष्टस्थान) परीक्षा →

1. **नाडी परीक्षा** – Examination of pulse – Examination of radial pulse, 1 cm below wrist joint of right hand of male patient or left hand of female patient with the help of first three fingers of physician → examine the गति (rate), ताल (rhythm), यति (volume), तनाव (tension) & वेग (force) of the pulse, and identify the वात, पित्त & कफ- pulsations at the index, middle and ring fingers respectively. दोषानुसार नाडीगतis are as follows→
 - (i) वात प्रकोप → जलौका or सर्प गति
 - (ii) पित्त प्रकोप → काक or मण्डूक गति
 - (iii) कफ प्रकोप → पारावत or हंस गति
2. **मूत्र परीक्षा** – For urine examination collect the sample of urine in early morning. Following tests are done for मूत्र परीक्षा →
मूत्र प्रवृत्ति (frequency of micturition), धार (stream of urine while micturition), संहनन (density of urine), मात्रा (quantity), गन्ध (smell), वर्ण (colour), सरक्त (presence of blood), सपूय (presence of pus), सशर्करा (presence of calculus), सशुक्र (presence of semen), तैलबिन्दु परीक्षा etc.
3. **मल परीक्षा** – Stool examination – The stool also should be examined like urine → मल प्रवृत्ति (frequency of defaecation), मात्रा, संहनन, वर्ण, गन्ध, सरक्तता, सापूयता, सकृमि, सकफ, आम, निराम etc.
4. **जिह्वा परीक्षा** – Tongue is examined for its वर्ण, आकृति, अंकुर, लिप्तता, स्पर्श, गति etc. For example, in अग्निमान्द्य, जिह्वा (tongue) becomes लिप्त or मलावृत (coated). In वातप्रकोप it becomes शीत, रूक्ष, स्फुटित (fissure);

In पित्तप्रकोप – रक्त-श्याम वर्ण; In कफप्रकोप – श्वेत & पिच्छिल; And in त्रिदोष प्रकोप – कृष्ण, कण्टकयुक्त जिह्वा.

5. **शब्द परीक्षा** – Examination of voice (स्वर) of the patient – in वातप्रकोप it becomes रूक्ष स्वर, in पित्तप्रकोप – तीव्र स्वर, in कफप्रकोप – गम्भीर स्वर.
6. **स्पर्श परीक्षा** – Examination of skin (त्वचा) of the patient – in वातप्रकोप it becomes शीत & कृष्णवर्ण त्वचा, in पित्तप्रकोप – उष्ण & पीतवर्ण त्वचा, in कफप्रकोप – आर्द्र (क्लेदयुक्त) & श्वेतवर्ण त्वचा.
7. **दृक परीक्षा** – Examination of eyes (नेत्र) of the patient – वर्ण, पाण्डुता (pallority), पीतता (icterus), रक्तता (redness), शोफ (swelling) etc. should be examined. In वातप्रकोप eyes become रूक्ष, धूमवर्ण; In पित्तप्रकोप – पीत, दाहयुक्त; In कफप्रकोप – स्निग्ध, जल (द्रव) युक्त.
8. **आकृति परीक्षा** – Examination of body built of the patient – Physique of the patient is examined on the basis of कृश, स्थूल, सामान्य, अतिह्रस्व, अतिदीर्घ etc.

(E) दशविध परीक्षा →

1. **प्रकृति** – Constitution of the patient (e.g. वातज, पित्तज, कफज etc.)
2. **विकृति** – Strength of the disease (e.g. प्रवर, मध्यम, अवर)
3. **सार** – dhatus of best quality (e.g. त्वक्सार, रक्तसार, मांससार etc.)
4. **संहनन** – Compactness of the body (प्रवर, मध्यम, अवर)
5. **सात्त्विक** – Habitual and suitable foods etc. (प्रवर, मध्यम, अवर)
6. **सत्त्व** – Mental strength (प्रवर, मध्यम, अवर)
7. **प्रमाण** – Measurements of the body (प्रवर, मध्यम, अवर)
8. **आहारशक्ति** – Strength of digestive fire is examined in two ways →
 - (i) अभ्यवहरण शक्ति – intake capacity (प्रवर, मध्यम, अवर)
 - (ii) जरण शक्ति – digestive capacity (प्रवर, मध्यम, अवर)
9. **व्यायामशक्ति** – strength of the patient is examined by his exercising or normal working capacity (प्रवर, मध्यम, अवर)
10. **वय** – age group of the patient (बाल-मध्यम-जीर्ण)

(F) प्रमाण (परीक्षा) →

- त्रिविध परीक्षा → आसोपदेश – प्रत्यक्ष – अनुमान
 - चतुर्विध परीक्षा → आसोपदेश – प्रत्यक्ष – अनुमान – युक्ति
1. आसोपदेश परीक्षा → Examination of the patient on the basis of आसवचन (told by acharyas) such as involvement of dosha-dushya, aggravating factors, how the disease is manifested, prognosis, complications, nomenclature of the disease etc.
 2. प्रत्यक्ष परीक्षा → Direct observation or examination by sense organs like पंचज्ञानेन्द्रिय परीक्षा or दर्शन-स्पर्शन परीक्षा etc
 3. अनुमानपरीक्षा → Examination of the patient with the help of inference, e.g. examination of अग्नि by observing पाचनशक्ति, examination of बल by व्यायामशक्ति, examination of ज्ञानेन्द्रिय by अर्थग्रहण शक्ति etc. It helps to examine and diagnose those conditions which cannot be observed directly. अनुमान प्रमाण gives knowledge of त्रिकाल (past-present-future).
 4. युक्ति परीक्षा → The physician has to use his skill and specification of circumstances such as देश (place), काल (time) etc. to plan the treatment by observing all the tests and effect of treatment.

रोग परीक्षा →

रोग परीक्षा plays important role in the diagnosis (रोग-निदान) as well as treatment (चिकित्सा). With the help of निदान पंचक the disease (रोग) is examined and then treatment is planned on the basis of भेद, बल, साध्यसाध्यता etc.

निदान पंचक →

निदान – पूर्वरूप – रूप – उपशय – सम्प्राप्ति

These are called निदान पंचक because of their utility in the diagnosis (रोग-निदान) and being five in number (पंचक).

1. निदान (हेतु) →

व्याधिविनिश्चय करणं निदानम् । (मधुकोष मा.नि. 1/4)

The causative factors of the disease are called निदान. It has various synonyms such as हेतु, निमित्त, आयतन, कारण etc.

It has various classifications →

Two types → (1) बाह्य हेतु – e.g. मिथ्या आहार-विहार

(2) आभ्यन्तर हेतु – e.g. दोष-दूष्य समूच्छेदन

(1) उत्पादक हेतु – produces disease directly

(2) व्यञ्जन हेतु – aggravating factors of disease

(1) अनुबन्ध हेतु – (प्रधान) – primary cause

(2) अनुबन्ध हेतु – (अप्रधान) – secondary cause

Three types → (1) असात्म्येन्द्रियार्थ संयोग – improper use of sense organs

(अयोग or हीनयोग – मिथ्यायोग – अतियोग of इन्द्रियाः)

(2) प्रज्ञापराध – sinful acts

(मिथ्यायोग of वाक्-मन-शरीर)

(3) परिणाम (काल) – seasonal imbalance

(अयोग – मिथ्यायोग – अतियोग of काल)

Four types → (1) सन्निकृष्ट हेतु – nearest cause – such as दोष प्रकोप

according to day, night, before and after food etc.

(2) विप्रकृष्ट हेतु – distant or slow cause – such as कफ-संचय in हेमंत ऋतु is responsible for कफ-प्रकोप in वसंत ऋतु.

(3) व्यभिचारी हेतु – weak cause – the etiological factors those produce disease with combination of other factors or when get favourable conditions.

(4) प्राधानिक हेतु – powerful cause – such as poison that manifests disease immediately.

Importance →

संक्षेपतः क्रियायोगो निदानपरिवर्जनम् । (सु.उ.1/25)

Avoiding the etiological factors is the first line of treatment for all the diseases. So निदान not only helps in diagnosis but also helps in prevention and treatment of disease.

2. पूर्वरूप →

पूर्वरूपं प्रागुत्पत्ति लक्षणं व्याधे । (च.नि.1/8)

Before complete manifestation of disease, the pre-clinical symptoms or prodromal symptoms are called पूर्वरूप.

It has two types →

- (1) सामान्य पूर्वरूप – e.g. – ज्वर सामान्य पूर्वरूप – श्रम अरति विवर्ण वैरस्य नयनप्लव
- (2) विशेष पूर्वरूप – e.g. – ज्वर विशेष पूर्वरूप – जृम्भा in वातज ज्वर
नयनदाह in पित्तज ज्वर
अनन्नाभिलाषा in कफज ज्वर

Importance →

स्थानसंश्रयिणः कुद्धा भावि व्याधि प्रबोधकम् । (मा.नि.1/5-6)

At the stage of स्थानसंश्रय, the prodromal symptoms (पूर्वरूप) arise and indicate the upcoming disease. So पूर्वरूप helps in diagnosis as well as prevention and treatment of disease at early stage.

3. रूप (लक्षण) →

व्याधेः स्वरूपं व्यक्तं तद् रूपम् । (मधुकोष मा.नि. 1/7)

Signs and symptoms of disease are called रूप or लक्षण.

It has two types →

- (1) सामान्य रूप – general symptoms of disease
- (2) विशेष रूप – specific symptoms of different types of disease – वातज, पित्तज, कफज etc.

Importance →

व्याधेः स्वरूपं...

For proper diagnosis the physician must have knowledge of clinical features of the disease.

4. उपशय →

उपशयः पुनः हेतुव्याधिविपरीतानां विपरीतार्थकारिणां च औषधाहारविहारानां उपयोगः सुखानुबन्धः ।

Therapeutic tests with use of relieving factors of the disease such as medicines, foods or activities are called उपशय. The aggravating factors are called अनुपशय.

उपशय has eighteen types →

अष्टादश उपशय	औषध	आहार	विहार
हेतु विपरीत	उष्ण द्रव्य प्रयोग in वात कफज ज्वर	मांसरस प्रयोग in वातज ज्वर	विश्राम in श्रमज ज्वर
व्याधि विपरीत	रास्ना & गुग्गुलु प्रयोग in वातव्याधि	यव प्रयोग in स्थौल्य	चक्रमण in प्रमेह
हेतु-व्याधि विपरीत	दशमूल प्रयोग as शोथहर & वातहर in वातिक शोथ	पेया प्रयोग in शीतजन्य वातिक ज्वर	रात्रिजागरण in स्निग्ध आहार & दिवास्वप्नजन्य तन्द्रा
हेतु विपरीतार्थकारी	उष्ण उपनाह प्रयोग in पित्तज व्रण	उष्ण & तीक्ष्ण द्रव्य प्रयोग in कामला	developing भय in वातज उन्माद
व्याधि विपरीतार्थकारी	मदनफल प्रयोग in हृल्लास or छर्दि	दुग्ध प्रयोग in अतिसार	प्रवाहण in हृल्लास or छर्दि
हेतु-व्याधि विपरीतार्थकारी	जांगल विष प्रयोग in स्थावर विष	मद्य सेवन in मदात्यय	प्लवन -swimming in उरुस्तम्भ

Importance →

गूढलिंगव्याधि उपशयानुपशयाभ्यां परीक्षेत् ।

Both the उपशय & अनुपशय helps in the diagnosis of गूढलिंग व्याधि (diseases of complex symptoms).

5. सम्प्राप्ति →

यथादुष्टेन दोषेन यथा चानुविसर्पिता ।
निवृत्तिरामयस्यासौ सम्प्राप्तिः जातिः आगतिः ॥

The mechanisms by which the doshas get vitiated, and pathways through which they produce the disease (pathogenesis), are called सम्प्राप्ति. It has synonyms such as सम्प्राप्ति, जाति, आगति.

It has six types →

- (1) संख्या – number of types of disease – e.g. अष्ट ज्वर, पंच गुल्म etc.
- (2) प्राधान्य – predominant dosha in particular disease denoted by तर, तम.
- (3) विकल्प – अंशांश कल्पना of प्रकुपित दोष that means the aggravated qualities of dosha such as गुरु-लघु, स्निग्ध-रूक्ष, शीतता-उष्णता etc..
- (4) बल – strength of the disease.
- (5) काल – relation of disease with time variations such as day, night, seasonal changes, before and after food etc.
- (6) विधि – subtypes of disease other than number - e.g. उर्ध्वग & अधोग रक्तपित्त, नवज्वर, जीर्णज्वर, पुनरावर्तक ज्वर, मृद्भक्षणजन्य पाण्डु etc.

As the क्रियाकाल also gives knowledge of pathogenesis of the disease, the षड्क्रियाकालाः (six stages of the disease) are also considered as types of सम्प्राप्ति itself.

Importance →

सम्प्राप्ति विघटनमेव चिकित्सा ।

Treatment is nothing but to break the pathogenesis of the disease.

षड् क्रियाकाल

Disease (व्याधि) is not a state; it is rather a process of changing manifestations, involving different stages. The physician must have knowledge of different stages of a particular disease for its prevention and cure. In Ayurveda, the different stages of the pathogenesis of diseases are described as क्रियाकाल.

संचयं च प्रकोपं च प्रसरं स्थानसंश्रयम् ।
व्यक्तिं भेदं च यो वेत्ति दोषाणां स भवेद् भिषक् ॥ (सु.सू.21/36)

There are six stages of the pathogenesis →

1. संचय, 2. प्रकोप, 3. प्रसर, 4. स्थानसंश्रय, 5. व्यक्त, 6. भेद

Who has the knowledge of all these stages is called physician (भिषक्). These stages (अवस्था) suggest the physician what action (क्रिया) should be taken to prevent or cure the disease, and that is why these are called क्रियाकाल.

1. संचय (Accumulation of doshas) →

‘संहति रूपा वृद्धिः चय’ Dosha-vriddhi in its own place (seat) is called chaya. Accumulation of doshas first takes place in their normal seats of predominance. Lakshanas of this stage are as follows →

तत्र संचितानां खलु दोषाणां स्तब्धपूर्णकोष्ठता पीतावभासता मन्दोष्मता चांगानां गौरवमालस्यं चयकारणविद्वेषश्चेति लिंगानि भवन्ति । (सु.सू. 21/18)

As soon as this accumulation starts, it is reflected in the mind in the form of a desire for the qualities opposite to those of the particular dosha and the patient develops hatred (द्वेष) for substances having similar qualities of the accumulated dosha. दोषानुसार लक्षणाः are →

- a. वात दोष संचय → स्तब्धता – पूर्णकोष्ठता
- b. पित्त दोष संचय → पीतावभासता – मन्दोष्मता
- c. कफ दोष संचय → अंग गौरवता – आलस्य

2. प्रकोप (Aggravation of doshas) →

‘विलयन रुपा वृद्धिः प्रकोपः’ Dosha-vridhi with its excitation or aggravation is called prakopa. दोषानुसार प्रकोप लक्षण →

तेषां प्रकोपात् कोष्ठतोदसञ्चरणाम्लिकापिपासापरिदाहान्नद्वेषहृदयोत्क्लेदाश्च जायन्ते ।
(सु.सू.21/27)

- वात प्रकोप → कोष्ठ तोद – सञ्चरण (वायु सञ्चरण)
- पित्त प्रकोप → अम्लिका (अम्लोद्गार) – पिपासा (तृष्णा) – परिदाह
- कफ प्रकोप → अन्नद्वेष (अरुचि) – हृदयोत्क्लेद (हृल्लास)

Aggravations of doshas are of two types →

- चय पूर्वक प्रकोप → Aggravation of dosha followed by its accumulation. This can be either (a) स्वाभाविक चय प्रकोप – such as seasonal chaya & prakopa, or (b) अस्वाभाविक चय प्रकोप – such as chaya & prakopa due to मिथ्या आहार विहार.
- अचय पूर्वक प्रकोप → Sudden aggravation of doshas can also be occurred without the stage of accumulation (i.e. अचय) due to harsh-activities (अतिव्यायाम), day-sleep (दिवास्वप्न), sun-burn (आतप सेवन), trauma (आगन्तुज) etc.

3. प्रसर (Spread of aggravated doshas) →

If the earlier stage (प्रकोपावस्था) is not treated, then the aggravated doshas expand and overflow just like starch with yeast, when soaked in water (पिष्ट+सुराबीज+जल) for overnight, it gets fermented and rises up in its vessel. In प्रसरावस्था the doshas start expanding, overflowing from their seats, and spreading to other parts of the body. दोषानुसार प्रसर लक्षण →

एवं प्रकुपितानां प्रसरतां वायोर्विमार्गगमनाटोपो, ओषचोषपरिदाहधूमायनानि पित्तस्य, अरोचकाविपाकांगसादश्छर्दिश्चेति श्लेष्मणो लिंगानि भवन्ति । (सु.सू. 21/32)

- वात प्रसर → वायु विमार्ग गमन – आटोप
- पित्त प्रसर → ओष – चोष – परिदाह – धूमायन
- कफ प्रसर → अरुचि – अविपाक – अंग साद – छर्दि

There are 15 types of spread (प्रसर) of aggravated dosha →

- | | | |
|---------------|-----------------|-------------------------|
| (1) वात | (6) वात-कफ | (11) वात-पित्त-शोणित |
| (2) पित्त | (7) पित्त-कफ | (12) वात-कफ-शोणित |
| (3) कफ | (8) वात-शोणित | (13) पित्त-कफ-शोणित |
| (4) रक्त | (9) पित्त-शोणित | (14) वात-पित्त-कफ |
| (5) वात-पित्त | (10) कफ-शोणित | (15) वात-पित्त-कफ-शोणित |

4. स्थानसंश्रय (Localization of the aggravated doshas) →

एवं प्रकुपिताः तांस्ताञ् शरीरप्रदेशानागम्य तांस्तान् व्याधीन् जनयन्ति । तेषामेव-मभिनिविष्टानां पूर्वरूपप्रादुर्भावः ॥ (सु.सू. 21/33)

The aggravated doshas circulating in all over the body during प्रसरावस्था, if not treated properly, then advance to next stage and get settled or localized in a part of the body, where they find favourable condition, or weak tissue, organ, channel or vitiated srotas (ख वैगुण्य). This stage of disease gives rise to prodromal symptoms (पूर्वरूप).

प्रकुपित दोष gets स्थानसंश्रय in different locations producing different types of diseases →

- उदरगत → गुल्म – विद्रधि – उदर रोग – अग्निसंग – आनाह – विसूचिका – अतिसार etc.
- बस्तिगत → प्रमेह – अश्मरी – मूत्राघात – मूत्रदोष etc.
- मेद्वगत → निरुद्धप्रकश – उपदंश – शूकदोष etc.
- वृषणगत → वृद्धि रोग etc.
- गुदगत → भगन्दर – अर्श etc.
- ऊर्ध्वजत्रुगत → ऊर्ध्वजत्रुगत विकार etc.
- त्वक्मांसरक्तगत → क्षुद्ररोग – कुष्ठ – विसर्प etc.
- मेदोगत → ग्रन्थि – अपची – अर्बुद – गलगण्ड – अलजी etc.
- अस्थिगत → अस्थिविद्रधि etc.
- सन्धिगत → सन्धिगत वात etc.
- पादगत → क्षीपद – वातरक्त – वातकण्ठक etc.

5. व्यक्त (Manifestation of signs & symptoms of disease) →

Later on दोष-दूष्य-सम्पूर्ण gets completed and the complete manifestation of signs and symptoms (रूप) of the disease takes place. If the disease is not treated in पूर्वरूपावस्था, it advances to रूपावस्था. For example: ज्वर लक्षण → सन्ताप (increased body temperature), अतिसार लक्षण → अतिद्रव सरण (excessive passage of watery stool) etc.

6. भेद (Chronicity or Differentiation of disease) →

After some time if the disease process continues, then the disease becomes chronic, and the physician can differentiate the exact types of dosha vitiation. If this stage is not treated properly, then complications may occur and disease becomes incurable (असाध्य).

Importance of Kriyakala →

1. 'क्रियाकालो चिकित्सा अवसरः' The knowledge of stages of disease provides opportunity to prevent it in earlier stage or treat it properly.
2. Kriyakala helps to understand the progress of diseases as well as to plan suitable preventive measures.
3. If the physician skips the earlier stages due to any reason and disease progresses to later stage, then also he can treat the disease and stop it to progress further with the help of Kriyakala.
4. Different stages require different types of treatment measures those should be applied to the respective stages only →

i. संचयावस्था → रोग-अनुत्पत्तिकर चिकित्सा	}	दोष प्रत्यनीक चिकित्सा
ii. प्रकोपावस्था → हेतु विपरीत चिकित्सा		
iii. प्रसरावस्था → हेतु विपरीत चिकित्सा		
iv. स्थानसंश्रयावस्था → दोष & दूष्य चिकित्सा	→	उभयप्रत्यनीक चिकित्सा
v. व्यक्तावस्था → व्याधि विशिष्ट चिकित्सा	}	व्याधि प्रत्यनीक चिकित्सा
vi. भेदावस्था → जीर्ण व्याधि विशिष्ट चिकित्सा		

वृद्धि & क्षय of दोष - धातु - मल

सामान्य निदान of दोषधातुमल क्षय →

रूक्ष & अल्प आहार – अनशन – अतिव्यायाम – अतिव्यवाय – प्रजागर – आतप सेवन – मारुत सेवन – चिन्ता – शोक – भय – कफ शोणित शुक्र मल अतिवर्तन – जीर्ण आयु etc.

- सर्वदा सर्वभावानां सामान्यं वृद्धिकारणम् ।
ह्लासहेतुः विशेषश्च॥ (च.सू.1/44-45)

Indulgence in the foods & activities (आहार-विहार) of similar qualities (सामान्य) results in वृद्धि, whereas the foods & activities of dissimilar qualities (विशेष) result in क्षय.

निदान of वात-वृद्धि (वातप्रकोप) →

- a. आहारज निदान → कटु तिक्त कषाय रस अतिसेवन – रूक्ष लघु शीतवीर्य आहार सेवन – अनशन – विषमाशन – सतीन – कलाय – मसूर – आढकी – शुष्क शाक etc.
- b. विहारज निदान → अतिव्यायाम – अतिव्यवाय – लंघन (उपवास) – प्लवन – पतन – अभिघात – रात्रिजागरण – वेगधारण etc.
- c. मानसिक निदान → अति शोक – चिन्ता – भय – ग्लानि etc.
- d. अन्य निदान → धातु क्षय – रक्तस्राव – सांयकाल – वर्षा ऋतु – वृद्धावस्था etc.

निदान of पित्त-वृद्धि (पित्तप्रकोप) →

- a. आहारज निदान → कटु अम्ल लवण रस अतिसेवन – क्षार तीक्ष्ण विदाही उष्णवीर्य आहार सेवन – अध्यशन – तिल – अतसी – सर्षप – कुलत्थ – माष – हरितशाक – कांजी – दधि – मांस of मत्स्य, गोधा, वाराह, माहिष, गव्य, अविक etc.
- b. विहारज निदान → अति आतप सेवन – अति स्त्री प्रसंग – अतिव्यायाम etc.
- c. मानसिक निदान → क्रोध – शोक – भय – ईर्ष्या – द्वेष etc.
- d. अन्य निदान → मध्याह्न काल – मध्य रात्रि – शरद ऋतु – मध्यावस्था etc.

निदान of कफ-वृद्धि (कफप्रकोप) →

- आहारज निदान → मधुर अम्ल लवण रस अतिसेवन – गुरु स्निग्ध पिच्छिल अभिष्यन्दि शीतवीर्य आहार सेवन – अध्यशन – समशन – नवान्न – इक्षुरस – गुड विकार – क्षीर विकार – दधि – गोधूम – माष – तिलपिष्ट – आनूप मांस etc.
- विहारज निदान → दिवास्वप्न – अतिस्वप्न – अव्यायाम – आलस्य etc.
- मानसिक निदान → हर्ष – संतोष – अचिन्ता – तमोगुण प्रधानता etc.
- अन्य निदान → प्रातः काल – भोजनोपरांत – वसंत ऋतु – बाल्यावस्था etc.

वृद्धि	लक्षण	चिकित्सा
वात वृद्धि	काश्य – काण्ठ्य – उष्णकामित्व – कंप – आनाह – शकुद् ग्रह – बल इन्द्रिय भ्रंश – प्रलाप – भ्रम – दीनता	- स्नेहन-स्वेदन पश्चात् मृदु संशोधन - अनुवासन & निरुह बस्ति - नस्य, गण्डूष, धूमपान - शिरोबस्ति, अभ्यंग, मर्दन, वेष्टन - मधुर-अम्ल-लवण रस युक्त, उष्ण & स्निग्ध आहार - वातशामक द्रव्य प्रयोग – तिलतैल, दशमूल, रास्ना, एरण्ड, निर्गुण्डी, अर्क, हरिद्रा, बला, अश्वगन्धा, लशुन, मांसरस etc.
पित्त वृद्धि	पीत विट् मूत्र नेत्र त्वक् – क्षुदा – तृष्णा – दाह – संताप – मूर्च्छा – अल्पनिद्रता – शीतकामित्व	- विरेचन & रक्तमोक्षण - शीत प्रदेह, परिषेक, अभ्यंग - मधुर-तिक्त, कषाय रस प्रधान & शीत द्रव्य सेवन - क्षीर & घृत प्रयोग - पित्तशामक द्रव्य प्रयोग – पंचतिक्त, चन्दन, उशीर, ह्रीबेर, मुक्ता, प्रवाल, यष्टिमधु etc.
कफ वृद्धि	अग्निसदन (अग्निमान्द्य) – प्रसेक – आलस्य – गौरव – श्वेत्य (श्वेतता) – शैत्य (शीतता) – क्ष्वांगत्व – श्वास – कास – अतिनिद्रता	- स्वेदन, वमन, शिरोविरेचन प्रयोग - तीक्ष्ण विरेचन - व्यायाम, रूक्ष उद्वर्तन - कटु-तिक्त-कषाय रस, तीक्ष्ण, ऊष्ण, रूक्ष आहार - कफशामक द्रव्य प्रयोग – मधु, पंचकोल, त्रिकटु, त्रिफला, लवंग, यवक्षार etc.

क्षय	लक्षण	चिकित्सा
वात क्षय	साद – अल्पभाषिता – संज्ञानाश – मोह – कफवृद्धि लक्षण	- कटु-तिक्त, कषाय रस प्रधान, रूक्ष, लघु, शीत गुण प्रधान द्रव्य प्रयोग – मुद्ग, सतीन, कलाय, आढकी, जांगल मांसरस etc. - रात्रिजागरण, व्यायाम, चिन्ता
पित्त क्षय	मन्दोष्मा – मन्दाग्नि (अग्निमान्द्य) – प्रभाहानि	- कटु-अम्ल-लवण रस प्रधान, क्षार, तीक्ष्ण, उष्णवीर्य द्रव्य प्रयोग – तिल, माष, कुलत्थ, अतसी, मत्स्य मांस, त्रिकटु etc. - आतपसेवन, व्यायाम
कफ क्षय	भ्रम – श्लेष्माशय शून्यत्व – हृद् द्रव – क्षथसंधिता	- मधुर-अम्ल-लवण रस प्रधान, गुरु, स्निग्ध, पिच्छिल, शीतवीर्य द्रव्य प्रयोग – नवान्न, गोधूम, इक्षुरस, गुड विकार, क्षीर, दधि, आनूप मांस etc. - दिवास्वप्न, विश्राम, हर्ष (अचिन्ता)

क्र.	धातु	वृद्धि लक्षण	क्षय लक्षण
1	रस	अग्निमान्द्य – प्रसेक – उत्क्लेद – आलस्य – गौरव – श्वेत्य – शैथिल्य – कास – श्वास – अतिनिद्रा	शब्द असहिष्णुता – हृद् द्रव – हृत्पीडा – ग्लानि – क्लम – शून्यता – तृष्णा – शोष – प्राकृत कर्म ह्रास – रक्तादि धातु अपचय
2	रक्त	विसर्प – प्लीहावृद्धि – विद्रधि – कुष्ठ – वातरक्त – रक्तपित्त – गुल्म – उपकुश – कामला – व्यंग – अग्निनाश – सम्मोह – त्वक् नेत्र मूत्र रक्तवर्णता	रक्ताल्पता – पाण्डु – त्वक् रूक्षता – त्वक् स्फोटन – सिरा शैथिल्य – त्वक् म्लानता – अग्निमान्द्य – शिरःकम्प – भ्रम – तिमिर
3	मांस	स्फिक्-गण्ड-औष्ठ-शिश्न-उरु-बाहु-जंघा स्थूलता – गौरव – अधिमांस – अर्बुद – अर्थ – ओष्ठ प्रकोप	स्फिक्-गण्ड-औष्ठ-शिश्न-वक्ष-ग्रीवा-कक्षा-उरु-उदर शुष्कता – रूक्षता – तोद – श्रम – धमनी शैथिल्य – संधिवेदना – नेत्र ग्लानि
4	मेद	स्निग्धांगता – उदर पार्श्व – वृद्धि – कास – श्वास – दौर्गन्ध्य – क्षुदातुर – तृष्णा – निद्रा – प्रमेह – स्थौल्य	प्लीहा वृद्धि – संधिशून्यता – रूक्षता – मांस भक्षण अभिलाषा – संधिस्फुटन – कृशता

क्र.	धातु	वृद्धि लक्षण	क्षय लक्षण
5	अस्थि	अध्यस्थि - अधिदन्त - केश नख अतिवृद्धि	अस्थिशूल - संधिशूल - अस्थि शिथिलता - केश लोम नख श्मश्रु रोमादि प्रपतन
6	मज्जा	सर्वांग & नेत्र गौरव - तमोदर्शन - भ्रम - मूर्च्छा - अस्थि पर्व व्रण	अल्पशुक्रता - पर्वभेद - अस्थि छिद्रता - अस्थि निस्तोद - भ्रम - तिमिर
7	शुक्र	शुक्राश्मरी - शुक्र अतिप्रवृत्ति - स्वप्रदोष - अतिस्त्रीकामता - शुक्र प्रदोषज विकार	दौर्बल्य - मुखशोष - पाण्डु - सदन - श्रम - क्लैब्य - शुक्र चिरस्त्राव - मेढ्र वृषण वेदना

क्र.	उपधातु	वृद्धि लक्षण	क्षय लक्षण
1	स्तन्य	स्तन स्थूलता - मुहुर्मुहुः स्तन्य प्रवृत्ति - स्तन तोद	स्तनम्लानता - क्षीराल्पता - क्षीर अनुत्पत्ति
2	आर्तव	आर्तव अतिप्रवृत्ति (रक्त प्रदर) - अंगमर्द - दौर्बल्य - दाह - पाण्डु - दौर्गन्ध्य	यथा उचितकाल अदर्शन - अल्प आर्तव - योनिवेदना
3	कण्डरा, सिरा, स्नायु	संकोच - खल्ली आदि विकार	कण्डरा सिरा स्नायु प्रदोषज विकार - स्तम्भ - ग्रन्थि - स्फुरण - सुप्ति

क्र.	मल	वृद्धि लक्षण	क्षय लक्षण
1	पुरीष	कुक्षि शूल - आटोप - आध्मान - कुक्षि गौरव - वेदना	हृदय पार्श्व पीडा - आन्त्रकूजन - सशब्द मल प्रवृत्ति - वायु उर्ध्वगमन - कुक्षि संचरण
2	मूत्र	मूत्राधिक्य - मुहुर्मुहुः मूत्र प्रवृत्ति - बस्तितोद - आध्मान	अल्पमूत्रता - बस्तितोद - मूत्रकृच्छ्र - अतितृष्णा - मुख शुष्कता - मूत्र वैवर्ण्य
3	स्वेद	तक् दौर्गन्ध्य - कण्डू - अतिस्वेद - रोमहर्ष	रोमकूप अवरोध - रोमकूप स्तब्धता - त्वक् रूक्षता - त्वक् स्फुटन - स्पर्श ज्ञान अभाव - रोम प्रपतन

दोष-धातु-मल वृद्धि & क्षय चिकित्सा →

दोषाः क्षीणा बृंहयितव्याः, कुपिताः प्रशमयितव्याः, वृद्धाः निर्हृतव्याः, समाः परिपाल्या इति सिद्धान्तः । (सु.चि.33/3)

To maintain the health of a patient, the physician should increase the decreased dosha, or decrease the increased dosha to bring the equilibrium state of dosha, with respective treatment measures.

सर्वदा सर्वभावानां सामान्यं वृद्धिकारणम् ।

ह्रासहेतुः विशेषश्च॥ (च.सू.1/44-45)

In case of वृद्धि, the physician should use विशेष चिकित्सा → as the foods and activities of dissimilar qualities of dosha, dhatu & mala cause क्षय and bring it to normal state.

In case of क्षय, the physician should use सामान्य चिकित्सा → as the foods and activities of similar qualities of dosha, dhatu & mala cause वृद्धि and bring it to normal state.

रसधातु प्रदोष चिकित्सा → लंघन (उपवास)

रक्त प्रदोष चिकित्सा → रक्तपित्तशामक चिकित्सा - विरेचन - रक्तमोक्षण

मांस प्रदोष चिकित्सा → संशोधन (पंचकर्म) - शस्त्रकर्म - क्षारकर्म - अग्निकर्म

मेदो प्रदोष चिकित्सा → अपतर्पण or कर्षण - स्थौल्यहर & कफ-मेदोहर चिकित्सा

अस्थि प्रदोष चिकित्सा → पंचकर्म - तिक्त क्षीर बस्ति - तिक्त घृत प्रयोग

मज्जा प्रदोष चिकित्सा → मधुर तिक्त द्रव्य - व्यवाय - व्यायाम - यथाकाल संशोधन

शुक्र प्रदोष चिकित्सा → शुक्रल द्रव्य प्रयोग - शोधन - वाजीकरण

पुरीष वृद्धि चिकित्सा → मलविरेचक & विबंधहर द्रव्यप्रयोग (e.g. एरण्डतैल, हरीतकी etc.)

पुरीष क्षय चिकित्सा → यव, माष, कुल्माष, शाक, धान्याम्ल आदि मलवर्धक द्रव्य प्रयोग.

मूत्र वृद्धि चिकित्सा → गोक्षुरादि मूत्रल or मूत्रविरेचनीय द्रव्य प्रयोग

मूत्र क्षय चिकित्सा → इक्षुरस, वारुणीमण्ड, द्रव, मधुर अम्ल लवण मूत्रवर्धक द्रव्य प्रयोग

स्वेद वृद्धि चिकित्सा → पित्तशामक द्रव्य, पंचतिक्त घृत गुग्गुलु प्रयोग

स्वेद क्षय चिकित्सा → अभ्यंग पश्चात् स्वेदन द्रव्य प्रयोग

ओज

ओजस्तु तेजो धातूनां शुक्रान्तानां परं स्मृतम् । (अ.ह.सू. 11/37)

Ojas is the essence of all the saptadhatu and it is formed after the shukradhatu. It is also called बल or प्राकृत श्लेष्मा.

ओज स्थान → हृदय

ओज प्रकार → 1. पर ओज → अष्टबिन्दु प्रमाण & हृदय गत
2. अपर ओज → अर्ध अंजली प्रमाण & सर्वशरीर गत

ओज गुण → गुरु शीतं मृदु क्षुण्णं बहलं मधुरं स्थिरम् ।
प्रसन्नं पिच्छिलं स्निग्धं ओजो दशगुणं स्मृतम् ॥ (च.चि.24/31)

ओज कर्म → प्राकृतस्तु बलं श्लेष्मा । (च.सू.17/73)

Ojas is responsible for the natural strength (innate immunity) of the body. Its functions are → to nourish the body, to provide strength and immunity, to make voice and complexion pleasant, to support life by residing in hridaya pradesha. Loss of ojas leads to death.

ओजोदुष्टी हेतु (निदान) →

रूक्ष & अल्प आहार – अनशन (क्षुधा) – अतिव्यायाम – अतिव्यवाय – स्वप्न विपर्यय – चिन्ता – क्रोध – शोक – भय – कफ शोणित शुक्र मल अतिवर्तन (निर्हरण) – धातुक्षय – अभिघात – जीर्ण आयु (वृद्धावस्था) etc.

ओजोदुष्टी →

There are three types of ojavaha sroto-dushti →

1. ओजो विस्रंस – Displacement of ojas from its normal place.
2. ओजो व्यापत् – Vitiation of ojas due to dushta dosha and dushya.
3. ओजो क्षय – Decrease of ojas in its normal quantity.

ओजोविस्रंस लक्षण → सन्धिविक्षेपो गात्राणां सदनं दोषच्यवनं क्रियासन्निरोधश्च विस्रंसे ।

- सन्धि विक्षेप (Looseness of the joints)
- गात्र सदन (Weakness of the body)
- दोष च्यवन (Displacement of doshas)
- क्रिया सन्निरोध (Impairment in activities)

ओजोव्यापद लक्षण → स्तब्धगुरुगात्रता वातशोफो वर्णभेदो ग्लानिस्तन्द्रानिद्रा च व्यापन्ने ।

- स्तब्धगुरुगात्रता (Stiffness & heaviness in body)
- वातशोफ (Swelling due to vata)
- वर्णभेदो (Discolouration or loss of complexion)
- ग्लानि (Exhaustion)
- तन्द्रा & निद्रा (Drowsiness and excessive sleep)

ओजोक्षय लक्षण → मूर्च्छा मांसक्षयो मोहः प्रलापो मरणमिति च क्षये ।

- मूर्च्छा (Fainting)
- मांसक्षय (Muscle wasting)
- मोह (Unconsciousness)
- प्रलाप (Delirium)
- मरण (Death)

ओजोदुष्टी चिकित्सा →

- In case of vitiation of ojavaha srotas, the physician should use हृद्य & ओजोऽनुकूल विशेष क्रिया,
- मधुर, स्निग्ध, शीतवीर्य, लघु, जीवनीय गण, बल्य, रसायन, ओजोवर्धक द्रव्य प्रयोग.
- नित्य गोदुग्ध & गोघृत सेवन.
- पथ्य आहार & विहार सेवन.
- हर्ष & अचिन्ता.
- As the ओज-क्षयावस्था is incurable (असाध्य) it should not be treated.

साम-निराम अवस्था

अन्न विष →

अपच्यमानं शुक्लत्वं यात्यन्नं विषरूपताम् । (च.चि.15/44)

Acharya Charaka has mentioned that undigested food is a poison. And the undigested food is responsible for production of ama.

आम →

ऊष्मणोऽल्पबलत्वेन धातुमाद्यमपाचितम् ।

दुष्टं आमाशयगतं रसमामं प्रचक्षते ॥ (अ.ह.सू.13/25)

Reduced digestive activities (मन्दाग्नि) lead to production of improper formed or uncooked first dhatu (अविपक्व रसधातु). This undigested and harmful toxic waste product gets accumulated in stomach (आमाशय), which is known as आम.

आमदोष हेतु (निदान) →

- आहारज निदान → अभोजन (अनशन) – अजीर्ण भोजन (अध्यशन) – अकाल भोजन (विषमाशन) – अतिभोजन – असात्म्य & विरुद्ध आहार – गुरु शीत अतिरूक्ष & दूषित आहार सेवन etc.
- विहारज निदान → दिवास्वप्न – आलस्य – वेगधारण etc.
- मानसिक निदान → शोक – तमोगुण प्रधानता etc.
- अन्य निदान → स्नेहन मिथ्यायोग – वमन & विरेचन मिथ्यायोग – ऋतु विषमता etc.

आम लक्षण →

मल विष्टम्भ – स्तम्भ – अंग सदन – शिरःशूल – पृष्ठ कटिग्रह – जृम्भा – अंगमर्द – तृष्णा – ज्वर – छर्दि – प्रवाहिका – अरुचि – अविपाक etc.

आमज रोग →

अलसीभूत आम – अलसक

विलम्बित आम – विलम्बिका

उर्ध्व अधः प्रवृत्त आम – विसूचिका

द्रव बहुल आम – अतिसार

कफ प्रधान आम – आमाजीर्ण & प्रवाहिका

पित्त प्रधान आम – विदग्धाजीर्ण, अम्लपित्त & भस्मक रोग

वात प्रधान आम – विष्टब्धाजीर्ण & ग्रहणी रोग

शेष रसजन्य आम – रसशेषाजीर्ण

साम →

अपक्व अन्नरस is called आमदोष; when this आमदोष is mixed with दोष & रोग, they are called साम दोष and साम रोग respectively.

साम → स + आम → with आम

निराम →

दोष & रोग without आमदोष are called निराम दोष and निराम रोग respectively.

निराम → निः + आम → without आम

साम-निराम सामान्य लक्षण →

स्रोतरोध बलभ्रंश गौरवानिलमूढताः ।

आलस्यापत्ति निष्ठीव मलसंग अरुचिक्लमाः ॥

लिंगं मलानां सामानां, निरामाणां विपर्ययः ॥ (अ.ह.सू.13/23-24)

साम लक्षण	निराम लक्षण
स्रोतरोध – बलभ्रंश – शरीर गौरव – अनिल मूढता – आलस्य – अविपाक – निष्ठीव – मलसंग – अरुचि – क्लम	प्राकृत स्रोतस – प्राकृत बल – शरीर लघुता – वायु अनुलोमता – न आलस्य – सम अग्नि – प्राकृत मल प्रवृत्ति – भोजन रुचि – न क्लम

साम-निराम दोष →

साम वात	विबन्ध – अग्निमान्द्य – तन्द्रा – आन्त्रकूजन – वेदना – शोफ – तोद – क्रमशः अंग पीडन – स्नेहन पश्चात् लक्षण वृद्धि – सूर्योदय, मेघोदय & रात्रि वृद्धि
निराम वात	विशद रूक्ष वायु – निर्विबन्ध – अल्पवेदना – विपरीत गुणैः शान्ति (स्नेहन पश्चात् लक्षण शान्ति) – त्वक् श्याम & ताम्र वर्णता

साम पित्त	दुर्गन्धि, हरित श्याववर्ण, अम्ल, घन, गुरु पित्त – अम्लीकाकण्ठ – हृददाह
निराम पित्त	ताम्र पीत वर्ण, कटु, अत्युष्ण, अस्थिर, विगन्धि, रुचि-पक्ति-बलवर्धक पित्त

साम कफ	आविल (turbid or viscus), तन्तु (thready), स्त्यान (thick) कफ – कण्ठदेशे अवतिष्ठते – दुर्गन्धी – क्षुधानाशक – उद्गारनाशक
निराम कफ	फेनवान् (frothy), पिण्डित (bolus), पाण्डु (white coloured), निःसार (clear, light substance), अगन्ध (free from foul smell), पक्व, मुखशुद्धिकारक कफ (cleanses oral cavity)

साम धातु लक्षण (धातु प्रदोषज विकार) →

साम रस	अश्रद्धा – अरुचि – आस्यवैरस्य – अरसज्ञता – हृल्लास – गौरव – तन्द्रा – अंगमर्द – ज्वर – तम – पाण्डु – स्रोतरोध – क्लैब्य – कृशता – अग्निनाश – अकाल वली
साम रक्त	कुष्ठ – विसर्प – पिडका – रक्तपित्त – असृग्दर (रक्तप्रदर) – वातरक्त – गुदपाक – मेढ्रपाक – मुखपाक – प्लीहा – गुल्म – विद्रधि – नीलिका – कामला – व्यंग – पिप्लु – तिलकालक – दद्रू – चर्मदल – श्वित्र (किलास) – पामा – कोठ – रक्तमण्डल – न्यच्छ – इन्द्रलुप्त – अर्श – अर्बुद – अंगमर्द
साम मांस	अधिमांस – अर्बुद – मांसकीलक – गलशालूक – गलशुण्डी – पूतिमांस – अलजी – गलगण्ड – गण्डमाला – उपजिवह्रिका – मांससंघात – ओष्ठ प्रकोप
साम मेद	अष्ट निन्दित पुरुष – प्रमेह पूर्वरूप (मुखमाधुर्य, हस्तपाद शून्यता व दाह, मुख तालु कण्ठ शोष, पिपासा, आलस्य, मलाधिक्य, मत्स्यगन्धी, मूत्रदोष etc.) – ग्रन्थि – वृद्धिरोग – गलगण्ड – अर्बुद – मेदोज ओष्ठप्रकोप – मधुमेह – अतिस्वेद
साम अस्थि	अध्यस्थि – अधिदन्त – दन्तभेद – दन्तशूल – अस्थिभेद – अस्थिशूल – अस्थि व दन्त विवर्णता – केश लोम नख श्मश्रु दोष – कुनख रोग
साम मज्जा	रूक् पर्वणां (सन्धिशूल) – भ्रम – मूर्च्छा – तमोदर्शन – नेत्राभिष्यन्द
साम शुक्र	क्लैब्य – अहर्षण – शुक्राशमरी – शुक्रमेह – शुक्रदोष – निष्फल शुक्र (shukra dhatu incapable to produce गर्भ, if conception occurs it leads to गर्भपात or birth of अल्पायु and / or विरूप सन्तान)

निराम धातु लक्षण →

Dhatus in niramavastha perform their normal functions →

प्रीणनं जीवनं लेपः स्नेहो धारण पूरणे ।

गर्भोत्पादश्च धातूनां श्रेष्ठ कर्म क्रमात् स्मृतम् ॥ (अ.ह.सू.11/4)

साम-निराम मल →

साम पुरीष	गुरु – जले मज्जति – दुर्गन्ध्युक्त – पिच्छिल – भ्रंश & विच्छिन्न पुरीष
निराम पुरीष	जले प्लवति – दुर्गन्ध रहित – पक्व & पिण्डित पुरीष – शरीर लघुता

आम दोष चिकित्सा →

आमप्रदोषजानां पुनर्विकाराणां अपतर्पणेनैव परमो भवतिः । (च.वि.2/13)

अपतर्पण चिकित्सा is best for treatment of आम प्रदोषज विकाराs. Acharya Charaka has mentioned three types of अपतर्पण चिकित्सा →

अपतर्पणमपि च त्रिविधं – लंघनं लंघनपाचनं दोषावसेचनं चति । (च.वि.3/43)

1. लंघन – Fasting (उपवास), or use of light foods (लघु आहार or द्रव्य) such as पेया, विलेपी, मुद्गयूष etc. for those patients who have अल्पदोष or अल्पबल.
2. पाचन – Along with लंघन or उपवास, use of digestants (पाचन द्रव्य) such as चित्रक, मुस्ता, त्रिकटु etc. for those patients who have मध्यमदोष or मध्यमबल.
3. दोषावसेचन – Purificatory procedures (संशोधन) for elimination of the aggravated dosha in those patients who are बलवान् or having बहुदोष.

शांतिरामविकाराणां भवति तु अपतर्पणात् ।

त्रिविधं त्रिविधे दोषे तत्समीक्ष्य प्रयोजयेत् ॥

तत्राल्पे लंघनं पथ्यं मध्ये लंघन पाचनम् ।

प्रभूते शोधनं तद्धि मूलादुन्मूलयेन्मलान् ॥ (अ.ह.सू.8/20-21)

For the three types of दोष-बल (अल्प-मध्यम-बहुदोष) the physician should use three types of अपतर्पण चिकित्सा →

1. For अल्प दोष → लंघन-पथ्य (उपवास & मण्ड, पेया, विलेपी, यूष, तक्र प्रयोग)
2. For मध्यम दोष → लंघन-पाचन (उपवास or लघु आहार & चित्रक, मुस्तादि प्रयोग)
3. For बहुदोषावस्था → संशोधन चिकित्सा (स्वेदन, वमन, विरेचन, निरूह वस्ति)

आमदोष नाशक औषध योग → अग्नितुण्डी रस – अजीर्णकण्टक रस – पंचकोल चूर्ण – त्रिकटु चूर्ण – लवणभास्कर चूर्ण – हिंवाष्टक चूर्ण – अजमोदादि चूर्ण – शंख वटी – अग्नितुण्डी वटी – चित्रकादि वटी – रसोन वटी – हिंवादि वटी – दशमूलारिष्ट – जीरकाद्यारिष्ट – कुमार्यासव – धान्यपंचक क्वाथ – शुण्ठी जीरक क्वाथ – पिप्पल्यादि घृत – चित्रकादि घृत

रोगानुत्पत्तिकर चिकित्सा

आयुर्वेद प्रयोजन →

प्रयोजनं चास्य स्वस्थस्य स्वास्थ्यरक्षणं आतुरस्य विकारप्रशमनं च । (च.सू.30/26)

The aims of Ayurveda are – (i) स्वस्थस्य स्वास्थ्यरक्षणं (i.e. prevention of disease), and (ii) आतुरस्य विकारप्रशमनं (i.e. cure of disease).

To prevent the disease, physician should use रोगानुत्पत्तिकर चिकित्सा (preventive treatment), and to cure the disease रोगप्रशमन चिकित्सा (curative treatment).

“रोगानुत्पत्तिकर → रोग + अन् + उत्पत्ति कर”

The treatment that prevents the occurrence of disease is called रोगानुत्पत्तिकर चिकित्सा. It is preventive treatment which includes स्वस्थवृत्त पालन, सद्वृत्त पालन, & रसायन-वाजीकरण प्रयोग.

1. स्वस्थवृत्त →

- दिनचर्या पालन → To follow the daily regimen such as waking up in ब्रह्ममुहूर्त, then after शौच कर्म – दन्तधावन, जिह्वानिलेखन, अंजन, प्रतिमर्ष नस्य, गण्डूष-कवल, धूमपान, अभ्यंग, उद्वर्तन, स्नान, स्वच्छ वस्त्रधारण etc.
- रात्रिचर्या → To follow night regimen such as avoiding the आहार, मैथुन, निद्रा, अध्ययन and मार्ग गमन in the evening (संध्याकाल). And to follow right methods of भोजन and शयन in the night (रात्रि).
- ऋतुचर्या पालन → To follow the seasonal regimen such as हेमन्त, शिशिर, वसन्त, ग्रीष्म, वर्षा, शरद ऋतुचर्या. The physician should use seasonal purificatory procedures (ऋतु अनुसार संशोधन) and advice regimens to avoid aggravation of respective doshas.
- योग → To practice yoga, asana, pranayama, dhyaan etc.

2. सद्वृत्त → To follow good conduct, and avoid the sinful acts.

3. रसायन & वाजीकरण प्रयोग → Rejuvenatives & Aphrodisiacs.

रोगप्रशमन चिकित्सा

रोगस्तु दोषवैषम्यं । (अ.ह.सू.1/20)

The imbalance of dosha and dushya is roga (disease). धातु वैषम्यता or दोष वैषम्यता is teated by application of रोगप्रशमन चिकित्सा (curative treatment).

To treat the disease (रोग) which is produced by धातु वैषम्यता, physician hast to bring दोष / धातु साम्यता (i.e. equilibrium state of dosha and dhatu).

It is achieved by following measures → शोधन चिकित्सा & शमन चिकित्सा

1. शोधन चिकित्सा → (purificatory treatments)

यद् इर्येद् बहिर्दोषान् पंचधा शोधनं च तत् ।

निरूहो वमनं कायशिरोरेको अस्त्रविसृतिः॥ (अ.ह.सू.14/5)

शोधन कर्म are the purificatory procedures those eliminate the aggravated doshas from the body. These are five in numbers; hence they are called as पंचकर्म. According to Acharya Sushruta and Vagbhata, Shodhana karmas are as follows →

वमन – विरेचन कर्म – शिरोविरेचन (नस्य कर्म) – निरूह वस्ति – रक्तमोक्षण

2. शमन चिकित्सा → (palliative or pacifying treatments)

न शोधयति यद्दोषान् समानोदीरयत्यपि ।

समीकरोति क्रुद्धाश्च तत् संशमनमुच्यते॥ (सु.सू.1, डल्हण)

Treatment measures those neither eliminate the doshas, nor aggravate the doshas, but they pacify the aggravated dosha and bring equilibrium in the body, are called Shamana chikitsa. (*detailed in page no.35*)

For रोगप्रशमन, following treatment measures can also be used →

→ द्रव्यभूत & अद्रव्यभूत चिकित्सा

→ संतर्पण & अपतर्पण चिकित्सा

→ हेतुविपरीत, व्याधिविपरीत & उभयार्थकारि चिकित्सा

→ दैवव्यपाश्रय, युक्तिव्यपाश्रय & सत्वाजय चिकित्सा

→ अन्तःपरिमार्जन, बहिःपरिमार्जन & शस्त्रप्रणिधान चिकित्सा

→ संशोधन, संशमन, आहार & आचार चिकित्सा

दोषोपक्रम

1. वातदोष उपक्रम →

वातस्योपक्रमः स्नेहः स्वेदः संशोधनं मृदु।

स्वाद्वस्त्रलवणोष्णानिभोज्यान्यभ्यंगमर्दनम् ॥ (अ.ह.सू.13/1-3)

- शोधन → स्नेहन – स्वेदन – मृदु संशोधन – निरूह बस्ति – अनुवासन बस्ति – स्निग्ध ऊष्ण बस्ति – मात्रा बस्ति – नस्यकर्म – धूमपान – शिरोबस्ति – गण्डूष etc.
- शमन → अभ्यंग – मर्दन – वेष्टन – उत्सादन – उपनाह – ऊष्ण परिषेक etc.
- आहार & औषध → मधुर-अम्ल-लवण रस युक्त, उष्ण & स्निग्ध आहार – दीपनीय, पाचनीय, वातशामक आहार – गौडिक मद्यपान – वृष्य & बल्य द्रव्य सेवन – मांसरस – तैल – गोधूम – नवान्न – दशमूल – रास्ना – एरण्ड – निर्गुण्डी – अर्क – हरिद्रा – बला – अश्वगन्धा – लशुन etc.
- विहार → आतपसेवन – ऊष्ण जलावगाहन – प्रावार – मृगचर्म – विस्मापन – विस्मारण – हेमन्त ऋतुचर्या पालन etc.

Among all above measures, निरूह बस्ति & अनुवासन बस्ति are the best, and among all the dravyas तैल is best to mitigate the प्रकुपित वात.

2. पित्तदोष उपक्रम →

पित्तस्य सर्पिषः पानं स्वादुशीतैर्विरेचनम्।

स्वादुतिक्तकषायाणि भोजनान्यौषधानि च ॥ (अ.ह.सू.13/4-9)

- शोधन → स्नेहन – विरेचन – रक्तमोक्षण etc.
- शमन → शीत प्रदेह – परिषेक – अभ्यंग etc.
- आहार & औषध → मधुर, तिक्त, कषाय रस प्रधान & पित्तहर द्रव्यसिद्ध घृत प्रयोग – क्षीर पान – शीतल हृद्य सुगन्धित आहार – जांगल मांसरस – पित्तशामक द्रव्य प्रयोग e.g. पंचतिक्त, चन्दन, उशीर, ह्रीबेर, मुक्ता, प्रवाल, यष्टिमधु etc.
- विहार → शीतल & मनोऽनुकूल विहार – प्रदेह – परिषेक – मुक्तामणि धारण – शीत वायु सेवन – भूगृह – चन्द्रकिरण सेवन – जलाशय & उद्यान विहार – सुगन्धित विहार – स्त्री स्पर्श – कर्पूर चन्दन उशीर लेप – मनोऽनुकूल गीत वाद्य यन्त्र – शरद ऋतुचर्या पालन etc.

Among all above measures, विरेचन कर्म is the best, and among all the dravyas घृत is best to mitigate the प्रकुपित पित्त.

कफदोष उपक्रम →

श्लेष्मणो विधिना युक्तं तीक्ष्णं वमनरेचनम्।

अन्नं रूक्षाल्पतीक्ष्णोष्णं कटुतिक्तकषायकम् ॥ (अ.ह.सू.13/10-12)

- शोधन → स्वेदन – तीक्ष्णोष्ण संशोधन (वमन, विरेचन, शिरोविरेचन, गण्डूष etc.)
- शमन → रूक्षोन्मर्दन – रूक्ष उद्वर्तन – उत्सादन – उपनाह – ऊष्ण स्नान etc.
- आहार & औषध → कटु तिक्त कषाय रस प्रधान – तीक्ष्ण – उष्ण – क्षार – रूक्ष आहार & औषध – मेदोघ्न उपचार – कफशामक द्रव्य प्रयोग e.g. मधु, पंचकोल, त्रिकटु, यवक्षार etc.
- विहार → लघु व्यायाम – जलावगाहन – चंक्रमण – स्त्रीसेवन – उपवास (लंघन) – रात्रिजागरण – चिन्ता – उष्ण स्थान निवास – वसन्त ऋतुचर्या पालन etc.

Among all above measures, वमन कर्म is the best, and among all the dravyas मधु is best to mitigate the प्रकुपित कफ.

स्थानान्तर दोष

स एव कुपितो दोषः समुत्थानविशेषतः।

स्थानान्तरगतश्चैव जनयत्यामयान् बहून् ॥ (च.सू.18/45)

When aggravated, a single dosha may cause various diseases, depending upon the various etiological factors and sites of manifestation. Even if the सम or प्राकृत दोष gets displaced or moves to other place, under the influence of वायु, it results in दोष वृद्धि of that place, and causes various diseases. This displaced dosha is called स्थानान्तरगत दोष.

आशयापकर्षक हेतु → Under the influence of aggravated vata, displacement or movement of normal dosha to other places, which results in manifestation of various disease, is also called आशयापकर्षक हेतु.

स्थानान्तरगत दोष चिकित्सा →

- Generally treatment should be adopted according to स्थान.
For example, In case of पित्तस्थानगत वात, treat the पित्त.
- In कफस्थानगत पित्त, treat the कफ.
- In वातस्थानगत कफ, treat the वात.

- While treating स्थानान्तरगत दोष, the physician should also consider दोष-बलाबल. If आगत दोष is सम, स्थानीय दोष should be treated, but if आगत दोष is बलवान, treatment is given for आगत दोष.
- When दोष moves to शाखा from कोष्ठ, it should be brought to कोष्ठ with the help of स्नेहन-स्वेदन and eliminated with शोधन (पंचकर्म).
- The aggravated doshas (कुपित दोष) produce various diseases due to various etiological factors (विभिन्न कारण) and different sites of manifestations (स्थानान्तर). Hence physician should consider various factors such as रोग प्रकृति, निदान, स्थान, दोष-बलाबल etc. and then plan the treatment.

लीन दोष

लीनमिति अनुत्क्लिष्टम् । (च.चि.15/76, चक्रपाणि)

Hidden or secretly accumulated doshas are called leena doshas.

लीनान-क्षिष्टान् अनुत्क्लिष्टान् स्वस्थानाद् ।

चलितान् न निर्हरेत्-वमनदिभिः न शोधयेत् ॥ (अ.ह.सू.13/28, सर्वांगसुन्दर)

Leena doshas are those which are stuck in their place, not showing prominent features, and can not be eliminated easily by shodhana like vamana –virechana etc. लीन दोषाs do not show prominent features but they produce various chronic diseases.

लीन दोष चिकित्सा →

- लीन दोष (secretely accumulated or hidden dosha) should not be eliminated forcefully, otherwise it can lead to destruction of their sites just as extraction of juice from unripe fruit.
- These doshas should be aggravated first (i.e. उत्क्लेष) and then eliminated with the help of suitable purificatory procedures.
- If आम is hidden in intestines (पक्वाशयस्थ लीन), then the patient should be given puration mixed with appetizers (सदीपन विरेचन).

आवरण

Avarana means Vata dosha gets enveloped or occluded and obstructed with other dosha and dushya.

आवरक → the dosha-dushya those are causing avarana.

आवृत → obstructed vata

Types of आवरण →

1. दोषावृत वात → 13
2. धातु, अन्न & मलावृत वात → 9
3. अन्योन्यावरण → 20

Total 42 types of आवरण are explained by Acharya Charaka →

13 दोषावृत वात → पित्तावृत वात – कफावृत वात – पित्तावृत प्राणवात – कफावृत प्राणवात – पित्तावृत उदानवात – कफावृत उदानवात – पित्तावृत समानवात – कफावृत समानवात – पित्तावृत व्यानवात – कफावृत व्यानवात – पित्तावृत अपानवात – कफावृत अपानवात – कफपित्तावृत वात (मिश्रावरण)

मिश्रावरण is again of 5 types → कफपित्तावृत प्राणवात – कफपित्तावृत उदानवात – कफपित्तावृत समानवात – कफपित्तावृत व्यानवात – कफपित्तावृत अपानवात

9 धातु, अन्न & मलावृत वात → रक्तावृत वात – मांसावृत वात – मेदसावृत वात – अस्थ्यावृत वात – मज्जावृत वात – शुक्रावृत वात – अन्नावृत वात – मलावृत वात (पुरीषावृत) – मूत्रावृत वात

20 अन्योन्यावरण → उदानावृत प्राणवात – समानावृत प्राणवात – व्यानावृत प्राणवात – अपानावृत प्राणवात – प्राणावृत उदानवात – समानावृत उदानवात – व्यानावृत उदानवात – अपानावृत उदानवात – प्राणावृत समानवात – उदानावृत समानवात – व्यानावृत समानवात – अपानावृत समानवात – प्राणावृत व्यानवात – उदानावृत व्यानवात – समानावृत व्यानवात – अपानावृत व्यानवात – प्राणावृत अपानवात – उदानावृत अपानवात – समानावृत अपानवात – व्यानावृत अपानवात

दोष आवरण	लक्षण
पित्तावृत वात	दाह – तृष्णा – शूल – शीतकामिता
कफावृत वात	शैत्य (शीतता) – गौरव – शूल – उष्णकामिता
पित्तावृत प्राणवात	मूर्च्छा – दाह – भ्रम – शूल – शीतकामिता – छर्दि
कफावृत प्राणवात	ष्ठीवन – क्षवथु – उद्गार – श्वासोच्छ्वाससंग्रह
पित्तावृत उदानवात	मूर्च्छा – दाह – भ्रम – क्लम – साद – ओजोभ्रंश
कफावृत उदानवात	विवर्णता – वाक्-स्वरग्रह – दौर्बल्य – गुरुता – अरुचि
पित्तावृत समानवात	अतिस्वेद – तृष्णा – दाह – मूर्च्छा – अरुचि
कफावृत समानवात	अस्वेद – अग्निमांद्य – लोमहर्ष – अतिशीतता
पित्तावृत व्यानवात	सर्वांगदाह – क्लम – गात्रविक्षेपसंग – सन्ताप
कफावृत व्यानवात	सर्वगात्रगौरव – सर्वसन्ध्यस्थिरुजा – गतिसंग
पित्तावृत अपानवात	हारिद्रमूत्रवर्च – तापगुदमेढ्रयो – अतिरजःस्राव
कफावृत अपानवात	भिन्न आम श्लेष्मसंसृष्ट गुरु वर्चः – कफजप्रमेह
कफपित्तावृत वात	पित्त-कफ मिश्रावरण लक्षण

धातु आवरण	लक्षण
रक्तावृत वात	त्वक् मांसान्तर अतिदाह & पीडा – सराग शोथ – मण्डल
मांसावृत वात	कठिन विवर्ण पिडका – शोथ – हर्ष – पिपीलिकानां संचार इव
मेदसावृत वात	(known as आढ्यवात) चलस्निग्धमृदुशीत शोथ – अरुचि
अस्थ्यावृत वात	उष्णस्पर्श & पीडन अभिनन्दति – अंग भंजन – सूचीभिरिव तोद
मज्जावृत वात	विनाम – जृम्भा – शूल – पाणिभ्यां पीड्यमाने सुखलभते
शुक्रावृत वात	शुक्र-अवेग or अतिवेग – निष्फल शुक्र

अन्न & मलावरण	लक्षण
अन्नावृत वात	भुक्ते उदरशूल & जीर्णे शाम्यति
मलावृत वात (पुरीषावृत)	मलावरोध – स्वे स्थाने परिकृन्तति (cutting type of pain in pakwashaya) – passing of hard stool with difficulty – pain in lower abdomen and lowback
मूत्रावृत वात	मूत्राघात – आध्मान in बस्ति

अन्योन्यावरण	लक्षण	चिकित्सा
प्राणावृत व्यानवात	सर्वेन्द्रियाणां शून्यत्वं – स्मृतिबलक्षय	ऊर्ध्वजत्रुगत चिकित्सा
व्यानावृत प्राणवात	अतिस्वेद – लोमहर्ष – त्वक विकार	स्नेहयुक्त विरेचन
प्राणावृत समानवात	जडता – गद्गद – मूकता	यापनावस्ति & चतुष्प्रयोग of स्नेह
समानावृत अपानवात	ग्रहणीदोष – हृदरोग – पार्श्वपीडा	अग्निदीपक घृत
प्राणावृत उदानवात	शिरोग्रह – निःश्वासोच्छ्वाससंग्रह	ऊर्ध्वजत्रुगत चिकित्सा
उदानावृत प्राणवात	ओज कर्म बलवर्ण नाश – मृत्यु	शीतजलसेचन आश्रासन
उदानावृत अपानवात	छर्दि – श्वास – कास	बस्ति & अनुलोमन

अपानावृत उदानवात	मोह – अग्निमांद्य – अतिसार	दीपन-ग्राही & वमन
व्यानावृत अपानवात	छर्दि – आध्मान – उदावर्त – गुल्म – परिकर्तिका	स्निग्ध & अनुलोमन
अपानावृत व्यानवात	विण्मूत्रेतस अतिप्रवृत्ति	संग्राही-स्तम्भक द्रव्य
समानावृत व्यानवात	मूर्च्छा – तन्द्रा – प्रलाप – अंगसाद – अग्नि ओज बल क्षय	व्यायाम & लघु भोजन
उदानावृत व्यानवात	स्तब्धता – अस्वेद – चेष्टाहानि	पथ्य & लघु भोजन

Remaining 8 types of Anyonyavarana should be understood on the basis of लक्षणस of above avarana.

धातु प्रदोषज विकार

आवरण चिकित्सा →

आवरक दोष or दूष्य should be treated first, then आवृत वात should be treated as per general line of treatment of वातव्याधि.

Though root-cause is वात, the obstruction should be cleared before treating आवृत वात.

- पित्तावृत वात चिकित्सा → शीत-उष्ण व्यत्यासक्रम (alternate) – जीवनीय घृत
- कफावृत वात चिकित्सा → वमन & विरेचन – तीक्ष्ण स्वेद – यव – तिल – सर्षप
- रक्तावृत वात चिकित्सा → वातरक्त चिकित्सा
- आमावृत वात चिकित्सा → प्रमेह चिकित्सा – वात & मेद नाशक चिकित्सा
- मांसावृत वात चिकित्सा → स्वेदन – अभ्यंग – मांस रस – क्षीर – घृत – तैल
- अस्थि मज्जावृत वात चिकित्सा → महास्नेह प्रयोग(घृत-तैल-वसा-मज्जा)
- शुक्रावृत वात चिकित्सा → शुक्रल & बल्य औषध-आहार-विहार
- अन्नावृत वात चिकित्सा → वमन – दीपन & पाचन – लघु आहार
- पुरीषावृत वात चिकित्सा → एरण्ड तैल पान – स्निग्ध द्रव्य – उदावर्त चिकित्सा
- मूत्रावृत वात चिकित्सा → मूत्रल औषध – उत्तरबस्ति – स्वेदन

रसायन प्रयोग in आवरण चिकित्सा :

- शिलाजतु
- गुग्गुलु
- च्यवनप्राश
- ब्रह्म रसायन
- अभयामलकी रसायन

रसधातु प्रदोषज विकार	अश्रद्धा – अरुचि – आस्यवैरस्य – असृजता – हृल्लास – गौरव – तन्द्रा – अंगमर्द – ज्वर – तम – पाण्डु – स्रोतरोध – क्लैब्य – कृशता – अग्निनाश – अकाल वली
रक्तधातु प्रदोषज विकार	कुष्ठ – विसर्प – पिडका – रक्तपित्त – असृग्दर (रक्तप्रदर) – वातरक्त – गुदपाक – मेढ्रपाक – मुखपाक – प्लीहा – गुल्म – विद्रधि – नीलिका – कामला – व्यंग – पिप्लु – तिलकालक – दद्रू – चर्मदल – श्वित्र (किलास) – पामा – कोठ – रक्तमण्डल – न्यच्छ – इन्द्रलुप्त – अर्श – अर्बुद – अंगमर्द
मांसधातु प्रदोषज विकार	अधिमांस – अर्बुद – मांसकीलक – गलशालूक – गलशुण्डी – पूतिमांस – अलजी – गलगण्ड – गण्डमाला – उपजिवहिक्का – मांससंघात – ओष्ठ प्रकोप
मेदोधातु प्रदोषज विकार	अष्ट निन्दित पुरुष – प्रमेह पूर्वरूप (मुखमाधुर्य, हस्तपाद शून्यता व दाह, मुख तालु कण्ठ शोष, पिपासा, आलस्य, मलाधिक्य, मत्स्यगन्धी, मूत्रदोष etc.) – ग्रन्थि – वृद्धिरोग – गलगण्ड – अर्बुद – मेदोज ओष्ठप्रकोप – मधुमेह – अतिस्वेद
अस्थिधातु प्रदोषज विकार	अध्यस्थि – अधिदन्त – दन्तभेद – दन्तशूल – अस्थिभेद – अस्थिशूल – अस्थि व दन्त विवर्णता – केश लोम नख श्मश्रु दोष – कुनख रोग
मज्जाधातु प्रदोषज विकार	रूक् पर्वणां (सन्धिशूल) – भ्रम – मूर्च्छा – तमोदर्शन – नेत्राभिष्यन्द
शुक्रधातु प्रदोषज विकार	क्लैब्य – अहर्षण – शुक्राशमरी – शुक्रमेह – शुक्रदोष – निष्फल शुक्र (shukra dhatu incapable to produce गर्भ, if conception occurs it leads to गर्भपात or birth of अल्पायु and / or विरूप सन्तान)

धातु प्रदोष and साम धातु are having similar लक्षणाs. But treatment differs for साम and निरामावस्था.

धातुप्रदोष चिकित्सा →

रसधातु प्रदोष चिकित्सा → लंघन (उपवास)

रक्त प्रदोष चिकित्सा → रक्तपित्तशामक चिकित्सा – विरेचन – रक्तमोक्षण

मांस प्रदोष चिकित्सा → संशोधन (पंचकर्म) – शस्त्रकर्म – क्षारकर्म – अग्निकर्म

मेदो प्रदोष चिकित्सा → अपतर्पण or कर्षण – स्थौल्यहर & कफ-मेदोहर चिकित्सा

अस्थि प्रदोष चिकित्सा → पंचकर्म – तिक्त क्षीर बस्ति – तिक्त घृत प्रयोग

मज्जा प्रदोष चिकित्सा → मधुर तिक्त द्रव्य – व्यवाय – व्यायाम – यथाकाल संशोधन

शुक्र प्रदोष चिकित्सा → शुक्रल द्रव्य प्रयोग – शोधन – वाजीकरण

सामान्यज & नानात्मज विकार

सामान्यज विकार →

सामान्यज विकाराऽ are those diseases which are produced by vitiated doshas either individually or in combinations of two or three. They may be एकदोषज (वातज /पित्तज /कफज), द्विदोषज (संसर्गज), or त्रिदोषज (सन्निपातज).

Examples of Samanyaja vikaras –

8 types of उदर, मूत्रकृच्छ्र, क्षीरदोष, शुक्रदोष; 7 types of कुष्ठ, प्रमेह पिडिका, विसर्प; 6 types of अतिसार, उदावर्त, गुल्म, प्लीहा, कास, श्वास, हिक्का, तृष्णा, छर्दि, अरुचि, शिरोरोग, हृदरोग, पाण्डुरोग, उन्माद; 4 types of नेत्ररोग, कर्णरोग, प्रतिश्याय, मुखरोग, ग्रहणी रोग, मद, मूर्च्छा, शोष, क्लैब्य; 3 types of शोथ, किलास, रक्तपित्त; 2 types of ज्वर, व्रण, आयाम, गृध्रसि, कामला, आम, वातरक्त, अर्श; 1 type of उरुस्तम्भ, सन्यास, महागद; 20 types of कृमिरोग, प्रमेह, योनिव्यापत.

नानात्मज विकार →

“न + अन् + आत्मज” The diseases which cannot be produced without involvement of the प्रधान दोष (predominant dosha), i.e only one dosha is responsible for such disease, whereas other doshas may be involved as अप्रधान दोष (associated doshas).

For examples –

वातव्याधि is produced by वात दोष, in which पित्त or कफ may or may not be involved as associated doshas, and the वातव्याधि cannot be produced without involvement of वात; hence it is a नानात्मज विकार.

वातज नानात्मज विकार → 80

पित्तज नानात्मज विकार → 40

कफज नानात्मज विकार → 20

वातज नानात्मज विकार → 80

Examples – अर्दित, एकांगवात, सर्वांगवात, पक्षावध (पक्षाघात), आक्षेपक, दण्डक, जानुभेद, गृध्रसि, विपादिका, पादशूल, खंज, कुब्ज, वामनत्व, पृष्ठग्रह, ग्रीवास्तम्भ, मन्यास्तम्भ, हनुभेद, ओष्ठभेद, अक्षिभेद, दन्तभेद, कर्णशूल, शिरोरुक् (शिरःशूल), बाधिर्य, मुखशोष, गुदभ्रंश, वेपथु, जृम्भा, हिक्का, रौक्ष्य, पारुष्य etc.

पित्तज नानात्मज विकार → 40

Examples – ओष, दाह, धूमक, आम्लक, अन्तर्दाह, त्वग्दाह, अंसदाह, ऊष्माधिक्य, अतिस्वेद, रक्तकोठ, रक्तविस्फोट, रक्तपित्त, रक्तमण्डल, कामला, तित्तास्यता, पूतिमुखता, तृष्णाधिक्य, अतृप्ति, मुखपाक, गलपाक, अक्षिपाक, गुदपाक, मेढ्रपाक, जीवादान, तमःप्रवेश, हारित-हारिद्र नेत्र मूत्र वर्च etc.

कफज नानात्मज विकार → 20

Examples – तृप्ति (अनन्नाभिलाषा), तन्द्रा, निद्राधिक्य, गुरुगात्रता, आलस्य, मुखमाधुर्य, मलाधिक्य, अविपाक, हृदयोपलेप, कण्ठोपलेप, गलगण्ड, अतिस्थौल्य, शीताग्नि (मन्दाग्नि), उदरद, श्वेतावभासता (श्वेतवर्ण त्वक नेत्र) etc.

सामान्यज & नानात्मज विकार चिकित्सा →

- These diseases are produced by imbalance or vitiation of doshas (दोष विषमता), therefore they can be treated by balancing the dosha (i.e. bringing about दोष साम्यता).
- निदान परिवर्जन
- बृंहण of क्षीण दोष, शमन of कुपित दोष, निर्हरण (शोधन) of वृद्ध दोष, and परिपालन of सम दोष are the general line of treatments for bringing about equilibrium state of dosha (दोष साम्यता). – सु.चि.33/3
- Treatment is given with consideration of रोगी-रोग बल & दोष बलाबल.
- दोषोपक्रम – For नानात्मज विकार, specific treatments for respective doshas should be adopted. (Detailed on page. no. 22)

Importance of दोष, दूष्य, देश, काल, बल, अग्नि, प्रकृति, वय, सत्त्व, सात्म्य, आहार and व्याधि अवस्था in चिकित्सा

A wise physician always should consider the following factors carefully before initiating any kind of therapy →

दोष → The शारीरिक दोष (वात-पित्त-कफ) & मानसिक दोष (सत्त्व-रज-तम), in normal state, play important role in normal physiological and psychological functions of the body. But when they get aggravated or vitiated, pathological changes occur in the body those lead to manifestation of various diseases. So health and disease depend upon state of दोष साम्यता & विषमता, and therefore physician should examine the शारीरिक & मानसिक दोष carefully before initiating any treatment.

Always treatment should be planned with consideration of दोष वृद्धि-क्षय-सम अवस्था, ऊर्ध्वगति, अधःगति, तिर्यकगति, कोष्ठगति, शाखागति, मर्मास्थिसन्धिगति, स्वस्थान प्रकोप, स्थानान्तरगत प्रकोप, स्वतन्त्र प्रकोप, परतन्त्र प्रकोप, आवरण, आम, द्वन्द्वज, सन्निपातज, दोष बलाबल, धातुगत, मलगत, संचय-प्रकोप-प्रसर-स्थानसंश्रय-व्यक्त – भेदावास्था, अंशांश कल्पना etc.

Disease is produced due to दोष-दूष्य सम्मूर्च्छन and the pathogenesis of disease (सम्प्राप्ति) includes निदान, दोष, दूष्य, स्रोतोदुष्टि, स्थानसंश्रय etc. Hence the general line of treatment for every disease is निदान परिवर्जन & treatment of दोष-दूष्य for सम्प्राप्ति विघटन.

दूष्य → Consideration of involved धातु is also equally important as of दोष, during treatment of any disease. Vitiated धातु & मल are called दूष्य. They are सम्प्राप्ति घटक, so they must be treated for सम्प्राप्ति विघटन. If दूष्य is ignored, it can result in unsuccessful treatment or relapse of the disease.

देश → The term 'देश' denote two meanings –(i) भूमि & (ii) आतुर.

(i) भूमि परीक्षा → To examine the place where the patient is born, brought about, or currently living, and most importantly the place where he got the disease. All these places should be examined for their climate, soil, common ahara-vihara etc. There are three types of भूमि →

1. जांगल देश → रुक्ष & वातप्रधान
2. आनूप देश → क्लेद & कफप्रधान
3. साधारण देश → सामान्य वातावरण, त्रिदोष सम

(ii) आतुर परीक्षा → The area where treatment is applied, is the body of patient. This body should be thoroughly examined to determine प्रमाण of आयु, दोष बल, and रोगी-रोग बल. Diagnosis, selection of treatment measures, selection of drugs and dosage of drugs depend upon रोगी-रोग बल, दोष बल, आयु etc. Also these factors are used to assess साध्यासाध्यता (prognosis of disease).

दशविध आतुर परीक्षा includes प्रकृति, विकृति, सार, संहनन, सात्म्य, सत्त्व, प्रमाण, आहार शक्ति, व्यायाम शक्ति and वय.

Patient's body can also be examined with अष्टस्थान परीक्षा → नाडी, मूत्र, मल, जिह्वा, शब्द, स्पर्श, दृक् and आकृति.

काल → There are two types of काल →

(i) नित्यग काल – क्षण, मुहुर्त्त, seconds, minutes, hours, दिन (days), रात्रि (night), पक्ष (fortnight), मास (months), ऋतु (seasons), अयन (उत्तरान & दक्षिणायन), संवत्सर (year), युग (era) etc. Physician has to consider काल for proper diagnosis, treatment and dose of drugs (औषध सेवन काल) etc.

(ii) आवस्थिक काल – It is either वयानुसार or व्याधि अवस्थानुसार – e.g. diseases are generally सुखसाध्य in युवावस्था, and कष्टसाध्य in वृद्धावस्था. And the treatment defers for different व्याधि अवस्था.

बल → Consideration of रोगी बल, रोग बल, दोष बल and अग्नि बल are having utmost importance in diagnosis and treatment, especially for selection of therapeutic procedures like shodhana & shaman, selection of drugs, and dose & duration of drug-therapy.

- रोगी बल of सहज (innate), कालज (seasonal) and युक्तिकृत (acquired) may be प्रवर (strong), मध्यम (medium), and अवर or अल्प (weak).
- रोग बल may also be प्रवर (strong), मध्यम (medium), and अवर (weak).
- दोष may be involved in disease with बहुदोष (excessive doshas) मध्यमदोष (moderate) and अल्पदोष (less) प्रकोप. They may be causing the disease individually (एकदोषज), in combination of two (संसर्गज), or in combination of all the three (सन्निपातज) with variable दोष बलाबल that is denoted by तर - तम.
- अग्नि बल may be प्रवर, मध्यम or अवर.

अग्नि → Consideration of अग्नि बल and अग्नि अवस्था such as assessment तीक्ष्णाग्नि, समान्नि, मन्दान्नि, and विषमान्नि help in diagnosis and treatment. The importance of अग्नि can be understood by following quotations →

कायचिकित्सा इति कायस्य अन्तरग्रेश्चिकित्सा । (गंगाधर)

रोगाः सर्वेऽपि मन्देऽग्नौ . . . । (अ.ह.नि.12/1)

प्रकृति → The constitution of patient is examined for proper diagnosis and initiation of proper treatment. Selection of drugs and treatment measures also depend upon the प्रकृति of the patient. प्रकृति may be वातज, पित्तज, कफज, द्वन्द्वज, or सन्निपातज.

वय → Many diseases are age-related, e.g., वार्धक्यजन्य विकाराः are produced due to degenerative changes in old age. Consideration of बाल्यावस्था, किशोरावस्था, युवावस्था, प्रौढ, वृद्ध, and जरावस्था is important in treatment for diagnosis, selection of drugs and prognosis etc.

सत्त्व → The mental strength or will power of the patient should be examined for assessment of severity and prognosis of disease, such as गुरुव्याधित रोगी appears to be having mild disease even if he has severe disease; but in case of लघुव्याधित, he appears to be having severe disease while actually having mild disease. सत्त्व may be प्रवर, मध्यम or अवर. A good mental strength indicates good prognosis.

सात्म्य → Acceptability of the body and mind of patient, or the स्वप्रकृति अनुकूल (suitable) पदार्थ (substances), रस (tastes) or जलवायु (climates) are called सात्म्य. For example, patient may be एकरस सात्म्य, सर्वरस सात्म्य, स्निग्ध or रूक्ष सात्म्य etc.

सात्म्य should be examined as देह सात्म्य, प्रकृति सात्म्य, ऋतु सात्म्य, देश सात्म्य, ओक सात्म्य, जाति सात्म्य, काल सात्म्य, बल सात्म्य etc. It is useful in the treatment for choosing the line of treatment and deciding dosage and duration of drug therapy.

आहार → आहार is having various types such as अशित, पीत, लीढ, खादित, शाकाहारी, मांसाहारी, विरुद्धाहार, अनशन, अध्यशन, विषमाशन, विरुद्धाशन, लघु, गुरु, शीत, उष्ण, प्रकृति, करण, सन्योग, राशि, देश, काल, उपयोगसंस्था, उपयोक्ता etc. All these types of ahara should be considered before initiating treatment.

व्याधि अवस्था → Assessment of the stages of a disease is essential for successful treatment. The knowledge of व्याधि अवस्था is useful in determining the strength of disease and deciding the line of treatment. व्याधि अवस्था may be नव or जीर्ण, and based on षट् क्रियाकाल it may be संचयावस्था, प्रकोपावस्था, प्रसरावस्था, स्थानसंश्रयावस्था, व्यक्तावस्था and भेदावस्था. The treatment measures for all these stages are different and those must be applied with proper assessment of the stage of disease.

द्विविधोपक्रम

उपक्रमस्य हि द्वित्वादिद्विधैवोपक्रमो मतः ।

एकः सन्तर्पणस्तत द्वितीयश्चापतर्पणः ॥ (अ.ह.सू.14/1)

Acharya Vagbhata has mentioned two types of therapies →

1. सन्तर्पण → Tonifying or nourishing therapy (बृंहण चिकित्सा)
2. अपतर्पण → Making the body light or reducing therapy (लंघन चिकित्सा)

सन्तर्पण means nourishment, and अपतर्पण means under nourishment.

Indications of सन्तर्पण चिकित्सा are अपतर्पणजन्य व्याधि such as क्षय, शोष, वातव्याधि etc., whereas indications of अपतर्पण are आमदोषजन्य & सन्तर्पणजन्य व्याधि such as स्थौल्य, मेदोविकार, कुष्ठ, प्रमेह etc.

द्विविध उपक्रम also includes six types of therapies (षडविध उपक्रम) →

लंघन रूक्षण स्वेदन → अपतर्पण

बृंहण स्नेहन स्तम्भन → सन्तर्पण

अपतर्पण उपक्रम →

This therapy includes लंघन चिकित्सा, which is indicated for आमदोषजन्य &

सन्तर्पणजन्य रोग. अपतर्पण or लंघन चिकित्सा is classified as follows →

→ 3 types of अपतर्पण → (i) लंघन, (ii) लंघन-पाचन, (iii) दोषावसेचन

→ 2 types of लंघन चिकित्सा → (i) शोधन चिकित्सा, (ii) शमन चिकित्सा

→ 10 types of लंघन चिकित्सा → (i) वमन

(ii) विरेचन

(iii) निरूह बस्ति

(iv) शिरोविरेचन (नस्य)

(v) पाचन

(vi) उपवास

(vii) पिपासा

(viii) आतप सेवन

(ix) मारुत सेवन

(x) व्यायाम

सन्तर्पण उपक्रम →

This therapy includes बृंहण चिकित्सा that is treatment with धातुपुष्टिकर & बृंहण औषध-आहार-विहार. It is indicated for अपतर्पणजन्य रोग.

सन्तर्पण or बृंहण चिकित्सा is classified as follows →

1. सद्य सन्तर्पण → Administration of therapies (चिकित्सा) and drugs (द्रव्य), those provide nourishment to the शरीर (body) and धातु (tissues) immediately, is known as सद्य सन्तर्पण.

For example – बृंहण बस्ति प्रयोग, बृंहण द्रव्य प्रयोग (द्राक्षा, खर्जूर, दाडिम, परूषक, वृक्षाम्ल etc.)

2. अभ्यास सन्तर्पण → Nourishing therapy that includes बृंहण द्रव्य प्रयोग and धातुपुष्टिकर आहार-विहार सेवन, continuously for longer duration, is known as अभ्यास सन्तर्पण.

For example – दुग्ध & घृत नित्यसेवन, मांसरस, सक्तू, मधुर अम्ल शीत स्निग्ध मृदु गुण युक्त बृंहण आहार सेवन, अभ्यंग, अनुवासन बस्ति etc.

[Note → Detailed explanation of लंघन and बृंहण, under the heading of षडविधोपक्रम can also be used to describe द्विविधोपक्रम, if द्विविधोपक्रम is asked for 15 marks.]

षड्विधोपक्रम

षडविध-उपक्रम (six types of therapies) are included in द्विविध-उपक्रम (two types of therapies – सन्तर्पण & अपतर्पण). अपतर्पण is used in स्थूल रोगी & साम रोग (सन्तर्पण जन्य रोग), whereas सन्तर्पण is used in कृशरोगी & निराम रोग (अपतर्पण जन्य रोग).

लंघनं बृंहणं काले रूक्षणं स्नेहनं तथा ।

स्वेदनं स्तम्भनं जानीते यः स वै भिषक् ॥ (च.सू.22/4)

According to Acharya Charaka, those who have the knowledge about all six types of therapies (षडविधोपक्रम) are called भिषक or वैद्य (physicians).

These therapies are लंघन, बृंहण, रूक्षण, स्नेहन, स्वेदन and स्तम्भन.

1. लंघन →

यत्किञ्चित् लाघवकरं देहे तल्लंघनं स्मृतम् ।

Reducing therapy – that which brings lightness in the body.

लंघन द्रव्य →

लघूष्णतीक्ष्णविशदं रूक्षं सूक्ष्मं खरं सरम् ।

कठिनं चैव यद्द्रव्यं प्रायस्तल्लंघनं स्मृतम् ॥ (च.सू.22/12)

लंघन द्रव्य are लघु (light), उष्ण(hot in potency), तीक्ष्ण (sharp), विशद (non slimy), रूक्ष (dry) सूक्ष्म (minute) खर (rough) सर (mobile) and कठिन (hard) in nature. For example – यव, मुद्ग, त्रिफला, मरिच, पिप्पली, विडंग, मेथिका etc.

लंघन भेद →**1. दश भेद →**

1. वमन – Emesis
2. विरेचन – Purgation
3. निरूह बस्ति – Decoction enema
4. नस्य (शिरोविरेचन) – Errhines
5. पाचन – Use of digestants
6. उपवास – Fasting
7. पिपासा – Thirst
8. आतपसेवन – Exposure to sun rays
9. मारुतसेवन – Exposure to fresh air
10. व्यायाम – Exercise

2. द्विभेद →

1. शोधन – Purificatory procedures
(वमन, विरेचन, निरूह बस्ति, शिरोविरेचन, and रक्तमोक्षण)
2. शमन – Palliative or pacifying procedures
(पाचन, दीपन, क्षुधा, तृष्णा, व्यायाम, आतप and मारुत सेवन)

Indications of लंघन → आमदोष जन्य विकार, सन्तर्पण जन्य रोग, स्थौल्य, प्रमेह, कुष्ठ, उरुस्तम्भ, अलसक, अतिस्लिग्ध पुरुष, रसधातुगत विकार, मेदो विकार, शिशिर ऋतु etc.

Contraindications of लंघन → वात वृद्धि, अपतर्पण जन्य व्याधि, कृश, रूक्ष पुरुष etc.

सम्यक् लंघन लक्षण → (i) शरीर लघुता

- (ii) आमपाचन & अग्निदीपन
- (iii) हृदय, कण्ठ & मुख शुद्धि
- (iv) सम्यक् मल मूत्र त्याग
- (v) सन्तर्पणोत्थ रोग प्रशमन

अतिलंघन लक्षण → (i) अंगमर्द, पर्वभेद, कास, श्वास, हिक्का, जृम्भा उत्पत्ति

- (ii) क्षुधानाश, देह & अग्नि बल नाश
- (iii) अरुचि
- (iv) तृष्णाधिक्य
- (v) चित्तविभ्रंश

अतिलंघन चिकित्सा → शर्करा, मांसरस, क्षीर, घृत आदि बृंहणीय द्रव्य प्रयोग.

2. बृंहण →

बृहत्त्वं यच्छरीरस्य जनयेत् तच्च बृंहणम् ।

Nourishing therapy – increases the musculature or bulk of body.

बृंहण द्रव्य →

गुरु शीतं मृदु स्निग्धं बहलं स्थूलपिच्छिलम् ।

प्रायो मन्दं स्थिरं श्लक्ष्णं द्रव्यं बृंहणमुच्यते ॥ (च.सू.22/13)

बृंहण द्रव्य are गुरु (heavy), शीत (cold), मृदु (soft), स्निग्ध (unctuous), बहल (thick), स्थूल (bulky), पिच्छिल (slimy), मन्द (sluggish), स्थिर (stable) and श्लक्ष्ण (smooth) in nature. For example – मधुर शीत स्निग्ध आहार, दुग्ध, घृत, शर्करा, मांसरस, द्राक्षा, मृद्विका, खर्जूर, दाडिम, परूषक, वृक्षाम्ल, आमलकी, शतावरी, बला, अश्वगन्धा, विदारीकन्द, अनुवासन बस्ति, अभ्यंग, उत्सादन, स्नान, निद्रा सेवन etc.

Indications of बृंहण → अपतर्पण जन्य रोग, वातव्याधि, क्षत, क्षीण, कृश, वृद्ध, दुर्बल पुरुष, स्त्री-मद्य नित्यसेवी, ग्रीष्मकाल etc.

Contraindications of बृंहण → सन्तर्पण जन्य रोग, आमदोष, अतिस्लिग्ध, अतिस्थूल पुरुष, उरुस्तम्भ, मेदोवृद्धि, कफवृद्धि etc.

सम्यक बृंहण लक्षण → (i) शरीर & धातु पुष्टि

(ii) बलवृद्धि

(iii) कृशता दोष नाश

(iv) अपतर्पणोत्थ रोग प्रशमन

अति बृंहण बृंहण → (i) शरीर स्थूलता

(ii) स्थौल्य अष्टदोष उत्पत्ति

(आयुहास – ज्वोपरोध – कृच्छ्रव्यवाय – दौर्बल्य

– दौर्गन्ध्य – अतिस्वेद – अतिक्षुधा – अतितृष्णा)

(iii) सन्तर्पणोत्थ रोग उत्पत्ति (प्रमेह-कुष्ठ-ज्वर-उदर etc.)

अतिबृंहण चिकित्सा → लघन, रूक्षण, कफहर & मेदोहर द्रव्य प्रयोग etc.

3. रूक्षण →

रौक्ष्यं खरत्वं वैशद्य यत्कुर्याद तद्विरूक्षणम् ।

Drying therapy – brings dryness, roughness and non-sliminess.

रूक्षण द्रव्य →

रूक्षं लघु खरं तीक्ष्णमुष्णं स्थिरमपिच्छिलम् ।

प्रायशः कठिनं चैव यद्द्रव्यं तद्विरूक्षणम् ॥ (च.सू.22/14)

रूक्षण द्रव्य are रूक्ष (dry), लघु (light), खर (rough), तीक्ष्ण (sharp), उष्ण (hot), स्थिर (stable), अपिच्छिल (non-slimy), and कठिन (hard) in nature. For example – कटु-तिक्त-कषाय रस प्रधान द्रव्य सेवन, स्त्री असंयम, सर्षप खली, तिलखली, तक्र, मधु, त्रिकटु, त्रिफला etc.

Indications of रूक्षण → अभिष्यन्द जन्य रोग (स्रोतोरोध), महादोष (दोष मात्राधिक्य), मर्मस्थान गत रोग, उरुस्तम्भ etc.

सम्यक रूक्षण लक्षण → सम्यक लघन लक्षण

अतिरूक्षण लक्षण → अति लघन लक्षण

अतिरूक्षण चिकित्सा → स्नेहन चिकित्सा, घृत, तैल, वसा, मज्जा, पंचप्रासृतिकी पेया, तिल-काम्बलिक आदि प्रयोग.

4. स्नेहन →

स्नेहनं स्नेह विष्यन्द मार्दवं क्लेद कारकम् ।

Oleation therapy – brings unctuousness, sliminess, softness & moisture.

स्नेहन द्रव्य →

द्रवं सूक्ष्मं सरं स्निग्धं पिच्छिलं गुरु शीतलम् ।

प्रायो मन्दं मृदु च यद् द्रव्यं तत् स्नेहनं मतम् ॥ (च.सू.22/15)

स्नेहन द्रव्य are द्रव (liquid), सूक्ष्म (minute), सर (mobile), स्निग्ध (unctuous), पिच्छिल (slimy), गुरु (heavy), शीतल (cold), मन्द (sluggish), and मृदु (soft) in nature. For example – महास्नेह (घृत-तैल-वसा-मज्जा); तिल, एरण्ड, सर्षप, निम्ब, करंज आदि स्थावर स्नेह; दधि, दुग्ध, घृत, मांस, वसा, मज्जा आदि जांगल स्नेह.

स्नेहन भेद →

There are various classifications of स्नेहन कर्म →

चतुर्विध स्नेह →

(1) घृत – Ghee

(2) तैल – Oil

(3) वसा – Muscle fat

(4) मज्जा – Bone marrow

Based on स्नेह योनि → (1) स्थावर स्नेह – Plant source

(2) जांगम स्नेह – Animal source

Based on स्नेह मात्रा → (1) हृसीयसी मात्रा – get digested in 1 याम (3 hrs)

(2) ह्रस्व मात्रा – get digested in 2 याम (6 hrs)

(2) मध्यम मात्रा – get digested in 4 याम (12 hrs)

(3) उत्तम मात्रा – get digested in 8 याम (24 hrs)

Based on स्नेह पाक → (1) मृदु पाक – Consistency of paste

(2) मध्यम पाक – Jelly consistency that can be poured out easily with spoon

(3) खर पाक – Thicker consistency that can be rolled between the fingers

Based on स्नेहन प्रयोग → (1) बाह्य स्नेहन – External application of Sneha

(2) आभ्यन्तर स्नेहन – Internal administration of Sneha

→ बाह्य स्नेह is of various types → such as – अभ्यंग, उद्वर्तन, लेप, मर्दन, उन्मर्दन, पादघात, परिषेक, गण्डूष, मूर्ध तैल (शिरो बस्ति-पिचू-सेक-अभ्यंग), कर्ण तर्पण, नासा तर्पण, अक्षि तर्पण, अवगाह etc.

→ आभ्यन्तर स्नेह (स्नेहपान) is of mainly three types →

1. शोधनांग स्नेहपान

-Used as पूर्वकर्म of शोधन चिकित्सा that eliminate the aggravated dosha

-Administered in the morning before the time of breakfast

2. शमनांग स्नेहपान

-Used for pacification (शमन) of the aggravated dosha

-Administered in the morning at the time of breakfast

3. बृंहणांग स्नेहपान

-Used for nourishment of the body (बृंहण)

-Can be administered before food, with food, or after food. Generally it is administered by mixing with food.

Other types based on स्नेह-योग →

(I) (1) अच्चेपेय – Sneha without mixing (केवल स्नेह)

(2) प्रविचारण – Sneha along with various preparations

प्रविचारण स्नेह is internal administration of sneha mixed with other food articles, and also external application of any sneha with or without mixing. It is of 24 types → ओदन, विलेपी, रस, मांस, पय, दधि, यवागु, सूप, शाक, यूप, काम्बलिक, खड, सक्तु, तिलपिष्ट, मद्य, लेह्य, भक्ष्य, अभ्यंजन, बस्ति, उत्तरबस्ति, गण्डूष, कर्णतैल, नस्य, and अक्षितर्पण.

(II) (1) यमक स्नेह – Combination of any two of चतुर्विध स्नेह

(2) त्रिवृत स्नेह – Combination of any three of चतुर्विध स्नेह

(3) महास्नेह – Combination of all the four चतुर्विध स्नेह

Indications of स्नेहन → स्वेद्याः शोधयितव्याश्च रुक्षा वातविकारिणः।

व्यायाममद्यस्त्रीनित्याः स्नेहाः स्युर्ये च चिन्तका ॥ (च.सू.13/52)

The person who is about to undergo sudation therapy (स्वेदन) or purificatory therapies (शोधन), dry (रुक्ष), suffering from वातव्याधि, and tired from regular exercise (व्यायाम), alcohol consumption (मद्यसेवन), sexual intercourse (स्त्रीसेवन), or excessive worry (चिन्ता).

स्नेहन कर्म is administered as पूर्वकर्म for the शोधन चिकित्सा (पंचकर्म), as प्रधानकर्म for the शमन चिकित्सा (best for वातव्याधि), and also for बृंहण चिकित्सा.

Contraindications of स्नेहन → आमदोष, नवज्वर, अरुचि, मन्दाग्नि, अतिस्थूल, तृष्णा, मूर्च्छा, हृदि, श्रम, क्लम, ग्लानि, दुर्बल, गर्भिणी, बस्तिकर्म पश्चात्, नस्यकर्म पश्चात् etc.

सम्यक् स्निग्ध लक्षण → वातानुलोम्यं दीप्तोऽग्निर्वर्चः स्निग्धमसंहतम्।

मार्दवं स्निग्धता चांगे स्निग्धानामुपजायते ॥ (च.सू.13/58)

→ वातानुलोमन – proper movement of vata (or flatus)

→ अग्निदीपन – improvement of digestion

→ स्निग्ध & असंहत वर्च – unctuous & loose stool

→ मार्दवता – softness of the body

→ स्निग्धता – unctuousness of the body

अतिस्निग्ध लक्षण → पाण्डुता गौरवं जाड्य पुरीषस्याविपक्वता।

तन्द्रारुचिरुत्क्लेशः स्याद् अतिस्निग्धलक्षणम् ॥ (च.सू.13/59)

→ पाण्डुता – pallority in the body

→ गौरवता – heaviness in the body

→ जाड्यता – dullness in the body

→ पुरीषस्य अविपक्वता – unformed stool

→ तन्द्रा – drowsiness

→ अरुचि – anorexia

→ उत्क्लेश – nausea

अतिस्निग्ध चिकित्सा → लघन & रुक्षण चिकित्सा.

5. स्वेदन →

स्तम्भगौरव शीतघ्नं स्वेदनं स्वेद कारकम् ।

Sudation – removes stiffness heaviness coldness, and induces sweating.

स्वेदन द्रव्य →

उष्णं तीक्ष्णं सरं स्निग्धं रूक्षं सूक्ष्मं द्रवं स्थिरम् ।

द्रव्यं गुरु च यत् प्रायस्तद्धि स्वेदमुच्यते ॥ (च.सू.22/16)

स्वेदन द्रव्य are उष्ण (hot), तीक्ष्ण (sharp), सर (mobile), स्निग्ध (unctuous) or रूक्ष (dry), सूक्ष्म (minute), द्रव (liquid), स्थिर (stable), and गुरु (heavy) in nature.

For example – शोभांजन, एरण्ड, अर्क, पुनर्नवा, यव, तिल, कुलत्थ, माष, बदर etc.

स्वेदन भेद →

There are various classifications of स्वेदन कर्म →

According to चरक → (1) एकांग स्वेद – Svedana to one part of the body

(2) सर्वांग स्वेद – Svedana all over the body

(1) स्निग्ध स्वेद – e.g. षष्टिकशालि पिण्ड स्वेद etc.

(2) रूक्ष स्वेद – e.g. वालुका स्वेद etc.

(1) अग्नि (साग्नि) स्वेद – Direct contact with अग्नि (fire)

(2) निरग्नि (अनग्नि) स्वेद – No direct contact with अग्नि

साग्नि स्वेद (13 types) → संकर प्रस्तरो नाडी परिषेकोऽवगाहनम् ।

जेन्ताकोऽश्मघनः कर्पूः कुटी भूः कुम्भिकैव च ॥

कूपो होलाक इत्येते स्वेदयन्ति त्रयोदश । (च.सू.14/39)

1. संकर स्वेद – Pinda Sveda (bolus) with or without Pottali
2. प्रस्तर स्वेद – Svedana with Stone Slab
3. नाडी स्वेद – Svedana with vapours through a pipe (Nadi)
4. परिषेक स्वेद – Pouring of warm oil or liquid over the body or part
5. अवगाह स्वेद – Hot Tub Bath
6. जेन्ताक स्वेद – Sudatorium (a special room for sveda)

7. अश्मघन स्वेद – Preheated slab of stone

8. कर्पू स्वेद – Trench (pit) containing a large space inside & small opening

9. कुटी स्वेद – Svedana in the Round shaped Hut

10. भू स्वेद – Svedana on the ground

11. कुम्भिक स्वेद – Svedana with the help of Kumbhi (pitcher)

12. कूप स्वेद – Svedana with the help of Kupa (a well-like pit)

13. होलाक स्वेद – A bed is prepared over the burnt, smokeless & hot dung

निरग्नि स्वेद (10 types) → व्यायाम उष्णसदनं गुरुप्रावरणं क्षुधा ।

बहुपानं भय क्रोध उपनाहाह्वातपाः ॥ (च.सू.14/64)

1. व्यायाम – Exercise
2. उष्णसदन – Warm room
3. गुरुप्रावरण – Covering with thick Blankets
4. क्षुधा – Hunger
5. बहुपान – Excessive alcohol
6. भय – Fear
7. क्रोध – Anger
8. उपनाह – Poultice
9. आहव – Wrestling
10. आतप – Sun bath

According to सुश्रुत → चतुर्विधः स्वेदः, तद्यथा – तापस्वेदः, उष्मस्वेदः,

उपनाहस्वेदो, द्रवस्वेद इति । (सु.चि.32/3)

- (1) ताप स्वेद – By patting the body with heated cloth, metal plate, palm of the hand, sand etc.
- (2) उष्म स्वेद – Steam of boiling liquids or the heat of the hot leaves, stone, cow-dung etc.
- (3) उपनाह स्वेद – By poultice or bolus (Pottali)
- (4) द्रव स्वेद – Pouring hot liquids

Indications of स्वेदन → प्रतिश्याये च कासे च हिक्का श्वासेषु लाघवे ।
कर्णं मन्या शिरःशूले स्वरभेदे गलग्रहे ॥
अर्दितैर् एकांग सर्वांग पक्षाघाते विनामिके ।
कोष्ठानाह विबन्धेषु मूत्राघाते विजृम्भके ॥
पार्श्वं पृष्ठं कटि कुक्षिसंग्रहे गृध्रसीषु च ।
मूत्रकृच्छ्रे महात्वे च मूष्कयोर् अंगमर्दके ॥
पाद जानू उरु जंघार्ति संग्रहे श्रयथावपि ।
खल्लीषु आमेषु शीते च वेपथौ वातकण्ठके ॥
संकोच आयाम शूलेषु स्तम्भ गौरव सुप्तिषु ।
सर्वाङ्गेषु विकारेषु स्वेदनं हितमुच्यते ॥ (च.सू.14/20-24)

प्रतिश्याय – कास – हिक्का – श्वास – कर्ण मन्या शिरःशूल – स्वरभेद – गलग्रह – अर्दित
एकाङ्गवात – सर्वाङ्गवात – पक्षाघात – विनामिका – कोष्ठ आनाह – विबन्ध – मूत्राघात –
पार्श्व पृष्ठ कटि कुक्षिग्रह – मूत्रकृच्छ्र – पाद उरु जानु जंघा ग्रह – शोथ – आमदोष – वेपथु
(कम्पवात) – वातकण्ठक – संकोच आयाम शूल – स्तम्भ गौरव सुप्तता – अर्बुद रोग – ग्रंथि
रोग – शुक्राघात – आढ्यवात (उरुस्तम्भ) etc.

Contraindications of स्वेदन → कषाय & मद्य नित्य सेवी – गर्भिणी – रक्तपित्त –
पित्तज रोग & पित्तज प्रकृति – अतिसार – मधुमेह – गुदभ्रंश – गुदपाक – नष्टसंज्ञा –
अतिस्थूल – क्षुधा – तृष्णा – पाण्डु – कामला – उदर – उरःक्षत – क्षय (राजयक्ष्मा) –
अजीर्ण – विष – वातरक्त – तिमिर – क्रोध – शोक etc.

सम्यक् स्विन्न लक्षण → शीतशूल व्युपरमे स्तम्भगौरव निग्रहे ।
संजाते मार्दवे स्वेदे स्वेदनातद्विरतिर्माता ॥ (च.सू.14/13)
→ शीत उपरम – Reduction of cold
→ शूल उपरम – Reduction of pain
→ स्तम्भ निग्रह – Relief from stiffness
→ गौरव निग्रह – Relief from heaviness
→ मार्दवता – Softness of the body
→ स्वेद प्रवृत्ति – Perspiration (sweating)

अतिस्विन्न लक्षण → पित्तप्रकोपो मूर्च्छा च शरीरसदनं तृषा ।
दाहः स्वरङ्गदौर्बल्यमतिस्विन्नस्यलक्षणम् ॥ (च.सू.14/13)
→ पित्त प्रकोप – Aggravation of pitta dosha
→ मूर्च्छा – Unconsciousness
→ शरीर सदन – Lassitude
→ तृष्णा – Excess thirst (polydipsia)
→ दाह – Burning sensation
→ स्वर दौर्बल्य – Weakness of voice
→ अङ्ग दौर्बल्य – Weakness of the body parts

अतिस्विन्न चिकित्सा → मधुर-स्निग्ध-शीत आहार विहार, ग्रीष्म ऋतुचर्या पालन, and
स्तम्भन चिकित्सा should be used to treat excess sudation.

6. स्तम्भन →

स्तम्भनं स्तम्भयति यद् गतिमन्तं चलं ध्रुवम् ।

Astringent therapy – that which stops the flow of doshas.

स्तम्भन द्रव्य →

शीतं मन्दं मृदु क्षुब्धं रूक्षं सूक्ष्मं द्रवं स्थिरम् ।

यद् द्रव्यं लघु चोद्दिष्टं प्रायस्तत् स्तम्भनं स्मृतम् ॥ (च.सू.22/17)

स्तम्भन द्रव्य are शीत (cold), मन्द (slow / sluggish), मृदु (soft), क्षुब्ध (smooth),
रूक्ष (dry), सूक्ष्म (minute), द्रव (liquid), स्थिर (stable) and लघु (light) in nature.

For example – अहिफेन, भंगा, मोचरस, कुटज, लोध्र, जातीफल, लाक्षा, वासा, खदिर,
दूर्वा, अशोक, उशीर, रक्तचन्दन, पद्मक, प्रवाल, गैरिक etc.

Indications of स्तम्भन → पित्त प्रकृति, क्षार-अग्नि दग्ध, वमन (छर्दि) or अतिसार
पीडित, विष, स्वेद-अतियोग, अतिप्रवृत्ति-स्रोतोदुष्टि etc.

सम्यक् स्तम्भन लक्षण → रोग शान्ति & बल प्राप्ति

अतिस्तम्भन लक्षण → श्यावता – स्तब्धगात्रता – उद्वेग – हनुग्रह – हृदग्रह – वर्चग्रह etc.

अतिस्तम्भन चिकित्सा → स्नेहन & स्वेदन, उष्ण-स्निग्ध-सर गुणयुक्त द्रव्य प्रयोग

शोधन & शमन

• शोधन चिकित्सा →

यद् इर्येद् बहिर्दोषान् पंचधा शोधनं च तत् ।

निरूहो वमनं कायशिरोरेको अस्त्रविस्तृतिः॥ (अ.ह.सू.14/5)

शोधन कर्म are the purificatory procedures those eliminate the aggravated doshas from the body. These are five in numbers; hence they are called as पंचकर्म. According to Acharya Sushruta and Vagbhata, Shodhana karmas are as follows →

वमन – विरेचन — निरूह बस्ति – शिरोविरेचन – रक्तमोक्षण

Indications of शोधन →

बहुदोष अवस्था – अविपाक – अरुचि – स्थौल्य – पाण्डुता – गौरव – क्लम – पिडका – कोठ – कण्डु – स्तम्भ – अरति – आलस्य – श्रम – दौर्बल्य – अवसाद – कफोत्क्लेश – पित्तोत्क्लेश – निद्रानाश – अतिनिद्रा – तन्द्रा – क्लैब्य – बलनाश – वर्णनाश etc.

Contraindication of शोधन → (अचिकित्स्य पुरुष)

अनपवाद प्रतीकार – अधन – अपरिचारक – वैद्यमानिन – चण्ड – असूयक – तीव्र धर्म अरुचि – अतिक्षीण बल मांस शोणित – असाध्य रोगोपहत – मुमुर्षु लिंगान्वित

Importance of शोधन →

→ ऋतु अनुसार शोधन (रोगानुत्पत्तिकर चिकित्सा) – e.g. वमन in वसंत ऋतु, विरेचन & रक्तमोक्षण in शरद ऋतु, बस्ति in वर्षा ऋतु – To prevent the diseases.

→ रसायन-वाजीकरण पूर्व शोधन – Acharyas has mentioned that “As dirty clothes (मलिन वस्त्र) cannot be coloured properly, patients without शोधन (अविशुद्ध शरीर) cannot get proper result of रसायन & वाजीकरण”.

→ दोषाः कदाचित् कृप्यंति जिता लंघनपाचनैः ।

जिताः संशोधनैर्येषु न तेषां पुनरुद्भवः ॥ (च.सू.16/20)

The diseases, treated with शमन चिकित्सा like लंघन-पाचन, may recur but the diseases treated with शोधन चिकित्सा do not recur.

1. वमन कर्म – Emesis

तत्र दोषहरणं उर्ध्वभागं वमन संज्ञकम् ।

Therapeutic procedure of eliminating the aggravated dosha upwards (i.e. through mouth) is called वमन कर्म.

It is considered as best purificatory treatment for कफ दोष & आमशय.

2. विरेचन कर्म – Purgation

तत्र दोषहरणं अधोभागं विरेचन संज्ञकम् ।

Therapeutic procedure of eliminating the aggravated dosha downwards (i.e. through anus) is called विरेचन कर्म.

It is considered as best purificatory treatment for पित्त दोष.

3. निरूह बस्ति – Decoction enema

“वसु निवासे” “वस् आच्छादने” The word बस्ति denotes the meaning of residing or covering the site of application. The बस्ति द्रव्य reside and cover the पक्वाशय and then eliminate the aggravated dosha.

“बस्तिना दीयते इति बस्ति” This procedure is called बस्ति because of the use of animal bladder as बस्तिपुटक.

It is considered as best purificatory treatment for वात दोष & पक्वाशय.

4. शिरोविरेचन (नस्य कर्म) – Errhines

औषधं औषधसिद्धं स्नेहो वा नासिकाभ्यां दीयते इति नस्यम् ।

Administration of medicines or medicated oils through nostrils is called नस्य. It is of three types based on action – रेचन, शमन & बृंहण. शिरोविरेचन refers to elimination of aggravated dosha through nostrils (i.e. Errhine). It is considered as best purificatory treatment for उर्ध्वजत्रुगत रोग (शिरोरोग-नासा-कर्ण-मन्या etc.)

5. रक्तमोक्षण – Blood letting (Venesection, leech therapy etc.)

रक्तस्य मोक्षणं रक्तमोक्षणम् ।

Letting out the vitiated blood is called रक्तमोक्षण. It is considered as best purificatory treatment for रक्तज व्याधि and त्वक गत विकार, and also used in पित्त प्रकोप.

• शमन चिकित्सा →

न शोधयति यद्दोषान् समानोदीरयत्यपि।

समीकरोति क्रुद्धाश्च तत् संशमनमुच्यते ॥ (सु.सू.1, डल्हण)

Treatment measures those neither eliminate the doshas, nor aggravate the doshas, but they pacify the aggravated dosha and bring equilibrium in the body, are called Shamana chikitsa.

There are 7 types of शमन चिकित्सा including 2 द्रव्यभूत & 5 अद्रव्यभूत चिकित्सा.

1. पाचन (आमदोष पाचन) – Using digestants e.g. चित्रक, मुस्ता
2. दीपन (अग्नि दीपन) – Using appetizers e.g. त्रिकटु (शुण्ठी, मरिच, पिप्पली)
3. क्षुधा (उपवास) – Fasting / or intake of less food
4. तृष्णा – Intake of less or no water
5. आतप सेवन – Exposure to sun rays
6. मारुत सेवन – Exposure to fresh air
7. व्यायाम – Exercise

Indications of शमन →

मध्यम & अल्प दोष अवस्था – वृद्ध – बाल – गर्भिणी – दुर्बल – मन्दाग्नि – अजीर्ण – छर्दि – अतिसार – विसूचिका – अलसक – हृद्रोग – गौरव – उद्गार – हृल्लास – विबन्ध etc.

Contraindication of शमन → अचिकित्स्य पुरुष

• Considerations for संशोधन & संशमन चिकित्सा →

- शोधन should always be used after अग्निदीपन-आमपाचन, after bringing कुपित दोष towards कोष्ठ from शाखा, and after making दोषोत्क्लेष; those can be achieved by दीपन-पाचन, स्नेहन-स्वेदन & शोधन पूर्व भोजन व्यवस्था.
- If शोधन कर्म is performed without अग्निदीपन & आमपाचन, the शोधन द्रव्य (drugs) are not digested properly and may lead to adverse effects.
- If शोधन कर्म is performed without कोष्ठगमन of कुपित दोष, the complete elimination of aggravated dosha can not be achieved that can result in relapse or recurrence of disease.

→ If शोधन कर्म is performed without दोषोत्क्लेष, the patient has to suffer just as extraction of juice from an unripe fruit results in destruction of that fruit.

→ In case of बहुदोषावस्था & बलवान् रोगी, the संशोधन चिकित्सा should be used and then संशमन चिकित्सा. And in case of अल्पदोष & दुर्बल रोगी, the संशमन चिकित्सा should only be used.

निदान परिवर्जन

- संक्षेपतः क्रियायोगो निदानपरिवर्जनम्। (सु.उ.1/25)
Avoiding the etiological factors is itself a treatment. निदान परिवर्जन is having utmost importance in Ayurvedic treatments.
- निदान परिवर्जन refers to avoiding etiological factors by patient and also treating the etiological factors by physician, e.g., In case वातव्याधि, the etiological factors of वात दोष are to be avoided by patient, and the physician has to treat वात प्रकोप.
- Various diseases can be cured with निदान परिवर्जन only, while all the Ayurvedic treatments are incomplete without निदान परिवर्जन.
- If a patient continues निदान सेवन, physician cannot treat him successfully. Or if the disease subsides due to शोधन & शमन but patient continues निदान सेवन, it leads to relapse or recurrence of disease that can advance to chronic or complicated stage.
- To avoid relapse or recurrence of any disease, patient has to avoid the etiological factors even after successful treatment.
- निदान परिवर्जन not only helps in treatment but it also helps in prevention of the diseases. For example, in ऋतुचर्या पालन the etiological factors of the respective accumulating doshas should be avoided to prevent their aggravation such as during वसंत ऋतु one should avoid कफवर्धक आहार-विहार.

औषध मात्रा

स्थितिर्नास्त्येव मात्रायाः कालमग्निं वयो बलम् ।
प्रकृतिं दोषदेशौ च दृष्ट्वा मात्रां प्रकल्पयेत् ॥ (शा.सं.पू.1/37)

औषध मात्रा (dose of a medicine) should always be decided with consideration of following factors →

- अग्नि – मन्दाग्नि / समाग्नि / तीक्ष्णाग्नि / विषमाग्नि
- वय – बाल / मध्यम / वृद्ध
- बल – अवर / मध्यम / प्रवर
- दोष – वात / पित्त / कफ / संसर्ग / सन्निपात / दोष बलाबल / दोष गति (वृद्धि-क्षयादि)
- प्रकृति – वातज / पित्तज / कफज / वातपित्तज / वातकफज / पित्तकफज / सम त्रिदोषज
- देश – भूमि देश (जांगल / आनूप / साधारण) & आतुर (दशविध परीक्षा)
- काल – ऋतु (हेमन्त-शिशिर-वसन्त-ग्रीष्म-वर्षा-शरद) / औषधसेवन काल (प्रातः / सांय)

बलानुसार औषध मात्रा → Inadequate or less dose of a medicine is unable to cure a disease just as less quantity of water is unable to stop a fire. Overdose of a medicine may lead to adverse effects and complications just as the excess rain destroys the crops. Therefore, dose of a medicine should neither be less nor be more, and it should be decided with proper assessment of दोषबल, व्याधिबल & रोगीबल.

बाल्यावस्थानुसार औषध मात्रा → Medicines should be given in द्रव or लेह रूप to children. चूर्ण or कल्क should be licked with घृत-मधु. वटी or गुटिका should not be given. According to Acharya Sushruta, upto 16 years of age there is बाल्यावस्था (कफ प्रधान) and the बाल्यावस्था is having three subtypes →

1. क्षीरप → The children of 1 month to 1 year of age and those consume breastmilk only are called क्षीरप. If medicine has to be given to these children, then it should be given to the nursing mother and the medicine can also be coated on nipples of mother in the quantity of अंगुलिपर्वद्वय मात्रा.

2. क्षीरान्नाद → Those children who consume breastmilk (क्षीर) and foods (अन्न) are called क्षीरान्नाद. They are of 1 to 2 years of age. चूर्ण or कल्क in dose of कोलास्थि मात्रा should be given to them.
3. अन्नाद → Those children who consume foods only are called अन्नाद. They are of 3 to 16 years of age. कोल मात्रा औषध should be given to them.

सामान्य औषध-मात्रा (Adult dose) →

क्र.	औषध कल्प	औषध मात्रा (dose)	
1	रस, भस्म, लौह, मण्डूर, पिष्टी	1 to 3 रत्ती	125 to 375 mg in divided dose
2	पर्पटी	2 to 4 रत्ती	250 to 500 mg in divided dose
3	वटी	½ to 1 तोला	500 mg to 1 gm in divided dose
4	स्वरस	½ पल	24 ml in divided dose
5	कल्क	1 कर्ष	12 gm in divided dose
6	क्वाथ	1 to 2 पल	48 to 96 ml in divided dose
7	चूर्ण	1 कर्ष	6 to 12 gm in divided dose
8	अवलेह पाक	½ पल	12 to 24 gm in divided dose
9	सिद्ध घृत तैल	1 पल	48 ml in divided dose
10	आसव अरिष्ट	1 पल	48 ml in divided dose

{*Note → While writing शमनौषधि for any disease → write common dosage and anupana, such as → रसौषधि – 125 to 250 mg BD/ TID त मधु / उष्णजल
चूर्ण – 3 to 6 gm BD/ TID त मधु / घृत / उष्णजल
वटी / गुग्गुलु – 500 mg to 1 g (1 to 2 pills) BD/ TID त उष्णजल
क्वाथ / कषाय – 24 to 48 ml BD / TID त जल / कोष्णजल
अवलेह/ पाक – 12 to 24 gm OD/ BD त दुग्ध / उष्णजल
घृत/ तैल – 12 to 24 ml OD/ BD त दुग्ध / उष्णजल
आसव/अरिष्ट – 12 to 24 ml OD/ BD त जल / गोमूत्र }

औषध सेवन काल

औषध सेवन काल (time of administration of medicine) depends upon रोगी-रोग अवस्था. For successful treatment, administration of medicine with proper dosage is essential.

भैषज्यकालो भुक्तादौ मध्ये पश्चान्मुहुर्मुहुः ।

सामुद्रं भक्तसंयुक्तं ग्रास ग्रासान्तरे दश ॥ (च.चि.30/298)

Acharya Charaka has mentioned 10 types of औषध सेवन काल →

1. अभक्त → Empty stomach

Intake of medicine empty stomach is useful in कफ प्रकोप & in case of बलवान रोगी.

2. पूर्वभक्त (प्राक् भक्त) → Before food (Pre-prandial)

Intake of medicine few minutes before the food is useful in अपान-वायु विकार such as विबन्ध, उदावर्त, अश्मरी, मूत्रकृच्छ्र, प्रदर etc.

3. मध्यभक्त → In between the intake of food

After intake of half of the food, administration of medicine and then intake of remaining food is called मध्यभक्त. It is useful in समान-वायु विकार such as ग्रहणी, अन्नद्रवशूल, परिणामशूल, उदरशूल, अतिसार, प्रवाहिका, अलसक, विलम्बिका etc.

4. सभक्त (सान्न) → Mixed with foods

Intake of medicine mixed with food is useful in अरुचि.

5. पश्चाद् भक्त (अधोभक्त) → After food (Post-prandial)

Intake of medicine just after food is useful in व्यानवायु विकार such as उच्च रक्तचाप & in उदान वायु विकार such as स्वरभेद etc.

6. सामुद्र → Pre & Post-prandial

Intake of medicine before and after food (pre & post prandial) is useful in कम्प, आक्षेपक, हिक्का etc.

7. भक्तान्तर → Repeatedly in between the meals

Intake of medicine frequently in between meals is useful in अरुचि etc.

8. सग्रास → With every bite (morsel) of meal

Intake of medicine with every morsel is useful in प्राणवायु विकार such as श्वास etc.

9. ग्रासान्तर (कवलान्तर) → In between two morsels

Intake of medicine in between two morsels is useful in प्राणवायु विकार.

10. मुहुर्मुहु → Frequent intake of medicine

Frequent intake of medicine is useful in विष विकार, छर्दि रोग, हिक्का, तृष्णा, श्वास, कास etc.

Acharya Vagbhata has told one more type →

11. निशि → At night (before going to bed) →

Intake of medicine at night is useful in उर्ध्वजत्रुगत विकार.

अनुपान

“अनु सह पश्चाद् वा पानं अनुपानम्” Anupana refers to substances taken with or after diet and drugs.

यदाहारगुणैः पानं विपरीतं तदिष्यते ।

अन्नानुपानं धातूनां दृष्टं यन्न विरोधि च ॥ (च.सू.27/319)

The best Anupanas have properties opposite to diet and drugs which is taken but not opposite to the Dhatus.

Anupana is considered as half medicine in Ayurveda, because it provides nourishment to the Dhatus, help in digestion of food, digestion & absorption of administered drugs etc. Anupana acts as vehicle for drugs which help in quick spread and action of the drugs in the body, and hence enhances the efficacy of drugs.

अनुपान कर्म (Actions of Anupana) → Anupana provide a medium of administration of drugs with acceptability & palatability. They acts as →

अनुपानं तर्पयति, ऊर्जयति, बृंहयति, पर्याप्तिमभिनिर्वर्तयति, भुक्तमवसादयति, अन्नसंघातं भिनत्ति, मार्दवमापादयति, क्लेदयति, जरयति, सुखपरिणामिता-माशुव्यवायितां चाहारस्योपजनयतीति। (च.सू.27/325)

Anupana brings about pleasure, energy, nourishment, satisfaction and steadiness in the food eaten. It helps in breaking down, softening, moistening, digestion, assimilation, and instant diffusion of food and drugs taken.

अनुपान निर्धारण → Anupana intake is decided on following three basis →

1. औषध-आहार अनुसार (According to Drugs and Diet)
2. रोगी अनुसार (According to Patient)
3. रोग अनुसार (According to Disease)

Anupana indicated according to drugs and diet →

क्र.	औषध व आहार	अनुपान
1	स्नेह पान (except भल्लातक & तुवरक तैल)	उष्ण जल
2	मधु – दधि – मद्य - भल्लातक तैल – तुवरक तैल	शीत जल
3	मांस or मांसरस	मद्य or फलाम्ल
4	लशुन	शर्करा, मधु or घृत
5	हरीतकी (ऋतु हरीतकी)	सैन्धव in वर्षा शर्करा in शरद शुण्ठी in हेमन्त पिप्पली in शिशिर मधु in वसन्त गुड in ग्रीष्म

Anupana indicated according to patient →

क्र.	रोगी	अनुपान
1	कृश रोगी	मांस रस or सुरा
2	स्थूल रोगी	मधूदक
3	क्षीण, बालक, वृद्ध रोगी	क्षीर

Anupana indicated according to disease →

क्र.	रोग	अनुपान	क्र.	रोग	अनुपान
1	वातज रोग	स्निग्ध-उष्ण अनुपान	18	अजीर्ण	उष्णोदक
2	पित्तज रोग	मधुर-शीत अनुपान	19	अरुचि	निम्बू स्वरस
3	कफज रोग	रुक्ष-उष्ण अनुपान	20	अनिद्रा	माहिष दुग्ध / मद्य
4	त्रिदोषज	आर्द्रक स्वरस + मधु	21	वातव्याधि	लशुन, गुग्गुलु
5	धातुक्षय	मांसरस	22	गुल्म	शिशू त्वक
6	क्षय रोग	क्षीर / मांसरस	23	प्लीहोदर	पिप्पली चूर्ण
7	ज्वर	मुस्ता, पर्पट, किराततित्त क्वाथ	24	अम्लपित्त	द्राक्षा, कूष्माण्ड
8	जीर्ण ज्वर	पिप्पली चूर्ण + मधु	25	छर्दि	लाजा / मधु
9	रक्तपित्त	दुग्ध / इक्षुरस / वासा	26	कुष्ठ	खदिरसार जल
10	पाण्डु	पुननेवा क्वाथ	27	श्वेत कुष्ठ	बाकुची फल
11	आमवात	एरण्ड तैल + गोमूत्र	28	नेत्ररोग	त्रिफला
12	वातरक्त	गुडूची	29	प्रमेह	त्रिफला, हरिद्रा
13	शूल	हिंगु + घृत	30	स्वरभेद	पुष्करमूल + मधु
14	ग्रहणी	तक्र	31	श्वास	त्रिकटु+मधु / शुण्ठी
15	अतिसार	कुटज त्वक क्वाथ	32	कास	वासा, कण्टकारी
16	अर्श	भल्लातक / कुटज	33	पार्श्वशूल	पुष्करमूल
17	कृमिरोग	विडंग	34	उन्माद	पुराणघृत / कौम्भघृत
			35	अपस्मार	ब्राह्मी / वचा स्वरस

If any anupana for particular condition, mentioned above, is not available then उष्ण जल (कोष्ण जल) should be used.

पथ्य-अपथ्य

पथ्य → पथ्य is a synonym (पर्याय) of भेषज or चिकित्सा.

पथ्यं पथोऽनपेतं यद्यच्चोक्तं मनसः प्रियम् । (च.सू.25/45)

पथ्य are those आहार-विहार which are wholesome for the पथ or स्रोतस and pleasant for मनस.

“पथः स्रोतोरुपात् शरीरमार्गात् अनपकारकं यत्तपथ्यम्” The substances, which are not harmful for srotas of body, are called पथ्य.

अपथ्य → अपथ्य is that which is harmful for body and unpleasant for mind.

पथ्यापथ्य नियामक भाव →

- पथ्यापथ्य depends upon following six factors →
 1. मात्रा, 2. काल, 3. क्रिया, 4. भूमि, 5. देह, 6. दोष.
- Consideration of all these six factors is essential before deciding पथ्यापथ्य, because use of proper पथ्य can act as a treatment itself, while improper use of पथ्य can act as अपथ्य.
- पथ्य व्यवस्था can be used as two types →
 1. स्वास्थ्य संरक्षणार्थं पथ्य व्यवस्था
 2. रोगकालीन पथ्य व्यवस्था
- अष्टविध आहारविधिविशेषायतन → For intake of beneficial diet (पथ्य आहार) Acharya Charaka has explained eight rules →

तत्र खल्विमान्यष्टावाहारविधिविशेषायतनानि भवन्ति,
तद्यथाप्रकृतिकरणसंयोगराशिदेशकालोपयोगसंस्थोपयोक्ताष्टमानि ॥ (च.वि.1/21)

 1. प्रकृति → Natural quality of dravya
 2. करण → Processing of dravyas, or Preparations of food items
 3. संयोग → Combinations of various dravyas
 4. राशि → Quantity of food

5. देश → Place of origin of the dravyas

6. काल → नित्यग (ऋतु- season) or आवस्थिक (दोष & रोग अवस्था- stages)

7. उपयोगसंस्था → Dietetic rules (Acharya charaka has explained आहार विधि विधान, and Acharya Sushruta has explained द्वादश अशन विचार)

8. उपयोक्ता → The user (consumer) has to take food as per his सात्म्यता.

• आहार विधि विधान →

Following rules are mentioned by Acharya Charaka in Vimanasthana →

→ आहार should be उष्ण, स्निग्ध and मात्रापूर्वक.

→ One should take food (आहार) after digestion of previous meal.

→ One should take अविरुद्ध वीर्य (the द्रव्य which are not वीर्यविरुद्ध)

→ One should take food while sitting in इष्ट देश (suitable place)

→ One should take food with इष्ट उपकरण (suitable dishes and dishwares)

→ During intake of food (eating or drinking) one should follow the rules like नाति शीघ्र (not too fast), नाति विलम्ब (not too slow), अजल्पन् (not while talking), अहसन् (not while laughing), स्व अग्निबल अनुसार (quantity of food should be according to person's digestion capacity), and आत्मनान् (Beneficial food should be taken with concentration).

• द्वादश अशन विचार →

Acharya Sushruta has explained 12 types of food items (द्वादश अशन) those are as follows →

1. शीत आहार → useful for उष्णता, मद & तृष्णा पीडित

2. उष्ण आहार → useful for कफ वातज रोग, स्नेहपीत, विरिक्त व्यक्ति

3. स्निग्ध आहार → useful for वात प्रकृति, वातव्याधि पीडित, कृश & दुर्बल व्यक्ति.

4. रुक्ष आहार → useful for मेदस्वी (मेदोवृद्धि), कफ प्रकृति, कफज रोगी, & प्रमेही.

5. द्रव आहार → useful for शुष्क शरीर, दुर्बल शरीर, & तृष्णा पीडित.

6. शुष्क आहार → useful for कुष्ठ, विसर्प, क्लिन्न शरीर (क्लेदयुक्त) & प्रमेही.

7. एक कालिक आहार → in case of मन्दाग्नि, food should be taken once a day (one meal in 24 hours) to increase the जठराग्नि.
8. द्विकालिक आहार → in case of मन्दाग्नि, food should be taken twice a day.
9. औषधयुक्त आहार → those patients who are औषधद्वेषी (not willing to take medicine) are given medicine mixed with food.
10. मात्राहीन आहार → persons having मन्दाग्नि or suffering from any disease (रोगी) should be given less food as compared to normal persons.
11. प्रशमन कारक आहार → ऋतु अनुसार आहार should be given for दोष प्रशमन.
12. स्वस्थवृत्ति प्रयोजक → स्वास्थ्य संरक्षणार्थ सर्वरसयुक्त आहार should be given.

• अपथ्य →

Those आहार or औषध which dislodge the दोष from their normal sites and aggravate them but do not eliminate them from body, are called विरुद्ध आहार, and these are harmful or अपथ्य.

There are 18 types of विरुद्धाहार →

देश विरुद्ध – काल विरुद्ध – अग्नि विरुद्ध – मात्रा विरुद्ध – सात्त्विक विरुद्ध – दोष विरुद्ध – संस्कार विरुद्ध – वीर्य विरुद्ध – कोष्ठ विरुद्ध – अवस्था विरुद्ध – क्रम विरुद्ध – परिहार विरुद्ध – उपचार विरुद्ध – पाक विरुद्ध – संयोग विरुद्ध – हृदय विरुद्ध – संपद विरुद्ध – and विधि विरुद्ध आहार

ऋतु अनुसार दोष प्रकोपक आहार is also अपथ्य that must be avoided for prevention of several diseases.

आहारज निदान (रोग उत्पादक हेतु) is अपथ्य that must be avoided for successful treatment.

विरुद्ध आहार & अपथ्य आहार सेवन जन्य रोग → कुष्ठ – विसर्प – विस्फोट – आमविष – क्लैब्य – जलोदर – भगन्दर – मद – मूर्च्छा – उन्माद – पाण्डु – ग्रहणी – अम्लपित्त – ज्वर
अपथ्यज रोग चिकित्सा → वमन – विरेचन – विपरीत गुणयुक्त आहार & औषध सेवन – व्याधि विपरीत चिकित्सा – रसायन सेवन etc.

Examples →

1. पथ्यापथ्य for वातज विकार →

	पथ्य	अपथ्य
आहार	घृत, तैल, गोधूम, षष्टिक शालि, पटोल, शिगु, लशुन, दाडिम, द्राक्षा, निम्बू, नारिकेल जल, गोमूत्र, गोदुग्ध, जांगल मांस	चणक, सतीन, आढकी, मुद्ग, मसूर, कोद्रव, निष्पाव, कारवेल्लक, जम्बू, पूग, शुष्क शाक, शुष्क मांस
विहार	जल क्रीडा, अभ्यंग, संवाहन, स्नान, वस्त्र धारण, बृंहण कर्म	रात्रिजागरण, अनशन, श्रम, वेगधारण, मैथुन, चिन्ता, शोक

1. पथ्यापथ्य for पित्तज विकार →

	पथ्य	अपथ्य
आहार	पुराण शालि, लाजा, गोधूम, मुद्ग, मसूर, निम्ब, पटोल, मूलक, पलाण्डु, कूष्माण्ड, अजादुग्ध, गोदुग्ध, घृत, जांगल मांसरस, आमलकी, कदली, द्राक्षा, मृद्विका, मिश्री	तिल, माष, कुलत्थ, सर्षप, गुड, दधि, क्षार, लवण, कटु तीक्ष्ण विदाही अन्नपान, मत्स्य मांस
विहार	शीतजल स्नान, शीत विहार, चन्दन लेप, मुक्ता मणिधारण, चन्द्रकिरण सेवन, वमन विरेचन नस्यादि	आतप सेवन, स्वेदन, वेगधारण, क्रोध

2. पथ्यापथ्य for कफज विकार →

	पथ्य	अपथ्य
आहार	मधु, यव, लाजा, मुद्ग, पुराण शालि, हरिद्रा, शुण्ठी, मरिच, पिप्पली, चित्रक, लवंग, एला, त्वक, तुलसी, खदिर, कटु-तिक्त-कषाय रस, तीक्ष्ण, ऊष्ण, रूक्ष आहार	नवान्न, पिष्टान्न, गोधूम, इक्षुरस, गुड विकार, क्षीर, दधि, आनूप मांस, विरुद्धाहार, विदाही अभिष्यन्दी गुरु स्निग्ध आहार
विहार	लघु व्यायाम, उद्वर्तन, वमन	दिवास्वप्न, अव्यायाम, वेगधारण

मनस & मानसिक रोग

“मन्यतेऽनेन इति मनः” / “मन ज्ञाने एवं मनु अवबोधने”

Manas or mind is defined as an object of cognition (indriya) through which one perceives knowledge.

मनस स्थान → हृदय

मनस-गुण → अणुत्वमथ च एकत्वं दौ गुणौ । (च.शा.1/19)

मनस कर्म → इन्द्रियाभिग्रहः कर्म मनसः स्वस्य निग्रहः । (च.शा.1/21)

मनस विषय → चिन्त्यं विचार्यमूह्यं च ध्येयं संकल्पमेव च ।
यत्किञ्चिन्मनसो ज्ञेयं तत् सर्वं ह्यर्थसंज्ञकम् ॥ (च.शा.1/20)

मनो-रोग सामान्य निदान →

→ अल्प अत्व

→ Vitiation of मानसिक दोष → रज & तम

→ Emotions or मनोभाव → (क्रोध-शोक-भय-हर्ष-विषाद-ईर्ष्या-मात्सर्य-काम-लोभ-मोह)

→ All of the Sharirika and Manasika rogas are caused by following three types of etiological factors →

1. असात्मेन्द्रियार्थ संयोग
2. प्रज्ञापराध
3. परिणाम (काल)

प्रज्ञापराध →

धीधृतिस्मृतिविभ्रष्टः कर्म यत् कुरुतेऽशुभम् ।

प्रज्ञापराधं तं विद्यात् सर्वदोषप्रकोपणम् ॥ (च.शा.1/102)

When a person does sinful acts because of the derangement of धी, धृति & स्मृति, it is called प्रज्ञापराध. Indulging in the improper activities knowingly is nothing but प्रज्ञापराध that leads to सर्वदोषप्रकोप.

मनो-रोग सामान्य लक्षण →

→ उद्वेग लक्षण → अस्थैर्य (instability) – वेपथु (tremor) – असहिष्णुता (intolerance) – शीघ्रक्रोप (Irritability) – स्वेदाहुल्य (Excessive perspiration) – तृट्वाहुल्य (Excessive thirst) – मुखशोष (Dryness of mouth) – भय (Fear) etc.

→ विषाद लक्षण → विषाद (depression) – उत्साहनाश (loss of enthusiasm) – आशाभंग (frustration) – अनन्नाभिलाषा (lack of interest in food) – अल्पचेष्टा (reduced activity) – अनिद्रा (sleeplessness)

• मनो-रोग सामान्य चिकित्सा →

1. दैव व्यापाश्रय चिकित्सा →

→ मन्त्र जप – औषध & मणिधारण – पूजन – यज्ञ – स्तुति – बलि – मंगल – होम – उपहार – ब्रह्मचर्य – तप – सत्य – नियम – साधुदर्शन etc.

2. युक्ति व्यापाश्रय चिकित्सा →

→ आहार – विहार – औषध प्रयोग

→ मेध्य द्रव्य → ब्राह्मी – मण्डूकपर्णी – यष्टिमधु – गुडूची – शंखपुष्पी – ज्योतिष्मति – जटामांसी – वचा – अश्वगन्धा etc.

3. सत्वाजय चिकित्सा →

Restrain mind from desire of unwholesome objects. The best approach to achieve the goal of sattvajaya chikitsa is through the propagation of: “मनसो ज्ञानविज्ञान धैर्यं स्मृति समादिभिः” (च.सू.1/52)

ज्ञान → knowledge of self

विज्ञान → professional / analytical knowledge

धैर्य → patience and confidence

स्मृति → keeping good memories

समाधि → concentration and meditation

• अद्रव्यभूत चिकित्सा →

त्रासन – भय दर्शन – विस्मापन – विस्मारण – क्षोभन – भर्त्सन – बंधन – हर्षण etc.

Nutritional Deficiency Disorders

Nutrition → Nutrition is the science or practice of consuming and utilizing foods. It includes food intake, absorption, assimilation, biosynthesis, catabolism, and excretion.

Nutrients → A nutrient is a substance used by humans to survive, grow, and reproduce. Generally nutrients are classified into macronutrients and micronutrients.

(a) Macronutrients → A class of chemical compounds which humans consume in the largest quantities and which provide humans with the bulk of energy. These are Proteins, Carbohydrates, and Fats (lipids).

(b) Micronutrients → Micronutrients are those vitamins and minerals required in very small quantities and essential for a number of physiological functions including normal growth and development.

Proteins → Proteins are organic compounds that consist of amino acids joined by peptide bonds. These are essential for growth and repair.

Carbohydrates → Carbohydrates are made of carbon, hydrogen & oxygen (CHO) such as sugars, starch, and cellulose. They are rich source of energy.

Fats → Fats are made of triglycerides and fatty acids. They are solid at room temperature. Fats are needed for construction and maintenance of cell membranes, to maintain a stable body temperature, and to sustain the health of skin and hair.

Vitamins → Vitamins are organic molecules which are essential micronutrients – these are vital substances which humans need in small quantities for the proper metabolism but cannot synthesize, either at all or in sufficient quantities, and therefore must obtain through diet.

There are two categories of vitamins →

(a) Fat soluble → Vitamin A, D, E, K

(b) Water soluble → Vitamin B complex (B₁, B₂, B₃, B₆, B₁₂ etc.) & Vit.C

Minerals → Minerals are chemical elements required as essential nutrient by humans to perform functions necessary for life. The five major minerals in the human body are calcium (Ca), phosphorus (P), potassium (K), sodium (Na), and magnesium (Mg). All of the remaining elements in a human body are called “trace elements”. The trace elements that have a specific biochemical function in the human body are Sulphur (S), Iron (Fe), Chlorine (Cl), Cobalt (Co), Copper (Cu), Zinc (Zn), Manganese (Mn), Molybdenum (Mo), Iodine (I) and Selenium (Se).

Antioxidants → The substances that protect cells against the effects of oxidative damage (i.e. free radicals – molecules produced when body breaks down food or is exposed to tobacco smoke and radiation). Vit C, vit E, β carotene etc. act as antioxidant. Rich sources of antioxidants are Apples, Berries, Carrots, Beets, Oranges, Potatoes, Green tea, Dark green veggies, Whole grains, Nuts etc.

S.No.	Macronutrients	Deficiency	Treatment
1	Proteins Source– Eggs, milk, meat, fish, lentils, cereals, nuts, soy, oats etc. RDA– 60 g /day	• In children– Kwashiorkor & Marasmus, Stunted growth • Flaky and splitting skin & patches of depigmented skin • Edema • Fatty liver • Fatigue & irritability • More prone to infections • Loss of muscle mass	• Nutritional powders • Protein shakes • Advice - Eggs, Milk, Curd, Lentil soups etc.

S.No.	Macronutrients	Deficiency	Treatment
2	Carbohydrates Source– Whole grain cereals, bread, potatoes, peas, beans, lentils, vegetables, and fruits; while refined carbs include sugar sweetened drinks, fruit juices, pastries, pasta, white rice etc. RDA– 225 to 325 g /day (2400 kcal energy for men and 2000 kcal for women is needed per day and approximately 45 to 55% of diet should be carbohydrates. 1 g carbs= 4 kcal / 1 g proteins = 4 kcal / 1 g fats= 9 kcal)	- Hypoglycemia → Hunger → Sweating → Fainting → Confusion → Slurred speech - Ketosis → Mental fatigue → Bad breath → Nausea → Headache → Painful swelling of the joints	- If mild- 3-4 tsf of sugar or chocolate, or glucose tablets - If moderate- Glucose water, or fruit juice - If severe- 25-50 ml 25-50% IV Dextrose, or Inj. Glucagon 1 mg SC or IM - Advice Carbs-rich diet
3	Fats Source– Cheese, butter, cream, whole milk, meat, coconut oil, palm oil etc. Essential fatty acid occurs naturally in whole grain cereal, seeds and nuts. RDA– 25 g /day	- Scaly dermatitis - Dry, scaly, flaky, dull, or bumpy skin - Dry, brittle, or lusterless hairs - Alopecia - Thrombocytopenia	Advice Essential fatty acids - rich diet→ Flax-seeds, Soybeans, Seafood, Ghee, Milk, Almonds, Walnuts etc.

S.No.	Micronutrients	Deficiency	Treatment
1	Vitamin A Source– Milk, butter, liver and fish liver oils, egg yolk, carrots, sweet potatoes, spinach etc. RDA– Retinol- 600 µg/day β carotene- 4800 µg/d (Retinol is vitamin A, and β carotene is a precursor of vitamin A)	Eyes → - Nyctalopia - Xerophthalmia → Conjunctival xerosis → Bitot's spot → Corneal ulceration with xerosis Skin → - Xerosis - Phrynodema (toad skin) - Follicular papules	- Tab. Vit. A (Retinol) 50,000 IU - Inj. Aquasol-A 50,000 IU per 2ml - Multivitamin capsules or syrup For eyes → - Soframycin eye ointment - Atropine eyedrops
2	Vitamin D (vit.D ₃ -cholecalciferol) Source– Milk, egg yolk, butter, yeast, liver, fish liver oil, synthesis in the skin when irradiated. RDA– 400 IU(10 µg/d)	In children → - Rickets - Tetany In adults → - Osteomalacia	- Inj. Vit. D3 - Oral calcirol sachets (one sachet- 60,000 IU) with milk - Calcimax 250 mg tab.
3	Vitamin E (Tocopherol) Source – Vegetable oil, sunflower oil, egg yolk, butter, maize, molasses, peas etc. RDA– 8 to 10 mg/day	- Skin → Erythematous popular eruptions - Anemia in premature infants - Ataxia - Muscle weakness - Retinopathy	Vitamin E 400 mg capsules (Tocopherol-acetate)

S.No.	Micronutrients	Deficiency	Treatment
4	Vitamin K Source– K1– Vegetables, spinach, tomatoes, fruits K2– Synthesized by bacteria in intestine. RDA– 55 µg/day	- Elevated prothrombin time (Delayed clotting time) & bleeding - Skin → Intracutaneous hemorrhage, purpura and ecchymosis.	- Vitamin K injection SC / IM / slow IV - Oral Vit. K in multivitamin tab / cap / pwd
5	Vitamin B₁ (Thiamine) Source– Whole grain cereals, rice, wheat, yeast, meat etc. RDA– 1.2 mg/day	- Beriberi → Wet beriberi → Dry beriberi - Muscle weakness - Peripheral neuropathy	- Tab Berin (100 mg) - Inj. Berin - Cap. Vit. B complex
6	Vitamin B₂ (Riboflavin) Source– Sprouted grains, germinating seeds, green vegetables, milk, egg, liver, kidney etc. RDA– 1.4 mg/day	- Angular stomatitis - Glossitis - Dermatitis - Blurred vision	- Tab. Riboflavin 5 mg BD - Inj. Riboflavin - Cap. Vit. B complex
7	Vitamin B₃ (Niacin) Source– Liver, kidney, meat, poultry, fish, legumes, rice etc. RDA– 16 mg/day	Pellagra characterized by – D3 - Diarrhea Dermatitis Dementia	- Rx Tab. Niacinamide 300 to 500 mg, or B complex - Avoid sunshine - Apply soothing lotion to skin

S.No.	Micronutrients	Deficiency	Treatment
8	Vitamin B₆ (Pyridoxine) Source– Whole grains, legumes, milk, liver, meat etc. RDA– 2 mg/day	- Dermatitis - Glossitis - Microcytic anemia - Weakened immunity	- Inj. Pyridoxine 100 mg stat - Tab Pyridoxine 10 mg BD
9	Vitamin B₁₂ (Cyanocobalamin) Source – Only animal sources such as milk, liver, kidney, meat, fish, egg etc. RDA– 1 µg/day	- Macrocytic anemia (megaloblastic anemia) - Stomatitis - Glossitis - Neuropathy - Reduced sensation of touch - Tingling feet	- Inj. Cyanocobalamin, Hydroxocobalamin - Cap. or Syrup of Vit. B complex, or Multivitamins
10	Folic acid (Folate / Vitamin B ₉) Source – Dairy products, liver, meat, fruits, cereals etc. RDA– 200 µg/day Dietary Folate	- Megaloblastic anemia - Pallor - Fatigue - Stomatitis	Tab. Folic acid 5 mg alone or with combination of iron or other vitamins
11	Vitamin C (Ascorbic acid) Source – Only vegetables – Citrus fruits, goose berry, lemon, orange etc. RDA– 40 mg/day	Scurvy → Bleeding gums → Bruising → Rash → Fatigue → Weight loss	- Inj. Vit. C - Tab Vit. C 500 mg - Vit. C drops for infants - Amla & Citrus fruits

S.No.	Minerals	Deficiency	Treatment
1	Calcium (Ca) Source– Milk, cheese, yogurt, soy beans, lentils, millets, nuts, fish etc. RDA– 600 mg/day (For children 800 mg/day & For pregnant and lactating woman 1200 mg/day)	• Hypocalcemia • Tetany • Muscle cramps • Weak and brittle nails • Osteoporosis	• Inj. Calcium gluconate • Calcium gluconate IV • Tab Calcium & vit D₃ • Calcium rich diet
2	Phosphorus (P) Source– Milk, beans, lentils, millets, nuts, pumpkin seeds, chicken, fish etc. RDA– 600 mg/day	• Hypo-phosphatemia • Rickets • Osteomalacia • Osteoporosis	• Phosphorus supplement orally or IV • Vitamin D supplement
3	Iron (Fe) Source– Spinach, beetroot, gooseberry (amla), dates, pomegranates, raisins, seeds, lentils, meat, fish etc. RDA– 17 to 21 mg/day (For pregnant women- 35 mg of iron per day)	• Iron deficiency anemia • Pallor • Dyspnea • Tachycardia • Dizziness • Fatigue	• Tab ferrous sulfate / fumarate • If oral iron is not tolerated → Inj. Imferon (Iron dextran) • Advice iron-rich diet
4	Iodine (I) Source– Iodized salt, dairy products, peas, beans, seafood, meat, eggs etc. RDA– 150 µg/day (One teaspoon of iodized salt contains approximately 400 µg iodine.)	• Goiter • Hypothyroidism • Unexpected weight gain • Hypothermia • Sensitivity to cold	• Tab. Levo-thyroxine 50 µg (0.05 mg) • Radioactive iodine • Surgery

5	Zinc (Zn) Source– Meat, seafood, pomegranates, legumes, seeds, nuts, dairy products, eggs, whole grains etc. RDA– 12 mg/day	• Thinning or loss of hairs (alopecia) • Eczema • Xerosis • Low immunity • Diarrhea • Weight loss	• Zinc supplements • Multivitamin capsules or syrup
6	Magnesium (Mg) Source– whole wheat, spinach, almonds, cashews, peanuts, banana etc. RDA– 340 mg/day	• Muscle twitching and cramps • Fatigue and muscle weakness • Mental disorders • Osteoporosis	• Oral magnesium supplements • IV Fluids (Electrolytes) • Diet
7	Sodium (Na) Source– Table salt, pickles, soy sauce, beets, carrots, spinach, sweet potatoes etc. RDA– 1.6 g sodium, or 4 g salt (2.54 g salt contains 1 g sodium)	• Hyponatremia • Tremors • Muscle weakness • Fatigue • Headache	• Intake of less water orally • IV Fluid (Normal saline) • Treat the underlying cause

Emaciation →

Emaciation is a state of extreme leanness. It is not a disease, but is an important symptom of various disorders.

Etiology → Malnutrition – Diseases of GIT – Marasmus – Addison's disease – TB – Anorexia nervosa – Cancer – Diabetes – Hyperthyroidism – Chronic diarrhoea – Parasites – Insomnia – Starvation – AIDS etc.

Management → To treat underlying cause – Symptomatic management – Nutritious and easily digestible diet – Nutritional supplements etc.

Obesity →

Obesity describes a weight of 120% or above. Obesity is often expressed in terms of body mass index (BMI), but pathophysiologically may be considered to be present when sufficient body fat has accumulated to adversely affect health.

BMI is calculated by measuring an individual's weight in kg, and dividing his/her height in meters square →

$$\text{BMI} = \frac{\text{Weight in kilograms}}{(\text{Height in meters})^2} = \frac{\text{kg}}{\text{m}^2}$$

WHO classification of overweight		
Classification	BMI (kg/m ²)	Associated health risk
• Underweight	< 18.5	Low (but risk of other problems increased)
• Normal	18.5 - 24.9	Average
• Overweight		
→ Pre-obese	25 - 29.9	Increased
→ Obese class I	30 - 34.9	Moderately increased
→ Obese class II	35 - 39.9	Severely increased
→ Obese class III	> 40	Very severely increased

Etiology →

Over eating – Lack of exercise – Hereditary predisposition or idiopathic or genetic obesity – Endocrinal factors (Hypothyroidism, Cushing's syndrome, PCOD / PCOS, Tumours, Puberty adiposity, Post-menopause, Pregnancy etc.)

Systemic effects of obesity →

1. **Cardiovascular system** → Coronary artery disease, or Ischemic heart disease (IHD) – Hypertension – Varicose veins – Pedal edema
2. **Respiratory system** → Breathlessness etc.
3. **Gastrointestinal system** → Hiatus hernia – Gallstones – Gall bladder diseases – Pancreatitis – Constipation etc.
4. **Joints** → Gout – Osteoarthritis – Backache etc.
5. **Pregnancy** → Neural tube defect – Perinatal mortality – Pre-eclampsia – Gestational diabetes – Preterm labour – DVT etc.
6. **Endocrine and Metabolic** → NIDDM – Hyperlipidemia – Hirsutism – Menstrual irregularities – Menorrhagia – Impotency etc.
7. **Skin** → Excess sweating – Dermal and sweat rashes etc.
8. **Psychosocial** → Psychosocial and emotional stress

Investigations → Lipid profile, Blood sugar, Hormonal assay, USG, BMI, Measurement of body fat with Skinfold Calipers / DEXA scan / Bioelectrical Impedance Analysis etc.

Management →

- Exercise → Extra calories should be burnt with exercises unless there is medical contraindication.
- Diet → 800 to 900 kcal daily with restricted diet – Bulkiness of food is important as the patient needs to be satiated, dietary fibers, salad, sprouts, oats, and protein-rich but low carbs diet with honey-water or warm water in the morning and metabolic booster herbs.
- Hormonal therapy (to treat endocrine diseases, if present)
- Use of appetite suppressants & Psychotherapy
- Surgery (if required)

वार्धक्यजन्य विकार

The branch of medicine dealing with the health and care of old people is known as Geriatrics (जरा चिकित्सा – a branch of अष्टांग आयुर्वेद).

आचार्य चरक has considered more than 60 years of age as जीर्ण or जरावस्था, while आचार्य सुश्रुत mentioned 70 years of age. जरावस्था is वात-प्रधान and also known as हीनावस्था. जरा (aging) is one among स्वाभाविक व्याधि.

वार्धक्यजन्य विकार refers to diseases of elderly (Loss of physical and mental abilities in old age is termed as senile). The diseases which are produced due to aging are called वार्धक्यजन्य विकार.

For example – खालित्य (Baldness), पालित्य (Greying of hair), इन्द्रिय दौर्बल्यता (Visual and hearing impairments, Parkinson's disease, Alzheimer's disease etc.), तिमिर or लिंगनाश (Cataract), उच्च रक्तचाप (Hypertension), सन्धिवात (Osteoarthritis), अस्थिसौशीर्य (Osteoporosis), क्लैब्य (Impotency), Atherosclerosis, Cardiovascular diseases, Urinary incontinence, Type-2 Diabetes, Cancer etc.

General principles of geriatric care →

The following principles help in caring for older adults →

1. Many disorders are multifactorial in origin and are best managed by multifactorial interventions.
2. Diseases often present atypically.
3. Not all abnormalities require evaluation and treatment.
4. Complex medication regimens, adherence problems, and polypharmacy are common challenges.

In Ayurveda वृद्ध रोगी is contraindicated for संशोधन चिकित्सा. They can be treated with अभ्यंग, मृदु स्वेद and बृंहण बस्ति, followed by संशमन चिकित्सा with proper assessment of रोगी-रोग बल and अग्निबल.

रसायन-वाजीकरण प्रयोग is best for improving स्मृति-मेधा-अग्नि-देह-इन्द्रियबल, it also delays the aging, e.g., च्यवनप्राश, ब्रह्म रसायन, मेध्य रसायन, शिलाजतु etc.

इन्द्रियप्रदोषज विकार

इन्द्रियाणि समाश्रित्य प्रकुप्यन्ति यदा मलाः ।

उपघातोपतापाभ्यां योजयन्तीन्द्रियाणि ते ॥ (च.सू.28/19)

When the vitiated doshas get localized in sense organs, इन्द्रिय प्रदोषज विकार is produced that results in either उपघात (destruction) or उपताप (diseases) of sense organs.

इन्द्रिय प्रदोषज विकार →

1. इन्द्रिय उपघात – destruction or complete damage of sense organs
e.g. लिंगनाश, घ्राणनाश, बाधिर्य, स्पर्शज्ञान etc.
2. इन्द्रिय उपताप – diseases or partial damage of sense organs
e.g. पूयालस, अर्म, अभिष्यन्द, कर्णशूल, कर्णक्ष्वेद, प्रतिश्याय, नासा अर्श etc.

इन्द्रिय प्रदोष चिकित्सा →

- निदान परिवर्जन
- शरीर शोधन
- शमनौषधि प्रयोग
- स्थानिक चिकित्सा
 - प्रतिसारण, सेक, आश्रयोतन, अंजन, विडालक etc. for नेत्ररोग
 - प्रक्षालन, प्रमार्जन, कर्णधूपन, कर्णपूरण etc. for कर्णरोग
 - प्रक्षालन, नस्य, धूम etc. for नासारोग
 - प्रतिसारण, धूम, कवल-गण्डूष etc. for जिह्वारोग
 - प्रलेप, प्रदेह, रक्तमोक्षण etc. for त्वकरोग
- शस्त्र चिकित्सा (भेदन / छेदन/ लेखन / व्यधन)
- पथ्यापथ्य पालन

“नासा हि शिरसो द्वारं” For all types of diseases of sense organs, नस्य कर्म is the best treatment, as the नासा (nostrils) are gateway of शिरस् (head-region) and शिरस् is the seat of all the sense organs.

A physician should use both सर्वदैहिक & स्थानिक चिकित्सा to cure the इन्द्रिय प्रदोषज विकार.

Alzheimer's disease

Alzheimer's disease (AD) is the commonest type of dementia. It is neuro-degenerative diseases that usually occurs after the age of 65 years, and is, therefore, considered senile dementia.

Dementia is chronic brain disorder in which there is progressive decline in acquired intellect, behavior and personality.

Etiology →

- Age – After 65 years of age, the incidence approximately doubles for every 5-year increase in age; at least until age 90 years. The incidence is similar in men and women but the prevalence is greater in women.
- Genetics - About 70% of the risk is believed to be genetic with many genes usually involved.
- Other risk factors include a history of head injuries, depression, or hypertension. The disease process is associated with plaques and tangles in the brain.

Clinical Features →

- Cognitive symptoms → The most common early symptom is difficulty in remembering recent events i.e. short-term memory loss. Patients have difficulty learning and retaining new information. Other cognitive defects include anomia, aphasia, apraxia, agnosia etc.
- Non-cognitive symptoms → As the disease advances, symptoms can include problems with thought (delusion of theft, infidelity and abandonment), disorders of perception (auditory, visual and other hallucinations), misidentifications, disorientation (including easily getting lost), mood swings, aggression or depression etc.

Investigations → CT scan, MRI, PET scan etc.

Management →

There is no cure for Alzheimer's disease; available treatments offer relatively small symptomatic benefit but remain palliative in nature. Current treatments can be divided into pharmaceutical, psychosocial and caregiving.

1. To treat the cognitive problems →
 - (a) Acetyl-cholinesterase inhibitors:
 - Tacrine
 - Rivastigmine
 - Galantamine
 - Donepezil
 - (b) NMDA receptor antagonist:
 - Memantine hydrochloride
2. To treat non-cognitive symptoms →
 - (a) Non-pharmacological treatment, particularly if symptoms not severe. Assessment of clear precipitants (e.g. suboptimal propting, personal care or toileting), social interaction, activity, and exercise.
 - (b) In depression – Non-tricyclic antidepressants (e.g. selective serotonin re-uptake inhibitors, trazodone, escitalopram)
 - (c) In psychosis – Atypical antipsychotics (e.g. risperidone, olanzapine)

Ayurvedic management → Elderly persons should be kept happy and family members should care and obey them. Vaidya should use मेध्य रसायन & वयस्थापन द्रव्य to keep elderly patient healthy, e.g. ब्राह्मी घृत, स्मृतिसागर रस, सारस्वतारिष्ट, अश्वगन्धारिष्ट, च्यवनप्राश, स्वर्ण भस्म etc.

Sleep disorders

A sleep disorder, or somnopathy, is a medical disorder of the sleep patterns of a person. Some sleep disorders are serious enough to interfere with normal physical, mental, social and emotional functioning. Sleep disorders are broadly classified into dyssomnias, parasomnias, and circadian rhythm sleep disorders.

- **Dyssomnias** → A broad category of sleep disorders characterized by either hypersomnia or insomnia. The 3 major subcategories include:
 - a. Intrinsic (i.e., arising from within the body),
 - b. Extrinsic (secondary to environmental conditions or various pathologic conditions), and
 - c. Disturbances of circadian rhythm
 - **Insomnia disorder** (primary insomnia), chronic difficulty in falling asleep and/or maintaining sleep when no other cause is found for these symptoms. Insomnia can also be comorbid with or secondary to other disorders.
 - **Primary hypersomnia** → Hypersomnia of central or brain origin.
- a. **Narcolepsy**: A chronic neurological disorder, which is caused by the brain's inability to control sleep and wakefulness.
- b. **Idiopathic hypersomnia**: a chronic neurological disease in which there is an increased amount of fatigue and sleep during the day. Patients who suffer from idiopathic hypersomnia cannot obtain a healthy amount of sleep for a regular day of activities. This hinders the patients' ability to perform well, and patients have to deal with this for the rest of their lives.
- **Recurrent hypersomnia** – including Kleine-Levin syndrome
- **Posttraumatic hypersomnia**
- **Menstrual-related hypersomnia**

- **Sleep disordered breathing (SDB)**, including (non exhaustive) →
 - a. Several types of Sleep apnea
 - b. Snoring
 - c. Upper airway resistance syndrome
- **Restless leg syndrome**
- **Periodic limb movement disorder**
- **Circadian rhythm sleep disorders** →
 - Delayed sleep phase disorder
 - Advanced sleep phase disorder
 - Non-24-hour sleep–wake disorder
- **Parasomnias** → A category of sleep disorders that involve abnormal and unnatural movements, behaviors, emotions, perceptions, and dreams in connection with sleep.
 - Bedwetting or sleep enuresis
 - Bruxism (Tooth-grinding)
 - Catathrenia – nocturnal groaning
 - Sleep terror (or Pavor nocturnus)- Characterized by a sudden arousal from deep sleep with a scream or cry, accompanied by some behavioral manifestations of intense fear.
 - REM sleep behaviour disorder
 - Sleepwalking (or somnambulism)
 - Sleep talking (or somniloquy)
 - Sleep sex (or sexsomnia)

Etiology → Factors that can cause sleep problems include Physical disturbances (e.g. pain from ulcers), Medical issues (e.g. asthma), Psychiatric disorders (e.g. stress, depression, and anxiety disorders), Environmental issues (e.g. alcohol use), Genetics, Night shift work etc.

Management of Sleep disorders →

Evaluation of the Patient with the Complaint of Excessive Daytime Somnolence

Findings on History and Physical Examination	Diagnostic Evaluation	Diagnosis	Therapy
Obesity, snoring, hypertension	Polysomnography with respiratory monitoring	Obstructive sleep apnea	Continuous positive airway pressure; ENT surgery (e.g., uvulopalatopharyngoplasty); dental appliance; pharmacologic therapy (e.g., protriptyline); weight loss
Cataplexy, hypnagogic hallucinations, sleep paralysis, Family history	Polysomnography with multiple sleep latency testing	Narcolepsy-cataplexy syndrome	Stimulants (e.g., modafinil, methylphenidate); REM-suppressant antidepressants (e.g., protriptyline); genetic counseling
Restless legs syndrome, disturbed sleep, predisposing medical condition (e.g., anemia or renal failure)	Polysomnography with bilateral anterior tibialis EMG monitoring	Periodic limb movements of sleep	Treatment of predisposing condition, if possible; dopamine agonists (e.g., pramipexole); benzodiazepines (e.g., clonazepam)
Disturbed sleep, predisposing medical conditions (e.g., asthma) and/or predisposing medical therapies (e.g., theophylline)	Sleep-wake diary recording	Insomnias	Treatment of predisposing condition and/or change in therapy, if possible; behavioral therapy; short-acting benzodiazepine receptor agonist (e.g., zolpidem)

- Treatments for sleep disorders generally can be grouped into four categories:
 1. Behavioral and psychotherapeutic treatment
 2. Rehabilitation and management
 3. Medication
 4. Other somatic treatment
- The treatment of narcolepsy is symptomatic. Somnolence is treated with wake-promoting therapeutics. Modafinil is now the drug of choice, principally because it is associated with fewer side effects than older stimulants and has a long half-life; 200 to 400 mg is given as a single daily dose. Older drugs such as methylphenidate (10 mg bid to 20 mg qid or dextroamphetamine (10 mg bid) are still used as alternatives, particularly in refractory patients.
- Treatment of the REM-related phenomena cataplexy, hypnagogic hallucinations, and sleep paralysis requires the potent REM sleep suppression produced by antidepressant medications. The tricyclic antidepressants [e.g., protriptyline (10 to 40 mg/d) and clomipramine (25–50 mg/d)] and the selective serotonin reuptake inhibitors (SSRIs) [e.g., fluoxetine (10 to 20 mg/d)] are commonly used for this purpose.
- Treatment with clonazepam (0.5 to 1.0 mg qhs) provides sustained improvement in almost all reported cases of REM.
- For nocturia, symptomatic pharmacotherapy is usually accomplished with desmopressin (0.2mg qhs), oxybutynin chloride (5 to 10 mg qhs) or imipramine (10 to 50 mg qhs). Nocturia
- Treatment options for Periodic limb movement disorder include dopaminergic medications or benzodiazepines.

General debility

General debility (दौर्बल्य) means the lack of strength in a person.

General debility is a state of general weakness or feebleness that may be a result or an outcome of one or more medical conditions that produce symptoms such as pain, fatigue, cachexia and physical disability, or deficits in attention, concentration, memory, development and / or learning.

Causes of debility →

- Lack of adequate nutrition (malnutrition) leading to conditions like anemia
- Working too hard, or over exertion
- Mental stress, lack of adequate sleep, or depression
- Common cold & cough
- Prolonged infection or injury
- Sedentary life style (without much activity in the routine)
- Obesity
- Vitamin deficiency
- Thyroid disorders
- Addison's disease
- Myasthenia gravis
- Guillain-Barre syndrome
- Liver disorders
- Renal disorders
- Tuberculosis
- Cancer
- Parkinson's disease
- All types of fevers

Management →

- Treat the underlying cause
- Symptomatic treatment
- IV Fluids (to provide glucose & correct electrolyte balance)
- Multivitamin capsules or syrup
- Nutritional supplements
- Appetizers & digestants with balanced diet should be advised

Aurvedic management →

- निदान परिवर्जन
- रोग प्रशमन चिकित्सा
- मधुर बल्य सुपाच्य द्रव्य प्रयोग
- रसायन-वाजीकरण प्रयोग
- अभ्यंग with महानारायण तैल or चन्दन बला लाक्षादि तैल
- औषध योग → आमलकी – हरीतकी – शतावरी – अश्वगन्धा – सफेद मुसली – एला – लशुन – द्राक्षा – खर्जूर – दाडिम – मांसरस – घृत – च्यवनप्राश – मुसली पाक – अश्वगन्धादि चूर्ण – अश्वगन्धारिष्ट – दशमूलारिष्ट – द्राक्षासव – कुमारी आसव – लोहासव etc.

General Introduction and Principles of Management of diseases produced by genetic, environmental and iatrogenic factors

Diseases produced by genetic factors →

A genetic disease is any disease that is caused by an abnormality in an individual's genome, the person's entire genetic makeup.

The abnormality can range from minuscule to major- from a discrete mutation in a single base in the DNA of a single gene to a gross chromosome abnormality involving the addition or subtraction of an entire chromosome or set of chromosomes. Some genetic disorders are inherited from the parents, while other genetic diseases are caused by acquired changes or mutations in a preexisting gene or group of genes.

There are a number of different types of genetic inheritance including:

1. Single gene genetic inheritance →

Single gene inheritance, also called Mendelian or monogenetic inheritance. This type of inheritance is caused by changes or mutations that occur in the DNA sequence of a single gene. These disorders are known as monogenetic disorders (disorders of a single gene). e.g.,

- Cystic fibrosis,
- Sickle cell anemia,
- Marfan syndrome,
- Huntington's disease, and
- Hemochromatosis.

Single-gene disorders are inherited in recognizable patterns: autosomal dominant, autosomal recessive, and X-linked.

2. Multifactorial genetic inheritance →

Multifactorial inheritance disorders (complex or polygenic inheritance), are caused by a combination of environmental factors and mutations in multiple genes. e.g.,

- Congenital diseases (birth defects) → (a) Clefft lip & cleft palate
(b) Club foot
(c) Pyloric stenosis
(d) Congenital heart disease
(e) Spina bifida
- Adult diseases → (a) Alzheimer's disease, (b) Schizophrenia, (c) Obesity,
(d) Diabetes, (e) Hypertension, (f) Multiple sclerosis, (g) Cancer etc.

Multifactorial inheritance also is associated with heritable traits such as fingerprint patterns, height, eye colour, and skin colour.

3. Chromosome abnormalities →

Chromosomes, distinct structures made up of DNA and protein, are located in the nucleus of each cell. Because chromosomes are the carriers of the genetic material, abnormalities in chromosome number or structure can result in disease. Abnormalities in chromosomes typically occur due to a problem with cell division. e.g.,

- Down syndrome, or trisomy 21, is a common disorder that occurs when a person has three copies of chromosome 21.
- Turner syndrome (45,X0), and
- Klinefelter syndrome (47,XXY).

4. Mitochondrial genetic inheritance →

This type of genetic disorder is caused by mutations in the non-nuclear DNA of mitochondria. Mitochondria are small round or rod-like organelles that are involved in cellular respiration and found in the cytoplasm of plant and animal cells. e.g.,

- An eye disease called Leber's hereditary optic atrophy;
- A type of epilepsy called MERRF which stands for myoclonic epilepsy with Ragged Red Fibers; and
- A form of dementia called MELAS for mitochondrialencephalopathy, lactic acidosis and stroke-like episodes.

Management of Genetic diseases →

- Many genetic disorders result from gene changes that are present in essentially every cell in the body. As a result, these disorders often affect many body systems, and most cannot be cured. However, approaches may be available to treat or manage some of the associated signs and symptoms.

- For genetic conditions, treatment and management strategies are designed to improve particular signs and symptoms associated with the disorder. These approaches vary by disorder and are specific to an individual's health needs.
 - For example, a genetic disorder associated with a heart defect might be treated with surgery to repair the defect or with a heart transplant.
 - Conditions that are characterized by defective blood cell formation, such as sickle cell disease, can sometimes be treated with a bone marrow transplant. Bone marrow transplantation can allow the formation of normal blood cells and, if done early in life, may help prevent episodes of pain and other future complications.
- Some genetic changes are associated with an increased risk of future health problems, such as certain forms of cancer. One well-known example is familial breast cancer related to mutations in the BRCA1 and BRCA2 genes. Management may include more frequent cancer screening or preventive (prophylactic) surgery to remove the tissues at highest risk of becoming cancerous.
- Most treatment strategies for genetic disorders do not alter the underlying genetic mutation; however, a few disorders have been treated with gene therapy.
- **Gene therapy** refers to a form of treatment where a healthy gene is introduced to a patient. This should alleviate the defect caused by a faulty gene or slow the progression of disease. Gene therapy, along with many other treatment and management approaches for genetic conditions, are under study in clinical trials.

- **Stem cell treatments** – The ongoing researches suggest that stem cells can be used for treating the genetic disorders as well. Apart from treating diseases like diabetes, brain injuries, Osteoarthritis, learning disabilities, various types of blood cancer, Sickle cell anemia, Parkinson's, Haemophilia, and Thalassemia, etc. The defects of cells due to mutation result in a wide of variety of genetic disorders. These genetic diseases can be treated by the transplantation of stem cells where the normal stem cell replaces the defected cells.

आनुवांशिक व्याधि →

In Ayurveda, hereditary diseases are also explained by ancient sages; Acharya Sushruta termed such diseases as आदिबल प्रवृत्त, Acharya Charaka mentioned कुलज व्याधि, Acharya Vagbhata mentioned सहज व्याधि, Acharya Bhela explained it as प्रकृति प्रभाव, and Acharya Yagyavalkya explained it as संचारी.

आदिबल प्रवृत्त रोग → तत्रादिबलप्रवृत्ता ये शुक्रशोणितदोषान्वयाः कुष्ठार्शः

प्रभृतयः, तेऽपि द्विविधाः मातृजाः पितृजाश्च ॥ (सु.सू.24/6)

The diseases such as कुष्ठ, अर्श etc. produced by vitiation or defect of shukra & shonita are called आदिबल प्रवृत्त रोग. There are two types of आदिबल प्रवृत्त रोग – (1) मातृज & (2) पितृज.

Examples of आदिबल प्रवृत्त or सहज रोग → कुष्ठ – अर्श – प्रमेह – श्वित्र – अपस्मार – बाधिर्य – मूकत्व – वामन – वर्णान्धता – तिमिर – उच्च रक्तचाप – वातरक्त – अर्बुद – मेदो वृद्धि – खण्डौष्ठ – खण्ड तालु etc.

चिकित्सा → निदान परिवर्जन – पथ्यापथ्य पालन – स्वस्थवृत्त – सद्वृत्त – दोषानुसार & रोग-लक्षणानुसार चिकित्सा

Diseases produced by environmental factors →

Diseases and conditions caused by factors in the environment are collectively called environmental diseases. Stress, physical and mental abuse, diet, exposure to toxins, pathogens, radiation, and chemicals are possible causes of a large segment of non-hereditary diseases.

If a disease process is concluded to be the result of a combination of genetic and environmental factor influences, its etiological origin can be referred to as having a multifactorial pattern.

There are many different types of environmental disease including:

- Stress-related diseases such as mental health disorders, or lifestyle disease such as cardiovascular diseases, obesity, arthritis, diseases caused by substance abuse such as alcoholism, and smoking-related diseases.
- Diseases caused by pollution such as hearing impairment due to noise pollution, breathing problems due to air pollution, GIT problems due to water pollution etc.
- Disease caused by physical factors in the environment, such as skin cancer caused by excessive exposure to UV radiation in sunlight.
- Disease caused by exposure to toxic or irritant chemicals in the environment such as toxic metals.

Prevention & Management of environmental diseases →

- To prevent environmental diseases, one should avoid exposure to environmental factors such as tobacco, alcohol, pollution etc., and live healthy life such as avoiding the stress and practicing yoga and exercises etc.
- Treatment includes avoidance of etiological factors (e.g. quitting smoking etc.) and symptomatic management.

Diseases produced by iatrogenic factors →

Iatrogenic disease is the result of diagnostic and therapeutic procedures undertaken on a patient (i.e. due to the activity of a physician or therapy).

With the multitude of drugs prescribed to a single patient adverse drug reactions are bound to occur. The Physician should take suitable steps to detect and manage them.

An iatrogenic disorder occurs when the deleterious effects of the therapeutic or diagnostic regimen causes pathology independent of the condition for which the regimen is advised. Diagnostic procedures (mechanical and radiological), therapeutic regimen (drugs, surgery, other invasive procedures), hospitalization and treating doctor himself can bring about iatrogenic disorders.

Examples of iatrogenic diseases → Adverse effects of prescribed drugs; Resistance due to over use of drugs; Incomplete or improper diagnosis and treatment due to medical error or negligence; Hospital-acquired infections; Infection, scar-formation, or other complications due to faulty procedures and surgical techniques such as post-operative hemorrhage, nerve injuries, incisional hernia etc.

Prevention & Management of environmental diseases →

- Primary prevention aims to stop iatrogenic disease before it starts, usually by reducing or eliminating risk factors. In secondary prevention, iatrogenic disease is detected and treated at an early stage, before symptoms or functional losses occur, thereby minimizing morbidity and mortality.
- Physician should treat the patient after proper diagnosis.
- Drugs & therapeutic procedures should be used with consideration of indications, contraindications and their side-effects.

Drug Allergy

A drug allergy is an allergy to a drug, most commonly a medication, and is a form of adverse drug reaction. It is the abnormal reaction of the immune system to a medication. Any medication – over the counter, prescription or herbal – is capable of inducing a drug allergy.

A drug allergy is not the same as a drug side effect, a known possible reaction listed on a drug label. A drug allergy is also different from drug toxicity caused by an overdose of medication.

Causes →

When a medication causes an allergic reaction, it is called an allergen. The following is a short list of the most common drug allergens:

- Antibiotics
 - Penicillin
 - Sulfa drugs
 - Tetracycline
- Analgesics
 - Codeine
 - Non-steroidal anti-inflammatory drugs (NSAIDs)
- Antiseizure
 - Phenytoin
 - Carbamazepine

Signs & symptoms of Drug allergy →

Symptoms of allergic reactions can range from mild itching to life-threatening conditions. Many drugs can also cause side effects or intolerances such as an upset stomach. These symptoms do not always indicate a true allergy to a drug.

During an allergic reaction, histamine and other chemicals can cause symptoms that include:

- Hives
- Skin rash
- Itchy skin or eyes
- Runny nose
- Congestion
- Swelling in the mouth and throat
- Difficulty breathing

Anaphylaxis is a rare, life-threatening reaction to a drug allergy that causes the widespread dysfunction of body systems. Signs and symptoms of anaphylaxis include:

- Tightening of the airways and throat, causing trouble breathing
- Nausea or abdominal cramps
- Vomiting or diarrhea
- Dizziness or lightheadedness
- Weak, rapid pulse
- Drop in blood pressure
- Seizure
- Loss of consciousness

Treatment →

- Withdrawal of the drug
- Injections of antihistamine and corticosteroids, e.g. Inj. Avil (Pheniramine maleate) + Inj. Dexamethasone / Hydrocortisone
- Oral antihistamine and corticosteroids
- Treatment of anaphylaxis → Anaphylaxis requires an immediate epinephrine injection as well as hospital care to maintain blood pressure and support breathing.

Food allergy

A food allergy is an abnormal immune response to food. The signs and symptoms may range from mild to severe. Even a tiny amount of the allergy-causing food can trigger signs and symptoms such as digestive problems, hives or swollen airways. In some people, a food allergy can cause severe symptoms or even a life-threatening reaction known as anaphylaxis.

Food intolerance and food poisoning are separate conditions.

Causes →

While any food can cause an adverse reaction, eight types of food account for about 90 percent of all reactions:

- Eggs
- Milk
- Peanuts
- Tree nuts
- Fish
- Shellfish
- Wheat
- Soy

Signs and symptoms →

Symptoms of an allergic reaction may involve the skin, the gastrointestinal tract, the cardiovascular system and the respiratory tract.

They can surface in one or more of the following ways:

- Vomiting and/or stomach cramps
- Hives
- Shortness of breath

- Wheezing
- Repetitive cough
- Shock or circulatory collapse
- Tight, hoarse throat; trouble swallowing
- Swelling of the tongue, affecting the ability to talk or breathe
- Weak pulse
- Pale or blue coloring of skin
- Dizziness or feeling faint
- Anaphylaxis, a potentially life-threatening reaction that can impair breathing and send the body into shock; reactions may simultaneously affect different parts of the body (for example, a stomachache accompanied by a rash)

Diagnosis →

Diagnosis is usually based on a medical history, elimination diet, skin prick test, or blood tests for food-specific IgE antibodies.

Treatment →

- To prevent the food allergy, follow the strict diet.
- Total avoidance of the foods identified as allergens.
- If the food is accidentally ingested and a systemic reaction (anaphylaxis) occurs, then epinephrine (adrenaline) should be used. A second dose of epinephrine may be required for severe reactions. The person should then be transported to the emergency room, where additional treatment can be given. Other treatments include antihistamines and steroids.

[Epinephrine (arenaline) is the first-line treatment for anaphylaxis. If administered in a timely manner, epinephrine can reverse its effects. Epinephrine relieves airway swelling and obstruction, and improves blood circulation; blood vessels are tightened and heart rate is increased, improving circulation to body organs.]